# Can Trust in Public Institutions Be Restored? Evidence from Two Experimental Studies *

Guglielmo Briscese † Maddalena Grignani‡

November 9, 2026

Trust in institutions is important for compliance with policies designed to reduce negative externalities, yet it has declined in many Western countries. In two experiments, we examine whether scalable information interventions can restore institutional trust. We find that lowering the cost of checking a fact against an institution's own data raises knowledge and, in a four-month follow-up, trust in that institution. Holding the institution and the search cost fixed, offering decision-relevant information raises trust more than offering low-value information about the same agency. These findings are consistent with a model of rational inattention in which individuals allocate attention to information according to its perceived benefits and costs. We discuss the implications for designing public information campaigns.

Keywords: Institutional Trust, Inattention, Information Acquisition, Experiments.
JEL Classification Codes: D83, H83, C93

*We are grateful to the Rustandy Center for Social Sector Innovation for providing research funding for Experiment I and the TESS initiative for allowing us to field Experiment II. We thank seminar participants of the University of Chicago Experimental Economics and the University of Pompeu Fabra. We thank Marianne Bertrand, Alessandra Cassar, James Druckman, Libertad González, Ruben Enikolopov, Nicola Lacetera, and Mario Macis for their valuable comments. All errors are our own. This study obtained ethics approval from the University of Chicago IRB No. 20-0471. AEA pre-analysis plan: AEARCTR-0014160

†The University of Chicago gubri@uchicago.edu

‡Pompeu Fabra University maddalena.grignani@upf.edu

# 1 Introduction

Trust in institutions is fundamental to economic stability in liberal democracies. When citizens view institutions as competent and trustworthy, they are more likely to comply with recommended behaviors and believe that others will do the same, creating a virtuous cycle that enhances social welfare, from taxation to public health. Yet trust in public institutions has been eroding across much of the Western world, with particularly steep declines in the U.S.[1]. Studies have shown that institutional trust erodes when citizens encounter information, truthful or not, that questions an institution's integrity or performance, reducing willingness to comply with recommended behaviors, even when this carries substantial personal and societal costs (Böhm et al., 2026; Darden and Macis, 2024; León-Ciliotta et al., 2023; Martinez-Bravo and Stegmann, 2022; Algan et al., 2021; Alsan and Wanamaker, 2018). Evidence on what scalable interventions can help *rebuild* trust is much more scant.

This study addresses this gap by providing experimental evidence on how consuming information from an institution that helps individuals make better decisions affects trust beliefs. A key hypothesis behind our study is that citizens often lack familiarity with the work of an institution and how it benefits them. While they could seek information to update their beliefs, they may be unwilling to incur the search costs if the perceived benefits are not immediately clear. In the absence of belief-correcting information, citizens rely on ideologically motivated heuristics, producing biased judgments about institutional trustworthiness. This creates a vicious cycle in which individuals avoid engaging with valuable institution-provided information, such as updates on public health, environmental risks, or product safety regulation, further strengthening their mistrust and reducing compliance with welfare-enhancing policies. From this hypothesis follow our main research questions: (1) can lowering the search cost increase consumption of institution-sourced information; (2) can this durably improve trust in the institution; and (3) does the *value* of the information provided matter, or is mere exposure sufficient? We study these questions in two experiments where we exogenously vary search costs and information relevance to assess whether scalable interventions can increase engagement with institutional information, improve trust, and willingness to follow institutional guidance.

In our first experiment, we recruited a representative sample of Americans to participate in a seven-wave longitudinal survey from April to October 2020. This was a period of intense political polarization and heightened skepticism toward government agencies, providing a demanding test of whether such interventions can work even under adverse conditions. We find a rapid decline in trust, particularly in institutions managing the pandemic (i.e., the scientific community and the healthcare sector). We show that this decline is better explained by consumption of partisan media rather than personal negative experiences. In one of the survey waves, we elicit respondents' factual knowledge about the gravity of the crisis (i.e., the COVID-19 death rate) and offer a random half of the sample an optional link to the webpage of the Centers for Disease Control and Prevention (CDC), which was in charge of monitoring the death rate, to check their answers. This randomized intervention reduced the

[1]For example, Gallup (2024)'s polls show that in 2024, only 37% of Americans reported trusting public institutions, compared to 70% in 1972

cost of accessing a clear, verifiable metric of the ongoing crisis's severity, without forcing or incentivizing respondents to consume it. Because both groups answered the same question and differed only in the cost of checking the answer against the institution's own data, the design isolates the effect of making institutional information easier to consult. In the control group, respondents with low pre-treatment trust in science spent markedly less time on the question than others; offering the link closed this gap, as the model predicts when a search cost keeps mistrustful citizens from consulting an institution at all. We find that, four months later, the treatment increased accurate knowledge on other metrics of the gravity of pandemic by 5.4 percentage points (from a control-group mean of 0.53, $p = 0.08$), the share reporting higher trust in the CDC by 6.2 percentage points (from a control-group mean of 0.12, $p = 0.03$), and perceptions of CDC political independence by 6 percentage points (from a control-group mean of 0.08, $p = 0.03$). Treated respondents were also 6.6 percentage points more likely to have taken a COVID-19 test (from a control-group mean of 0.28, $p = 0.03$), although changes in compliance with other preventive behaviors are more muted. Long-term follow up meta-cognitive questions suggest that improvements in trust reflect a conscious belief updating. These results are robust to the introduction of several features aimed at minimizing the risks of social desirability bias and experiment demand effects: (i) a four-month time lag between treatment and outcomes, with the in-between period dominated by other topics on the Presidential election campaign, (ii) placebo institutions shown in random order in our outcome questions, (iii) long-term obfuscated follow up knowledge accuracy question that could not be answered using the original source, and (iv) redirected respondents' attention on another hotly debated topic (i.e., the Black Lives Matter protests) after the treatment to reduce salience.

We implemented a second experiment four years later, in August 2024, fielded ahead of a Presidential election so that both studies test the intervention under similarly polarized political conditions. This second study focused on trust in the Environmental Protection Agency (EPA), one of the most debated public institutions during that political campaign. In this experiment, we randomly assigned respondents to three different conditions: two groups received a question about current air quality in their zip code, one with an optional link to the EPA's tracker (low search cost), one without (high search cost). A third randomized group received an equivalent low-cost link for a lower-value question about the location of their EPA regional office. Holding the institutional source constant across all arms reduces the likelihood that differences in outcomes reflect messenger endorsement or experimenter demand, thereby making the effects more plausibly attributable to the content and relevance of the information itself. We find that low awareness of the EPA is correlated with low baseline trust, supporting our main hypothesis. In line with our first study, we find that reducing search costs significantly improves knowledge accuracy: close to half of respondents with a link answered correctly, compared to fewer than one in five without ($p < 0.01$). Respondents offered high-value air quality data were 6.2 percentage points more likely to view the EPA as politically independent than those who were offered lower-value office location information, a 34% increase on the office-location arm's base of 18% ($p = 0.01$), even at equivalent search costs, suggesting that the value of what the institution offers, and not simply exposure to the institution, shapes trust. Effects on other trust dimensions, benevolence (the EPA improves lives) and competence (support for the EPA's involvement in local research activities), are directionally consistent.

Across our two studies, we provide novel evidence that informational frictions contribute to institutional mistrust: citizens who face high costs of engaging with institution-sourced information may default to ideological shortcuts, and targeted outreach campaigns may reduce search costs, helping break this cycle, as long as the information is valuable to the citizen. We interpret these findings through a model of rational inattention, in which individuals allocate attention to institutional information according to its expected benefits and costs: citizens' attention is scarce and allocated to information that is valuable to make decisions, and ignoring an institution can be optimal when benefits are unclear. While rebuilding trust in public institutions is likely to be a multifaceted challenge, our results suggest that large-scale digital information campaigns may help achieve this goal. Institutions should prioritize delivering timely content that citizens may find useful for their decisions, rather than initiatives focused solely on increasing their visibility.

Our study contributes to two main streams of literature. First, we add to the literature on the relationship between institutional trust and economic outcomes[2]. Besley et al. (2026) use opinion surveys across 166 countries over three decades and find that living in a country that experiences recent economic growth is associated with higher trust in government. Other studies have shown that trust can also erode just as rapidly in response to failures or negative media coverage, reducing compliance with welfare-enhancing behaviors such as vaccination (Rafkin et al., 2021; Darden and Macis, 2024; Alsan and Wanamaker, 2018; León-Ciliotta et al., 2023), preventive health interventions (Algan et al., 2021), and tax compliance (Besley and Dray, 2024). Comparatively less attention has been dedicated to the study of strategies to restore trust. Brouwer and De Haan (2026) show in a survey experiment that educating citizens about the EU central bank's independence raises trust in the institution, and communicating the bank's institutional structure and objectives mitigates partisan misperceptions of it (Kuang et al., 2025). Similarly, Acemoglu et al. (2020) show in a lab-in-the-field experiment in Pakistan that information about positive judicial reforms raises trust in courts. We build on this limited set of studies and address some of their limitations. Information in these survey experiments is often shown unconditionally to a randomized set of respondents rather than sought by them, leaving open whether citizens would acquire it on their own, given the opportunity-cost both within and outside of a study setting. Further, these studies do not vary the content of the information provided while holding fixed that any information is provided, making it unclear whether an observed effect reflects the value of the information itself or simply exposure to the institution. This distinction carries different policy implications: if trust responds to exposure alone, outreach campaigns can succeed by maximizing contact, while if it responds only to information that citizens consider useful, campaigns must be built on delivering relevant information, an arguably more demanding effort. Finally, most studies measure the outcomes right after the intervention and do not evaluate whether the effects persist in the long run. We complement existing evidence by

[2]As we discuss when we present our survey outcomes in Section 3, we borrow from foundational political science theories that identified competence, integrity, and legitimacy as the pillars of institutional trust (Levi, 1988; Tyler, 1990; Mayer et al., 1995). Studies have linked higher trust across these dimensions and citizens' willingness to pay taxes, follow public health guidelines, and other high stakes behaviors with important externalities (Besley and Dray, 2024; Levi and Stoker, 2000; Tyler, 1990; Levi, 1988; Smith, 1759, 1776; Tyler, 1990; Levi and Stoker, 2000; Darden and Macis, 2024; Besley and Dray, 2022; Fancourt et al., 2020; Martinez-Bravo and Stegmann, 2022)

showing that trust is higher four months after our intervention, suggesting that the vicious cycle of misinformation can be broken.

Second, we contribute to the literature on rational inattention and information consumption. Rational inattention holds that attention is scarce and allocated to what matters for the decision at hand (Sims, 2003; Gabaix, 2014); consistent with this, attention to macroeconomic news rises with its payoff relevance (Link et al., 2026), inattention shapes the treatment effects recovered in randomized trials (Maćkowiak and Wiederholt, 2025), and merely making an attribute salient changes choices (Conlon, 2026). Related to this work, we build on the survey-experimental literature on information provision and belief updating (Haaland and Roth, 2023; De Quidt et al., 2018; Fuster et al., 2022; Martinez-Bravo and Sanz, 2022), which supplies the design tools we use to elicit and move beliefs credibly. What is missing from both strands is a link between the *demand* for the information an institution produces and *trust* in that institution. We model trust as expected usefulness, which enters the attention function directly and updates mainly when the information is actually consumed. That change allows us to use the model to derive comparative statics in search cost and information value, which our experiments test.

The remainder of the paper is structured as follows. Section 2 presents a model of rational inattention that formalizes our hypotheses and experimental design. Sections 3 and 4 describe the data, methodology, and results of Study I and II respectively. Section 5 concludes with suggested directions for future studies and policy implications.

# 2 Theoretical Framework

We ground our experimental design in a model of rational inattention (Sims, 2003), using the sparsity formulation of Gabaix (2014, 2019), which yields a closed form in which attention falls to exactly zero below a threshold. Citizens are attention-constrained: they face costs when searching for or processing information, and ignoring a source altogether can be optimal. An individual allocates attention $m_i \in [0, 1]$ to each attribute $x_i$ relevant to a decision, trading off the benefit of being better informed against the cost of acquiring and processing the information. That cost rises with a search-cost parameter $c_i > 0$, specific to each piece of information, and with an individual-level cognitive cost $\kappa > 0$.

We depart from the standard framework in one respect, which is central to our argument. We define *trust* in institution $j$ as $\theta_j \in [0, 1]$, the share of $j$'s work that the citizen expects to be useful to her[3]. This is a deliberately narrow, operational definition: it is closest to the competence dimension in Mayer et al. (1995), and it parallels the way beliefs about an information source's accuracy are modeled in previous studies (Gentzkow et al., 2025). Trust so defined enters the agent's problem directly, because an attribute the citizen expects to be useless is costless to ignore: the expected loss from inattention to an attribute produced by $j$ is scaled by $\theta_j$. The optimal attention devoted to attribute $x_i$ is then

$$m_i^* = A\left(\frac{\theta_j \, \sigma_i^2 |a_{x_i} u_{aa} a_{x_i}|}{c_i \kappa}\right), \qquad A(z) = \max\{0,\, 1 - 1/z\} \tag{1}$$

[3]Since trust is inherently a multi-dimensional belief, we use a validated set of questions to track it in our studies, as detailed in Section 3.

where $\sigma_i^2$ captures the variability of the information (how informative it is), $a_{x_i}$ how much a change in $x_i$ shifts the optimal action, and $u_{aa}$ the stakes of that action. Together these terms imply that an individual pays more attention to information that is more variable, more consequential for decisions, and less costly to acquire — and that comes from an institution she expects to be useful.

Because $A(\cdot)$ is zero below a threshold, the model has a corner solution. The citizen consults institution $j$ about attribute $i$ if and only if

$$\theta_j \;>\; \bar{\theta}_i \;\equiv\; \frac{\kappa c_i}{\sigma_i^2 |a_{x_i} u_{aa} a_{x_i}|}. \tag{2}$$

Low trust, high search costs, or information of little relevance to any decision can each push a citizen below the threshold and produce *zero* attention, not merely less of it. This corner is what generates a vicious cycle, because trust updates only upon consumption. In practice, being told what an institution produces is itself a cheap signal about the relevance of its output and can move trust without consultation; we return to this channel later. A citizen who consults $j$ observes whether the information was in fact useful and revises $\theta_j$ toward that experience; a citizen who does not consult $j$ observes nothing and leaves $\theta_j$ unchanged. Someone whose trust starts below $\bar{\theta}_i$ therefore never acquires the evidence that might revise it, and mistrust persists whether or not it was warranted in the first place. Further, the model allows consultation to reduce trust: a citizen who consults $j$ and finds the information of no use to her revises $\theta_j$ downward. Appendix A1.1 gives the full derivation, including the updating rule, and Appendix A1.1.8 works through a numerical example showing how the arms of our two experiments map onto the parameters of equation (1).

Our experiments intervene on the right-hand side of equation (2). Lowering the search cost $c_i$ (i.e., offering the link to check an answer in our experiments), lowers the threshold. Varying the decision-relevance $a_{x_i}$ of what lies behind the link (i.e., our between-treatment group variation in our second study) changes it as well. If the intervention brings citizens over the threshold, they consult, they learn whether the information was useful, and $\theta_j$ changes. This also clarifies what the model predicts for the low-value arm. Lowering $c_i$ for information with a small $a_{x_i}$ still brings some citizens over the threshold (fewer than for high-value information, since the threshold is higher) but what they find there is less likely to be of use to them, so trust rises less and can even fall.

One further distinction, which we use to interpret which dimension of trust moves in Study II, is worth making explicit. An attribute can fail to be useful for two reasons: because it bears on no decision the citizen takes, or because it is distorted, such as being reported in a way that serves the institution's political principals rather than the citizen. Perceived politicization is the citizen's belief about the second. The two failures are learned about in different ways. Relevance is largely known from the question itself: a respondent told that the EPA tracks air pollution in real time already knows whether that bears on her decisions. Distortion is not: it is learned by consulting a number that can be checked against the air outside, a weather application, or the local news, and finding it accurate. Consulting a verifiable, current figure therefore updates beliefs about distortion more than beliefs about relevance, and the model predicts that the T1–T3 contrast should move perceived political independence more than perceived benevolence, and should do so most for respondents in a position to verify the number. We stress that this reading is an interpretation offered after

seeing which outcome moved, not a prediction the design was built to test; Remark 3 in Appendix A1.1 states the decomposition formally.

## 2.1 Empirical Predictions

The model yields three testable predictions that map directly onto our experimental designs. Our first hypothesis is that lowering search costs will increase an individual's attention. While this hypothesis is consistent with previous studies on rational inattention, it has not been investigated in a context in which individuals may hold polarized beliefs about the institution providing such information. Would reducing search costs be enough to partly offset the information avoidance tendency driven by strongly held beliefs? Our first hypothesis thus reads as follows:

*H1: Reducing search costs ($c_i$) increases attention ($m_i^*$).*

Our second hypothesis is that increased attention leads to improved knowledge accuracy. This hypothesis also deserves to be tested empirically since it is unclear a priori whether citizens do in fact revise their factual knowledge as a result of consuming information from a public institution, or whether they discard it if not in line with their prior beliefs:

*H2: Reducing search costs ($c_i$) increases knowledge accuracy.*

Our third hypothesis is that after consuming institutional information, individuals will appreciate its usefulness for their decision-making and update their beliefs about institutional capabilities, thereby increasing their trust.

*H3: Consuming high-value information increases trust in the institution providing the information.*

Finally, the model focuses on individual-level attention costs and information value as determinants of trust. It abstracts from the macro-level forces behind the sharp aggregate decline in institutional trust we document in Section 3.2, including elite political attacks, partisan media narratives, and the pandemic shock. We view these as complementary rather than competing mechanisms: the former help explain why trust declined so sharply, while our framework motivates why it may be partly recoverable through targeted informational interventions. Our experiments test this more focused question: conditional on these political headwinds, can reducing attention costs still shift trust?

# 3 Study I: Search Costs, Information Acquisition, and Trust Beliefs

## 3.1 Data

Our first study focuses on Americans' trust in the U.S. Centers for Disease Control and Prevention (CDC). During the COVID-19 pandemic, the CDC became a central public institution, guiding the national response with health recommendations and policies. However, its role was frequently debated along political lines, with conflicting opinions on its guidance on lockdowns, mask mandates, and vaccines (see selected media extracts in Figure A2 in the Appendix). We partnered with NORC, the organization that administers the General Social Survey (GSS), using their AmeriSpeak Panel.[4] We recruited 1,440 U.S. citizens from the AmeriSpeak panel to participate in a seven-wave longitudinal survey conducted between April and October 2020 (see Table A8 in the Appendix for demographic and socioeconomic characteristics).[5] This longitudinal design allowed us to track shifts in beliefs over time, observe changes in the absence of an intervention for the control group, evaluate the long-term effects of our experimental intervention, and reduce the risk of social desirability bias. The first wave, conducted in early April 2020 as COVID-19 deaths surged across the U.S.,[6] gathered baseline data on trust in institutions, media consumption, and beliefs, as well as information on respondents' personal experiences with the crisis. The following three weekly waves tracked lived experiences during the critical first month of the pandemic. Wave 5, conducted the week of May 18, measured perceptions of the crisis's severity. Wave 6, administered on June 22, 2020, contained the randomized information provision intervention. The final wave, conducted the week of October 19, 2020, was timed to capture changes in beliefs and preferences four months after the intervention and just before the presidential election. While respondents were incentivized to complete all waves, 1,076 of the original 1,440 took part in the final wave, and 990 of these had also taken part in wave 6, when treatment was assigned; the October outcome regressions are estimated on these 990 respondents.[7] Attrition is correlated with several demographic characteristics: income, age, Hispanic ethnicity, cohabitation status, parenthood, and direct COVID-19 exposure. Importantly, attrition does not differ by political beliefs, news consumption, or treatment assignment (see Table A9 in the Appendix). We address selective attrition through NORC-provided survey weights, calibrated to population benchmarks to maintain

[4]Funded and operated by NORC at the University of Chicago, AmeriSpeak® is a probability-based multi-client household panel designed to be representative of the U.S. household population, covering approximately 97% of households. NORC places particular emphasis on recruiting underrepresented groups, including households without internet access, providing them with connectivity and tablets to enable participation.

[5]This was part of a larger study on COVID-19 that also collected data on respondents' mental health and well-being — topics unrelated to the present study — gathered in a way that did not interfere with our experimental design (Bertrand et al., 2020).

[6]See Figure A3 in the Appendix for the evolution of the COVID-19 death rate alongside our survey waves.

[7]The 86 wave-7 respondents who skipped wave 6 were never assigned to a treatment arm and are therefore excluded from the experimental analysis. Item non-response accounts for the remaining differences in sample size across outcomes (990 for perceived independence, 978 for the self-reported change in confidence, 977 for the knowledge ranking).

representativeness across key demographic dimensions, which we use throughout our analyses. We further examine whether treatment effects vary by the characteristics that predict attrition to assess whether selective retention biases our estimates.

## 3.2 Pre-experimental findings

We report here the descriptive results from the first five waves of our longitudinal survey. The most striking pattern is the rapid erosion of trust in public health institutions, specifically hospitals, healthcare professionals, and the scientific community, over just four weeks, with a similar decline observed for health insurance companies. Our repeated survey waves allowed us to track this erosion closely and confirm that trust did not return to pre-pandemic levels by the final wave about six months later (see Figure A4 in the Appendix). This decline in trust was closely linked to compliance. Table A10 in the Appendix shows a strong correlation between trust in healthcare institutions and self-reported adoption of preventive behaviors, both in levels and in trends. Respondents with higher trust in health institutions were more likely to follow recommended guidelines (i.e., handwashing, physical distancing, and mask-wearing) and those who increased their trust between the first and last week of April were also more likely to report adopting additional preventive behaviors over the same period. Notably, we do not observe this correlation with trust in government more broadly (the White House and the Federal Government), consistent with studies documenting how declining trust in specific institutions leads to suboptimal health behaviors (Martinez-Bravo and Stegmann, 2019). Partisan media consumption amplified these differences: when asked to project the number of additional COVID-19 deaths by year's end, respondents consuming left-leaning news estimated a figure twice as high as those consuming right-leaning news (Figure A5 and Table A12 in the Appendix). Taken together, these patterns document a rapid, politically polarized erosion of trust, exacerbated by slanted media consumption, that was associated with lower self-reported compliance with behaviors carrying significant negative externalities. This motivates our core research question: can scalable interventions improve individuals' understanding of the crisis, rebuild institutional trust, and increase their willingness to follow public health recommendations?

## 3.3 Experimental methodology

Our experiment tests whether mistrust in public institutions stems partly from citizens' limited familiarity with institutional work and their unwillingness to incur the search costs required to remedy this. The experiment reduces these costs and allows individuals to self-select into consuming institution-sourced information, simulating real-world settings where credible information is made readily accessible through public information campaigns or timely crisis communications. Respondents were asked a verifiable question about the COVID-19 death rate, a quantifiable, personally relevant measure of the crisis. Half were randomly assigned to a treatment group and offered a prompt encouraging them to look up the answer before responding: *"If you wish to do so, you can look up the answer on the official CDC website at the following link: https://www.cdc.gov/coronavirus/2019-ncov/cases-updates/ cases-in-us.html".* Respondents were first asked to estimate the

coronavirus death toll in their state on an eight-point categorical scale,[8] and then to estimate the nationwide death toll on a sliding scale from 0 to 200,000 in increments of 20,000.[9] The nationwide question was deliberately framed differently from the state-level question to minimize anchoring. Both questions appeared in the first block of questions in that wave to reduce survey fatigue. The link was spelled out in full so that respondents who distrusted the CDC could choose not to click it. Those who did were redirected to a new browser window to the CDC's national statistics page, where an interactive map allowed them to view state-level death counts by hovering over their state. We matched each respondent's answers to official CDC statistics from the day of survey completion to assess accuracy. These were the only questions about the current COVID-19 death rate across all seven waves, and the only occasion on which treated respondents were exposed to CDC-sourced information. This design allowed participants to voluntarily engage with the CDC — motivated by the opportunity to verify their own knowledge — without coercion and without financial incentives, preserving the naturalistic character of information acquisition.[10][11]

### 3.3.1 Addressing Experimenter Demand and Messenger Effects

A central concern in information-provision experiments is whether treatment effects reflect genuine belief updating or experimenter demand, where respondents infer the researcher's objectives and adjust answers accordingly. Khan et al. (2021) show that modest positive effects of an information campaign in Pakistan were largely attributable to experimenter demand, underscoring the importance of accounting for this mechanism. We incorporated three features to mitigate this risk. First, we introduced a four-month delay between the information treatment (June 2020) and our primary trust outcomes (October 2020). This temporal separation reduces the salience of the treatment when outcomes are measured. Second, we employed an obfuscated measurement strategy (Haaland et al., 2023). Trust in the CDC was measured four months after treatment, embedded within a battery of seven institutions presented in random order (i.e., the scientific community, banks, hospitals, health insurance companies, the police force, the private sector, and the CDC), making it difficult for respondents to identify it as the institution of interest. The follow-up knowledge question asked respondents to rank the U.S. relative to other countries in COVID-19 deaths per capita, a question that could not be answered using the original CDC link, further obscuring the connection to the initial intervention. Third, we concluded the treatment

[8] "*How many people have died in your state because of coronavirus from the first death until today?*" Response options: *Less than 500; Between 500 and 1,000; Between 1,000 and 3,000; Between 3,000 and 5,000; Between 5,000 and 10,000; Between 10,000 and 15,000; Between 15,000 and 30,000; More than 30,000.*

[9] "*How many people have died in the U.S. because of coronavirus from the first death until today?*"

[10] Some studies advocate for incentive-compatible mechanisms to elicit accurate knowledge. We opted against financial rewards to maintain the voluntary nature of engagement. See Belot and Briscese (2023) on the importance of non-incentivized self-selection into contact.

[11] A technical error meant the pre-analysis plan for this study was registered after trial launch. To mitigate this and to strengthen the reliability of findings from the first experiment, we pre-registered Study II in full and applied the same procedures to both studies (i.e., detailed analysis plan, identical procedures, decision rules, and analytical methods.)

wave with questions about the George Floyd protests and the Black Lives Matter movement, a politically salient topic unrelated to the pandemic, so that respondents' final impression of the wave was not focused on COVID-19 content. Further, the intervening period coincided with the 2020 Presidential election campaign, keeping respondents' attention focused on politically polarizing events unrelated to our study.

**Outcomes and empirical model.** We track four outcomes: (i) engagement with the treatment; (ii) response accuracy; (iii) trust in public institutions; and (iv) behavioral change. In the treatment wave, we tracked only the first two and deliberately avoided trust questions to prevent anchoring or priming. In wave 7 (October 2020), we measured institutional trust through perceived politicization,[12] which we view as a primary determinant of institutional trust: when citizens perceive an institution as highly politicized, they question its objectivity and ability to serve the public interest without bias (Braithwaite and Levi, 1998). Perceived politicization was measured across the same seven institutions as above, presented in random order. After a set of unrelated questions to minimize consistency effects, respondents were asked to reflect on changes in their trust beliefs since before the pandemic.[13] This meta-cognitive measure captures respondents' change in trust, along with awareness of their own belief updating. For behavioral outcomes, we elicited self-reported compliance with CDC-recommended practices, focusing on past behaviors rather than intentions to reduce social desirability bias. We asked whether respondents had received a COVID-19 test, whether they had tested positive (to assess whether increased testing reflected higher virus exposure rather than compliance), and which preventive actions they regularly adopted.[14]

**Moderators and covariates.** Our key moderator is media diet, which we argue shapes institutional trust by reinforcing the ideological cues that behaviorally inattentive individuals rely on when assessing unfamiliar institutions. We constructed a media slant index following the methodology described in Appendix A4.0.2. Respondents who consume slanted media exhibit different pandemic perceptions, lower knowledge accuracy, and distinct trajectories of institutional trust over the course of the crisis. We also collected four measures of pandemic exposure that serve as additional controls: (i) direct impact, either financial (a month-on-month drop of more than 20% in household income since the start of the pandemic) or health-related (hospitalization of the respondent or someone close to them with COVID-19); (ii) indirect impact, economic (weekly variation in consumer expenditures in the respondent's county) or health-related (log of the COVID-19 case counts in the respondent's county). Construction details are in Appendix A4.0.2. We estimate a standard OLS regression with

[12] "*Currently, how independent versus politicized do you think the following institutions are?*" Response options: (1) Not influenced by politics at all; (2) Not too influenced by politics; (3) Somewhat influenced by politics; (4) Very influenced by politics.

[13] "*Thinking about your confidence in the institutions before the COVID-19 crisis started and today, to what extent would you say your views have changed?*" Response options: (1) I have a worse opinion than before; (2) My opinion did not change; (3) I have a better opinion than before. The institution list included the same seven institutions, plus the President, and the U.S. Senate and Congress.

[14] Options: (1) I do not leave home unless strictly needed; (2) I keep a safe distance from others; (3) I try to avoid touching objects when outside; (4) I wash my hands thoroughly and frequently; (5) I cover my mouth in public with a mask or scarf; (6) None of the above.

survey weights to maintain representativeness despite attrition:

$$\text{outcome}_{izw} = \alpha + \beta\ \textit{Treated}_i + \underline{\gamma}\ \textit{Covid19}_{izw} + \underline{\theta}\ \textit{News}_i + \underline{\psi}\ \textit{Demographics}_i + \epsilon_{izw} \quad (3)$$

where outcomes are measured in wave $w$ for individual $i$ living in zip code $z$. Standard errors are robust to heteroskedasticity and clustered at the primary sampling unit to account for the survey design.

## 3.4 Results

### 3.4.1 Knowledge Accuracy and Trust

Engagement with the knowledge question was significantly higher in the treatment group than the control group — a 20% increase from a baseline of just under one minute, as tracked by a hidden embedded timer (Table A14 in the Appendix), providing support for H1.[15] This additional time likely reflects respondents clicking on the CDC link, consistent with equation (1): reducing search costs $c_i$ increased attention $m^*$ to a high-variance piece of information — the COVID-19 death rate, arguably the most salient indicator of the pandemic's severity — that could inform respondents' decisions. We use a stringent accuracy measure, flagging as correct only responses where both the national and state death estimates matched CDC figures. In line with our second hypothesis, Table 1 shows that the treatment increased the probability of a correct answer by 10 percentage points (35% relative to a baseline of 29%). In the control group, 24% underestimated and 26% overestimated state deaths; in the treatment group, these shares fell to 16% and 19%, respectively. The pattern is similar for national deaths: underestimates fell from 23% to 19% and overestimates from 26% to 21%. The treatment also had lasting effects on knowledge: treated respondents were 10% more likely to correctly rank the U.S. in terms of COVID-19 deaths per capita four months later. Turning to trust, the treatment significantly increased the perception that the CDC was politically independent by 6 percentage points (control-group mean: 8.0%), and treated respondents were 6.2 percentage points more likely to report that their confidence in the CDC had increased since the start of the pandemic (control-group mean: 12.3%), supporting our third hypothesis. This is the comparison in our design that most directly tests whether making institutional information easier to consult raises trust. Both arms answered an identical question; they differed only in whether a link to the CDC's data was offered. Treated respondents did see the CDC named in the prompt. However, what the prompt conveyed about the agency — that it publishes national death counts — was widely known in June 2020, unlike the EPA's real-time local monitoring in Study II. Being reminded of a familiar function is therefore a less plausible explanation for trust effects measured four months later, among outcomes on seven institutions presented in random order, of which only the CDC and the closely related scientific community moved (Table A19). An instrumental variable approach is consistent with these effects operating through knowledge accuracy: respondents who correctly estimated COVID-19 deaths were

[15] At the time of implementation, NORC did not have the capability to track click-through rates, which we do in our second experiment.

substantially more likely to report higher trust in the CDC (Table A15 in the Appendix). The first stage is strong when conditioning on demographics (F = 13.5), though weaker without controls (F = 6.24). Because Table 1 tests four outcomes, Appendix A3 reports inference adjusted for multiple testing. The June knowledge effect survives the adjustment. With controls, the two trust effects have sharpened false-discovery-rate q-values of 0.04 within the family of four outcomes, and the long-run knowledge effect a q-value of 0.05, at the margin, so we treat it as suggestive (Table A6, which also reports the more conservative Romano-Wolf family-wise p-values).

Table 1: Treatment effects on knowledge in the short- and long-run, and on trust beliefs.

| | June (w6) | | October (w7) | | | | | |
|---|---|---|---|---|---|---|---|---|
| VARIABLES | (1) Correct death est. | (2) Correct death est. | (3) Correct US death rank | (4) Correct US death rank | (5) Incr. trust in CDC | (6) Incr. trust in CDC | (7) CDC not infl. by politics | (8) CDC not infl. by politics |
| Informative T | 0.101*** | 0.125*** | 0.0406 | 0.0539* | 0.0637* | 0.0618** | 0.0545* | 0.0599** |
| | (0.0324) | (0.0286) | (0.0336) | (0.0305) | (0.0325) | (0.0292) | (0.0301) | (0.0269) |
| Constant | 0.290*** | 0.334** | 0.528*** | 1.039** | 0.123*** | 0.391 | 0.0795*** | -0.0399 |
| | (0.0217) | (0.143) | (0.0294) | (0.443) | (0.0176) | (0.281) | (0.0194) | (0.188) |
| Observations | 1,147 | 1,147 | 977 | 977 | 978 | 978 | 990 | 990 |
| R-squared | 0.022 | 0.161 | 0.002 | 0.082 | 0.014 | 0.088 | 0.021 | 0.136 |
| Demographics | No | Yes | No | Yes | No | Yes | No | Yes |
| Covid shock | No | Yes | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 0.330 | 0.330 | 0.550 | 0.550 | 0.162 | 0.162 | 0.113 | 0.113 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1. OLS coefficients. Cols (1–2): correct estimation of COVID-19 deaths in the U.S. and respondent's state, June; cols (3–4): correct ranking of the U.S. by COVID-19 deaths per capita, October; cols (5–6): self-reported increase in confidence in the CDC since the start of the pandemic, October; cols (7–8): CDC perceived as not influenced by politics, October. All regressions use population survey weights. Control variables are described in the Appendix.

**Meta-cognition.** A potential concern with survey experiments is that self-reported changes in trust may not reflect actual belief updating. To assess the validity of this meta-cognitive measure, we examine whether self-reported changes predict observed variations in trust across a set of institutions tracked between April and October. We find that self-reported changes predict measured changes positively and significantly for four of five institutions, indicating that respondents' reports carry real information about how their trust changed (Tables A27 and A28 in the Appendix). This is consistent with prior evidence that self-reported trust captures meaningful underlying beliefs (Sofianos, 2022), and extends it by showing that meta-cognitive measures are informative not only about levels, but also about changes. More generally, meta-cognitive measures can serve as a useful complement for tracking within-individual belief changes when panel data are limited.

**Prior beliefs and who searches.** This analysis provides the most direct evidence in either study on the attention mechanism of Section 2, because it holds the question fixed and asks who acquires the information when it is costly to do so. News consumption affects the beliefs a respondent brings to the question, and trust in the scientific community may raise or lower the perceived cost of engaging with the institution. Both predict knowledge accuracy strongly. Respondents who follow U.S. news for at least thirty minutes a day are 9.7 to 12.4 percentage points more likely to answer correctly, against a 39% base rate, while

those reporting at most "some confidence" in the scientific community in wave 1, two months before the experiment, and so pre-treatment, are 7.2 to 12.2 points less likely to do so (Table A2 in the Appendix). The gap in the time respondents spend searching appears only where searching was costly. In the control group, which faced the higher search cost, low-trust respondents spent 19.3 fewer seconds on the question; among the treated, who were offered the link, the gap disappears, and the interaction is 31.8 seconds with a standard error of 13.7 ($p = 0.022$). This is what the model predicts: when search is costly, only those who find it cheap — the trusting and the news-attentive — search; lowering the cost brings in those who otherwise would not. Appendix A2 reports the full results, including the parallel evidence from Study II.

### 3.4.2 Behaviors

We hypothesized that rebuilding institutional trust would increase compliance with public health guidelines, in line with previous studies documenting the effect in the opposite direction (Tyler, 1990; Braithwaite and Levi, 1998). The evidence here is positive but more mixed (Table A16 in the Appendix). Treated respondents were 20% more likely to have taken a COVID-19 nasal swab test (6.6 percentage points, from 28.1% in the control group). This effect was not driven by a higher rate of positive tests (Table A18 in the Appendix), suggesting the increase reflects greater compliance with public health recommendations rather than illness. Adjusted for the four behavioural outcomes tested together, the testing effect has a sharpened q-value of 0.06 (Table A6 in the Appendix, which also reports the more conservative Romano-Wolf family-wise p-values), which is why we describe the behavioural evidence as suggestive. Whether driven by improved knowledge of the pandemic's severity or by higher trust in the CDC, respondents acted in line with institutional guidance. For other preventive behaviors, the treatment increased the probability of adopting at least one recommended behavior, though this effect is concentrated among a small number of individuals and should be interpreted cautiously. By October 2020, most pandemic restrictions had been lifted, and masks were no longer mandatory, which likely diminished the relevance of these measures and explains the absence of detectable effects. Of the behaviors still actively recommended at that time, COVID-19 testing was the most consequential for limiting viral spread, making it the most meaningful behavioral outcome to shift. An IV regression analogous to the one above suggests the effects could be driven by knowledge accuracy, though TSLS coefficients are significant only with demographic controls (Table A17 in the Appendix).

### 3.4.3 Robustness Checks and Alternative Mechanisms

We summarize here the additional analyses reported in full in the Appendix.

**Placebo outcomes.** We measured trust across seven institutions that were shown in random order to survey respondents and found no significant treatment effects on any institution other than the CDC (Table A19 in the Appendix), confirming that the trust improvement was targeted rather than a broad attitudinal shift. The only exception is a modest spillover to the scientific community, which is unsurprising given the CDC's identity

as a scientific institution and suggests that trust gains can extend to closely related credible institutions.

**Attrition.** Approximately 75% of respondents completed the final survey wave, with an attrition rate of 17% between the last two waves. All analyses use survey weights that adjust for attrition to preserve population representativeness. Nonetheless, attrition between the experimental and outcome waves could bias estimates if it differs systematically by treatment status. We rule out this risk in three ways. First, attrition rates do not differ significantly across treatment arms, either unconditionally ($p = 0.84$) or conditional on baseline covariates ($p = 0.94$). Second, although baseline covariates jointly predict attrition (F-test = 5.56, $p < 0.001$), consistent with selective panel retention in survey data, we find no evidence that treatment assignment alters these patterns (joint F-test of treatment $\times$ covariate interactions = 1.02, $p = 0.42$). Third, treatment effects generally do not vary with characteristics that predict attrition (Tables A20-A23 in the Appendix). The only exception is the perceived political independence of the CDC, where estimated effects are larger among Hispanic respondents (N=133; treatment effect: 0.15, $p < 0.05$) than among non-Hispanic respondents (0.034, $p = 0.2$). This pattern does not extend to other trust or behavioral outcomes, and the Hispanic subsample is relatively small. Notably, treatment reduced attrition among Hispanic respondents by 9 percentage points, suggesting the intervention may have been particularly engaging for this group. Together, weighting, balance tests, and the absence of treatment–covariate interactions in attrition suggest that any selective panel retention is orthogonal to treatment assignment, reducing concerns that differential selection into the outcome sample biases the estimated treatment effects.

**Other trust outcomes.** Controlling for baseline interpersonal trust reported in wave 1 leaves the treatment effect unchanged (Table A24 in the Appendix), ruling out the possibility that our results are driven by generalized social trust. In line with the literature (Bolsen and Druckman, 2018; McLaughlin et al., 2021; Van Scoy et al., 2021; Clark et al., 2023), trust erosion tracked closely with perceived politicization. Respondents who believed an institution was affected by politics reported lower confidence in it relative to the first wave and were more likely to report having decreased their trust over time (Table A11 in the Appendix).

**Heterogeneity.** We find no evidence that our effects are driven by particular subgroups or heterogeneity by media consumption, political affiliation, education, COVID-19 exposure, or baseline trust in science. The one interaction on a trust outcome that approaches conventional significance is that between treatment and Republican-leaning news consumption on perceived political independence, which is negative ($-0.099$, s.e. 0.060, $p = 0.10$; Table A29 in the Appendix), so that the point estimate of the effect for this group is close to zero. The interactions for Republican- and Democratic-leaning consumers cannot be distinguished from each other ($p = 0.24$), and no comparable pattern appears for self-reported confidence, so we read it as suggestive at most. Significant heterogeneity emerges only for long-term knowledge accuracy: the treatment effect is 26 percentage points larger ($p < 0.001$) among Republican-leaning news consumers than Democratic-leaning ones and 11 percentage points larger ($p < 0.1$) among respondents without a college degree,

both groups exhibiting substantially lower baseline knowledge in the control condition. The absence of robust partisan heterogeneity in trust is notable given evidence that individuals often reject information based on motivated reasoning. Several features of our design may help explain this pattern. The intervention delivered factual, verifiable metrics rather than normative policy recommendations and was administered through NORC, a nonpartisan survey organization, potentially reducing resistance at the point of intake. In addition, the near-zero search costs embedded in the intervention may have limited opportunities for motivated avoidance. While we cannot distinguish empirically among these mechanisms, the results highlight the importance of informational design in shaping the effectiveness of institutional communication.

Complementary evidence from the control group is consistent with the search-cost mechanism emphasized in our model. Among untreated respondents, those consuming Republican-leaning media who report lower baseline trust in the scientific community spent significantly less time on the knowledge question than those consuming Democratic-leaning media ($p = 0.08$ without controls and $p = 0.02$ with them; Table A30 in the Appendix). This pattern suggests less voluntary engagement with institutional information among lower-trust individuals. The corresponding association with knowledge accuracy is directionally similar but not robust to covariate adjustment, likely reflecting confounding by socioeconomic characteristics. Taken together, these results are consistent with a framework in which trust facilitates voluntary information acquisition, whereas the experimental reduction in search costs attenuates reliance on such trust-driven engagement.

**Alternative specifications.** Results are robust to a probit model (Table A25). Examining the reverse direction of our outcomes, that is whether treatment reduces the likelihood of decreased confidence or increased perceived politicization, confirms the main results and rules out outlier-driven effects (Table A26, cols 1-4). Treating perceived politicization as a four-point scale rather than a binary outcome yields consistent results (Table A26, cols 5-6).

# 4 Study II: The Importance of Information Value

Our first study showed that reducing search costs for institution-sourced information encourages citizens to engage with it, update their factual knowledge, and increase trust, with suggestive behavioral improvements. Because the question was held fixed, that study speaks to whether making institutional information easier to consult raises trust. It does not tell us whether the kind of information matters. A second study provides evidence on this complementary margin: does increased trust arise from mere exposure to institutional communication, or does the value of the information offered matter?

## 4.1 Data

We partnered again with NORC to administer a survey experiment to the GSS AmeriSpeak panel in August 2024, four years after our original study.[16] We recruited 1,630 respondents

[16]For this study, we leveraged the Time-sharing Experiments for the Social Sciences (TESS) initiative. TESS is an NSF-funded initiative that fields survey experiments on NORC's AmeriSpeak Panel at no cost

and implemented a three-arm design that replicates the key features of the first study while introducing variation in information value to isolate the mechanism. We intentionally timed this experiment ahead of the 2024 Presidential elections to mirror the politically polarized context of our first study, and to examine whether the same intervention produces similar effects outside a public health crisis and across a different institution. We chose to focus on trust in the U.S. Environmental Protection Agency (EPA). EPA policies directly affect Americans' daily lives in ways that are personally salient, unlike institutions such as NASA or NOAA, whose mandates are less immediately felt, and compliance with EPA-recommended behaviors (e.g., reducing one's carbon footprint) generates positive welfare spillovers comparable to following CDC guidance during the pandemic. Like the CDC in 2020, the EPA was subject to significant politically polarizing attacks at the time of our study, with Republican leadership calling for the resignation of the EPA's administrator and seeking a Supreme Court injunction to block new environmental protection rules, making it a suitable institutional context for our complementary study.

The survey comprised six questions. The first assessed respondents' baseline familiarity with the EPA,[17] allowing us to test whether, consistent with our behavioral inattention hypothesis, unfamiliarity correlates with lower baseline trust. The second introduced our randomized intervention (detailed below). Questions three and four measured two dimensions of trust: perceived benevolence[18] and perceived politicization,[19] both elicited on 7-point Likert scales following Stantcheva (2023), from 1 ("Always worsen" / "Completely politicized") to 7 ("Always improve" / "Completely independent"), with 4 as the neutral midpoint. The same questions were asked about the IRS as a placebo. Question five elicited respondents' support for EPA involvement in a hypothetical community research scenario.[20] Since government agencies cannot receive donations, this willingness-to-recommend question serves as our behavioral proxy for institutional support. The sixth and final question asked about preferred news sources, enabling us to construct a media slant index using the same methodology as the first study.[21]

---

to researchers, though it limits investigators to six items per study.

[17] *"Look at the list of government agencies below and select the ones that you are familiar with — that is, the ones you know something about their mission, role, and responsibilities."* Respondents chose from a randomized list including: EPA, CDC, NOAA, National Park Service, FDA, IRS, Federal Laboratory Consortium for Technology Transfer, and Open World Leadership Center.

[18] *"Do you think the people running the following institutions recommend policies and regulations that improve or worsen your life and the lives of your loved ones?"*

[19] *"How politicized versus independent do you think the following institutions are?"*

[20] *"Imagine your state is planning a study to measure air pollution levels in our community. They are asking residents like you which organization they trust most to lead this important project. What is the percent chance on a scale from 0% (absolutely no chance) to 100% (absolutely certain) that you would recommend the Environmental Protection Agency (EPA)?"* We use Wiswall and Zafar (2015)'s approach to aid interpretation of percentage responses, providing illustrative anchors: e.g., 2–5% as "almost no chance," 47–52% as "pretty even chance," 95–98% as "almost certain."

[21] After the last question, all respondents were shown links to the EPA's Facebook, X and Instagram pages and invited to follow the agency; whether they clicked any of them was a pre-registered secondary outcome, intended as a low-cost measure of willingness to keep receiving the institution's information. We find nothing on it: 9.3% of respondents clicked a link, the share is not higher in T1 than in T3 (1.5 percentage points, s.e. 1.8), and among the two link arms it does not differ between respondents who did and did not open the treatment link (−1.0 points, s.e. 2.0). The one difference across arms is that T2, which received no link,

## 4.2 Experimental Methodology

Regardless of prior EPA familiarity, all respondents first received a brief description of the agency.[22] This provided a common baseline understanding of the EPA's mission for all participants. We then tested factual knowledge using a three-arm randomization. In both treatment arms (T1 and T2), respondents were shown a two-sentence description of the Air Quality Index (AQI) - the EPA's real-time measure of air pollution — and asked to report the current AQI in their zip code using a slider from 0 to 500.[23] In T1 (low search cost, high-value information), respondents were additionally offered a direct link to the EPA's air quality tracker, mirroring the design of the first experiment. The control group (T2) received the AQI question without a link, facing higher search costs. In T3 (low search cost, low-value information), respondents were instead asked about the location of the EPA regional office responsible for their state, with an equivalent link provided.[24] We chose this treatment because citizens may value physical proximity of an EPA office (e.g., greater appreciation of community needs, faster response time in an emergency), but it is arguably a less decision-relevant piece of information. This design thus allows us to vary two parameters of equation (1) independently: the search cost $c_i$ (T1 vs. T2) and the informational value $x_i$ (T1 vs. T3), while holding the institutional source constant across all arms. Crucially, because all respondents are exposed to information about the same institution, differences in outcomes are unlikely to reflect messenger endorsement or general experimenter demand. They reflect the content of the information task (i.e., what respondents are told the EPA does and what they find if they consult it). As in the first experiment, respondents were not incentivized to consult the EPA website and could freely choose whether to do so. Respondents could also leave the factual question blank, and 14% did: 3% in T2, against 20% in T1 and 19% in T3, where non-response is concentrated among those who opened the link (28 and 34% of clickers, against 6 and 2% of non-clickers), possibly because the link opened in a new window and some respondents did not return to the question. This is why the knowledge columns of Table 2 have 1,400 observations and the trust columns about 1,600; because leaving the question blank is a post-treatment choice, the trust outcomes are estimated on the full sample rather than conditioned on answering, and the T1–T3 trust effects are, if anything, slightly larger among those who answered. Our measure of information consumption, the attention $m_i^*$ of the model in Section 2, is whether the respondent answered the factual question correctly.

We record whether respondents opened the link, but a click is a weak measure of consumption: we cannot observe what happens after the page opens, and a respondent may close it immediately. Answering correctly, by contrast, requires having actually taken in the

clicked 4.3 points more often than T3 ($p = 0.01$), a contrast the design gives us no reason to expect and which is not distinguishable from T1 ($p = 0.12$); we do not interpret it. Table A36 in the Appendix reports the estimates.

[22] *"The U.S. Environmental Protection Agency (EPA) is the federal government agency responsible for implementing policies that protect human health and the environment, such as reducing emissions."*

[23] *"EPA also manages the NowCast Air Quality Index (AQI), which tracks levels of air pollution in real time across the country. The higher the NowCast AQI score, the worse the air quality and the greater the health concern. To the best of your knowledge, what is the current level of pollution in your ZIP code?"*

[24] *"EPA has multiple regional offices to address health and environmental concerns in specific areas of the country, including where you live. To the best of your knowledge, in which city is the EPA regional office that is responsible for your state of residence?"* (see Figure A8)

number, whether from the EPA's page or elsewhere, so we use it as the primary measure throughout and treat clicking as a supplementary one. We verified response accuracy by merging respondents' zip codes and survey completion times with publicly available EPA AirNow data, and cross-referenced T3 responses with EPA regional office assignments by state. Because AirNow displays the AQI of whichever pollutant is highest rather than PM2.5 specifically, we score respondents against the headline AQI for their reporting area, and we convert survey timestamps to GMT before matching. Our pre-analysis plan specified a ±10 window. We report ±5 in the paper because it allows for a tighter match, and the ±10 in the Appendix . In Table A33 in Appendix, we replicate the analysis using a ±10 rule and show that neither the search-cost result nor any trust outcome depends on this choice.[25] We estimate a standard OLS regression for individual $i$ in zip code $z$, excluding T3 so that the T1 coefficient captures the effect of low-cost, high-value information relative to low-cost, low-value information:

$$\text{outcome}_{iz} = \alpha + \beta_1 T1_i + \beta_2 T2_i + \underline{\theta}\, News_i + \underline{\psi}\, Demographics_i + \epsilon_{iz} \tag{4}$$

Standard errors in Table 2 are classical; the Appendix tables state where heteroskedasticity-robust standard errors are used instead.

## 4.3 Results

We examine two dimensions of trust, perceived benevolence and political independence, and assess whether search costs and information value shape them differently. Consistent with H2 from Section 2, both low-search-cost conditions (T1 and T3) yield significantly higher knowledge accuracy than the no-link condition (T2), which reduces accuracy by approximately 30 percentage points relative to T3 (cols. 1–2 of Table 2) and by 29 points relative to T1 (p-val T1 = T2, Table 2). At the ±5 window, the two link arms answer their respective questions correctly at the same rate, 46% in T1 and 48% in T3. At the pre-registered ±10 window, T1 is more accurate (62% against 48%; Table A33 in the Appendix), because widening the window raises accuracy on the air quality question but cannot change it on the office-location question. Respondents in T1 are also 12 percentage points more likely to click on the provided link than those in T3 ($p < 0.001$; Table A35 in the Appendix),[26]

[25] AirNow's hourly monitor files report NowCast values: recomputing the AirNow PM NowCast from the reported hourly concentrations reproduces the published PM2.5 index in 97.9% of 318,242 monitor-hours. For each reporting area and hour we take the maximum across monitors of each pollutant's index and then the maximum across pollutants, which is the pollutant AirNow flags as primary in every reporting area we can check against its own reporting-area files. In our field window PM2.5 is the headline pollutant in only 62% of area-hours, with ozone accounting for a further 33% and rising to about half in the late afternoon. Survey timestamps are recorded in a single fixed United States zone, which the survey documentation does not identify; we select the offset empirically (UTC−4), which maximises agreement with the answers of respondents known to have opened the AirNow page. Among those respondents the benchmark reproduces the reported value exactly in 18.4% of cases and to within ±10 in 74.8%, against 6.0 and 58.0% under a PM2.5-only match using unconverted timestamps. Because AirNow publishes reporting-area values through a separate agency-reported process, a reconstruction from the monitor files is close to but not identical with the displayed number, so the exact-match rate is a lower bound on the share of respondents who consulted the page.

[26] We do not compare time spent across treatment arms, as respondents completed different tasks.

so more informative cues attract greater attention even when the cost of reaching them is identical.

Turning to trust, we follow the pre-analysis plan and binarise each outcome: agreeing that the EPA improves lives and that it is politically independent (above the midpoint of the 7-point scale), and reporting a likelihood above 50 of recommending the EPA to lead a community study. Providing access to local AQI data raises the probability that respondents view the EPA as politically independent by 6.2 percentage points relative to the office-location link ($p = 0.013$), a 34% increase on the T3 base of 18.0%, and by 4.6 points with the full control set ($p = 0.063$; cols. 5–6). It also raises the share who agree that the EPA improves lives by 5.0 points (cols. 3–4) and the share who would back the EPA to lead a study by 6.3 points (cols. 7–8); both are significant without controls and attenuate with them. Because three trust outcomes are tested at once, Appendix A3 reports adjusted inference: without controls the independence and support effects both have sharpened false-discovery-rate q-values of 0.04; with the full control set the q-values of all three outcomes rise to 0.18 (Table A7, which also reports the more conservative Romano-Wolf family-wise p-values), so we read the controlled trust effects as suggestive. The same outcomes on their original scales give the same pattern (Table A32 in the Appendix). The T1–T3 contrast is the central test of H3 (Sec. 2) in our design, and it is worth being explicit about why. Both arms are given a link to the same agency, in the same format, at the same point in the survey, and both therefore face the same search cost $c_i$, but they differ in the information task - that is, the question asked and the page it links to. The contrast therefore identifies the effect of offering decision-relevant rather than low-value information, holding the institution, the search cost and the format of the prompt fixed. It rules out explanations based on the cost of reaching the information, the identity of the messenger, or a general willingness to comply with the survey. Our preferred interpretation is that real-time air quality data signals greater technical rigor and accountability in EPA operations, strengthening confidence in the agency. That the effect is concentrated on perceived independence rather than benevolence is consistent with the distinction between relevance and distortion drawn at the end of Section 2, although, as the T2 results below show, part of this signal may be conveyed by the question itself.

Table 2: Results on trust outcomes.

| | (1) Knowledge accuracy (±5) | (2) Knowledge accuracy (±5) | (3) EPA improves lives (0/1) | (4) EPA improves lives (0/1) | (5) EPA independent (0/1) | (6) EPA independent (0/1) | (7) Support for EPA (0/1) | (8) Support for EPA (0/1) |
|---|---|---|---|---|---|---|---|---|
| T1: Low search costs | -0.0187 | -0.0276 | 0.0498* | 0.0233 | 0.0615** | 0.0456* | 0.0634** | 0.0478 |
| & high value info | (0.0311) | (0.0312) | (0.0296) | (0.0286) | (0.0248) | (0.0245) | (0.0300) | (0.0291) |
| T2: High search costs | -0.305*** | -0.300*** | 0.0177 | 0.0117 | 0.0335 | 0.0252 | 0.0393 | 0.0331 |
| & high value info | (0.0292) | (0.0293) | (0.0292) | (0.0280) | (0.0244) | (0.0240) | (0.0296) | (0.0287) |
| Constant | 0.487*** | 0.465*** | 0.596*** | 0.394*** | 0.180*** | 0.237*** | 0.591*** | 0.607*** |
| | (0.0216) | (0.0935) | (0.0207) | (0.0871) | (0.0174) | (0.0749) | (0.0210) | (0.0895) |
| Observations | 1,400 | 1,400 | 1,626 | 1,626 | 1,613 | 1,613 | 1,580 | 1,580 |
| R-squared | 0.097 | 0.127 | 0.015 | 0.117 | 0.006 | 0.067 | 0.004 | 0.094 |
| Demographics | No | Yes | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes | No | Yes |
| p-val T1 = T2 | 0.000 | 0.000 | 0.280 | 0.686 | 0.260 | 0.406 | 0.426 | 0.616 |
| Mean dep. var. | 0.356 | 0.356 | 0.609 | 0.609 | 0.207 | 0.207 | 0.622 | 0.622 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1. OLS coefficients. Excluded group: low search cost, low-value information (T3). Cols (1–2): knowledge accuracy, scored to within ±5 of the headline AQI (T1, T2) or as the correct regional-office city (T3); the pre-registered ±10 window is in Table A33 in the Appendix; cols (3–8): trust outcomes binarised as specified in the pre-analysis plan — agrees the EPA improves lives (cols 3–4) and that it is politically independent (cols 5–6), both above the midpoint of the 7-point scale, and reports a likelihood above 50 of recommending the EPA to lead a community air quality study (cols 7–8). The same outcomes on their original scales are in Table A32 in the Appendix. Standard errors and the p-val row are classical throughout. Controls are described in the Appendix.

The p-values in the second-to-last row show that T1 and T2 never differ significantly in trust outcomes, despite T1 producing substantially higher knowledge accuracy.[27] Two complementary explanations account for this pattern.

First, the T1–T2 contrast has limited power to detect a trust effect operating through consultation. By equation 16 in Appendix, the intention-to-treat effect of the link is the change in the share who consult times the update among those who do. With a change in accuracy of 29 points, a per-consumer update of 10 percentage points would produce an intention-to-treat effect of about 3 points. The observed T1–T2 difference in perceived independence is 2.8 points, with a standard error of roughly 2.5. Detecting an effect with 80% power would require a true effect of about 7 points, which implies an implausibly large per-consumer update of about 24 points. The null is therefore uninformative about the consultation channel rather than evidence against it.

Second, the AQI question itself — by explicitly referencing EPA's real-time pollution monitoring — may have signaled institutional competence and relevance to all high-value information respondents, regardless of whether the link was provided.

The data is consistent with this: on the original 7-point scale, T2 respondents, who received no link, already rate the EPA 0.16 points more independent than T3 respondents, compared with 0.20 to 0.24 points for T1 (Table A32). Between two-thirds and four-fifths of the T1 effect on this scale is therefore present without the link. This interpretation is supported by the T3 results: when the question referenced lower-value information (office locations), trust effects were smaller or null even with a link, consistent with the information content itself — not merely the link — driving the trust response. Study II thus establishes that the value of the information an institution offers shapes trust, holding the source and the search cost fixed, but it cannot separate learning from the prompt from learning by consulting the page. Study I, in which the question was identical across arms and only

[27] Our pre-analysis plan specified the comparison with the no-link arm (T2) as primary and the T1–T3 comparison as secondary. We report both in full. We discuss the T1–T3 contrast at greater length because it is the only comparison that holds the search cost fixed while varying information value, not because the primary contrast is null.

the search cost varied, provides cleaner evidence on the consultation channel. For the design of information campaigns, both channels carry the same implication: campaigns that foreground information useful to citizens raise trust, while those that only increase the institution's visibility are less likely to do so. The model allows consulting low-value information to lower trust (Remark 2 in the Appendix), but we find no evidence of such a backfire: trust in T3 is below T2 on all three outcomes, by 1.8 to 3.9 percentage points, but never significantly (Table 2), and within T3 respondents who opened the office-location link are not significantly less trusting on any outcome than those who did not (Table 3, row *Clicked link*). Because T2 respondents also saw the air quality question, the T2–T3 gap cannot by itself separate a negative effect of the office link from a positive effect of the question.

#### 4.3.1 Mechanisms: Engagement, Accuracy, and Trust

A natural concern with the T1–T3 comparison is that respondents in T1 might trust the EPA more simply because more of them answered correctly, so that the trust effect reflects knowledge rather than the value of the information consulted. At the ±5 window used in the main text this cannot be the explanation: T1 and T3 answer their respective questions correctly at indistinguishable rates (46 and 48%; Table 2, cols. 1–2). At the pre-registered ±10 window T1 is more accurate (Table A33), so we do not rest the argument on equal accuracy alone. Two further facts suggest that knowledge is not what drives the trust effect: the trust columns of Table 2 do not depend on the window, and in the T1–T2 comparison a 29-point gain in accuracy produces no detectable gain in trust.

Clicking behavior offers descriptive evidence on the role of content, although it cannot separate the two channels discussed above. Table 3 compares respondents who opened the link in T1 with respondents who opened the link in T3, the same action, taken at the same point in the survey, leading to different pages. Among those who clicked, respondents shown the air quality index are 5.9 percentage points more likely to view the EPA as politically independent than respondents shown the regional office (col. 4, $p = 0.058$; 7.0 points without controls, $p = 0.026$), and 0.26 points higher on the seven-point scale ($p = 0.010$; Table A34 in the Appendix). Among those who did not click, the T1–T3 difference is smaller and not significant. The two groups' differences cannot themselves be distinguished statistically, so we do not claim the effect is confined to clickers; but holding the act of consulting fixed and varying only the content, it is the informative content that goes with higher trust.

We point to two clarifications. First, opening the link is a post-treatment choice, so every coefficient in Table 3 is a descriptive association within arms; the causal evidence remains the intention-to-treat comparison in Table 2. Second, clicking is a proxy of consumption (i.e., it records that the page was opened, not whether anything was read, and a T1 respondent who did not click may still have consulted the index on a phone) which is why answering correctly, not clicking, is our primary measure of consumption in Table 2, and why the click-based contrasts here should be read as attenuated and as uninformative about whether consultation, rather than the prompt, drives the trust effect.

Table 3: Trust and opening the link, by treatment arm.

| VARIABLES | (1) | (2) | (3) | (4) | (5) | (6) |
|---|---|---|---|---|---|---|
| | EPA improves lives | | EPA independent (binary) | | Support for EPA | |
| T1: Low search costs & high value info | 0.0578 | 0.0382 | 0.0532 | 0.0422 | 0.0387 | 0.0241 |
| | (0.0485) | (0.0475) | (0.0410) | (0.0418) | (0.0487) | (0.0483) |
| Clicked link | 0.0788* | 0.0700* | -0.0216 | -0.0158 | -0.0008 | -0.0079 |
| | (0.0420) | (0.0407) | (0.0329) | (0.0333) | (0.0426) | (0.0415) |
| Clicked × T1 | -0.0263 | -0.0305 | 0.0170 | 0.0165 | 0.0374 | 0.0390 |
| | (0.0615) | (0.0599) | (0.0517) | (0.0519) | (0.0621) | (0.0610) |
| Constant | 0.552*** | 0.245** | 0.191*** | 0.269*** | 0.592*** | 0.503*** |
| | (0.0316) | (0.105) | (0.0252) | (0.0874) | (0.0317) | (0.115) |
| T1 vs T3, among those who clicked | 0.0315 | 0.0076 | 0.0702** | 0.0587* | 0.0761** | 0.0631* |
| | (0.0379) | (0.0369) | (0.0314) | (0.0309) | (0.0386) | (0.0374) |
| T1 vs T3, among those who did not | 0.0578 | 0.0382 | 0.0532 | 0.0422 | 0.0387 | 0.0241 |
| | (0.0485) | (0.0475) | (0.0410) | (0.0418) | (0.0487) | (0.0483) |
| Observations | 1,075 | 1,075 | 1,067 | 1,067 | 1,049 | 1,049 |
| R-squared | 0.020 | 0.123 | 0.009 | 0.070 | 0.006 | 0.093 |
| Demographics | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 0.611 | 0.611 | 0.206 | 0.206 | 0.619 | 0.619 |

*Notes*: Standard errors in parentheses, robust to heteroskedasticity. *** p<0.01, ** p<0.05, * p<0.1.
OLS on the two arms that were offered a link (T1 and T3), the only arms in which clicking is recorded. Excluded arm: T3. The rows below the line report the T1–T3 difference among respondents who opened the link and among those who did not; the latter equals the coefficient on T1. Outcomes are binary, as in Study I: agrees that the EPA improves lives and that it is politically independent (above the scale midpoint), and would support the EPA leading a community air quality study (> 50); original scales in Table A34. Clicking is a post-treatment choice, so these are descriptive associations. Note also that the air quality index is available from many weather applications, so a T1 respondent who did not open the EPA link may still have consulted the number elsewhere.

### 4.3.2 Robustness Checks

**Knowing the EPA.** Approximately 30% of respondents did not identify the EPA as a familiar institution. As shown in Table A37, familiarity with the EPA is positively correlated with perceived benevolence and with support for its involvement in a community study, but negatively correlated with perceived political independence: respondents who know the EPA rate it 0.15 to 0.18 points more politicized on the seven-point scale. The two patterns are not in tension once one asks who the familiar respondents are. Familiarity is not the same as having consumed useful information from the agency. For a partisan respondent, knowing the EPA can mean knowing it as an object of political controversy, and in August 2024 the agency was under sustained attack from Republican leadership. Table A38 splits the association by news slant. Among consumers of Republican-leaning news, knowing the EPA is associated with a 0.64 point lower independence rating (s.e. 0.17), against 0.22 points (s.e. 0.09) among consumers of balanced news; among consumers of Democratic-leaning news the sign reverses, and familiarity goes with a 0.53 point *higher* rating (s.e. 0.19). In raw means, Republican-leaning respondents who know the EPA rate its independence at 2.5 on the seven-point scale, against 3.1 for Republican-leaning respondents who do not know it and about 3.5 for everyone else. The negative unconditional association therefore reflects the presence, among those who know the agency, of Republican-leaning

respondents who know it and distrust it, rather than a general tendency for familiarity to breed a perception of politicization. Consistent with the first study, Republican-leaning media consumers were also less likely to report knowing the EPA ($p < 0.10$; Table A39). Among respondents in the control group T2 (no link provided), knowing the EPA remains a strong predictor of trust even after controlling for knowledge accuracy, consistent with our theoretical prediction that familiarity lowers perceived information costs and reinforces trust independently of any single informational interaction.

**Placebo outcomes.** As specified in the pre-analysis plan, we asked the same trust questions about the IRS to check whether respondents differentiate across institutions. Treatment effects on IRS trust are not significant in any specification but one, and that effect disappears with controls (Table A40 in the Appendix). They are not zero, however. On the binary measure, T1 raises the share viewing the IRS as politically independent by 3.4 percentage points relative to T3 (s.e. 2.4), about half the effect on the EPA, and regressing the within-respondent gap between the EPA and IRS measures on treatment, the EPA effect exceeds the IRS effect by 2.8 points (s.e. 2.4, $p = 0.25$); the same comparison on the original scales is in Table A40. We therefore cannot rule out that part of the T1–T3 effect reflects a broader shift in views of federal agencies rather than of the EPA alone, a spillover that would parallel the one to the scientific community in Study I. It also means that the placebo by itself cannot exclude demand effects; the case against them rests on the T1–T3 design, in which both arms receive a link to the same agency.

**Alternative specifications.** The trust outcomes on their original Likert and 0–100 scales (Table A32 in the Appendix) give the same pattern as the binarised versions in Table 2; a probit first stage and ordered probit for the Likert-scale outcomes are also consistent (Table A41).

**EPA office location.** The null trust effects in T3 are not driven by respondents discovering that the EPA has no office in their state and updating negatively. There is no evidence of this mechanism in the data (Table A42 in the Appendix).

**Heterogeneity.** In line with Study I results, we find no significant heterogeneity by news consumption, political affiliation, education, or age. Respondents in more polluted areas (zip codes with a headline AQI above 50) are somewhat more likely to answer correctly when treated, but the interaction is significant only once controls are included and is far smaller than the main treatment effect, and we find no corresponding heterogeneity in any trust outcome (Table A43 in the Appendix). We would in any case caution against reading this split as a test of the $\sigma_i^2$ channel. An AQI above 50 records how *severe* air quality is at the moment of the survey, whereas $\sigma_i^2$ is the variability of the index over time, which is what would make it worth consulting repeatedly; the two are close to orthogonal across the zip codes in our sample. Measuring that variability credibly requires several months of AirNow history rather than the field window itself, and we leave it to future work. Our evidence that information value matters therefore rests on the T1–T3 contrast, which is the cleaner test in any case: it holds the institution, the search cost, and the format of the prompt fixed, and varies the value of the information offered, in both the question and the linked page.

We cannot test whether respondents in the control group spent more time on the treatment question by news consumption, as we did not measure this dimension. We have access to the time spent on the survey overall, but this measure is noisier.

# 5 Conclusions

Trust in public institutions matters to promote behaviors that address societal challenges, but it has declined across Western democracies with implications for social welfare. In this paper, we show that low institutional trust can also be explained by limited understanding of an institution's work and limited engagement with institution-sourced information, triggering a vicious cycle. We use two survey experiments conducted on nationally representative samples of U.S. residents to explore this question. In Study I, reducing the cost of consulting an institution's information increases factual knowledge and raises trust four months later. In Study II, offering decision-relevant information raises trust more than offering low-value information about the same institution. When individuals engage with institution-provided content, they become better informed and report higher trust, with suggestive evidence of increased compliance with recommended behaviors. In Study I, knowledge and trust effects are visible four months after a single exposure. In Study II, trust responds to the value of the information offered. These findings point to the importance for organizations to design outreach communication campaigns that provide valuable and timely information, and that this can increase trust in the organization itself. These results also suggest that part of the erosion in institutional trust may reflect inattention rather than deeply entrenched preferences. A design that holds the prompt fixed while varying whether valuable information can be consulted would separate the awareness and consultation channels, which a future study could explore.

Importantly, both experiments were conducted in politically salient environments, where baseline attention to institutions may already be elevated. While this could attenuate the impact of our treatments, the consistency of our results across different institutional contexts and time periods nonetheless suggests that the underlying mechanism does not need to be confined to moments of crisis. At the same time, survey experiments such as ours always rely on an intermediary platform (e.g., NORC) which may amplify the observed effects. A large-scale field experiment, conducted in partnership with an organization, can offer evidence on the exact magnitude of these effects when messaging comes directly from a public institution. Future work could also examine the scalability of these interventions across diverse institutional and political environments. Extending the framework internationally would help clarify how cultural and educational contexts shape the impact of interventions. Additional research may also explore whether trust gains spill over into other domains of social capital and how responses vary when institutional information is more complex or cognitively demanding, helping to disentangle the roles of cognitive capacity and behavioral inattention in shaping trust (Nunnari et al., 2024; Dal Bó et al., 2018).

Although economic growth, elite cues, and partisan news will remain central in shaping citizens' trust in their institutions, our results indicate that outreach information campaigns may help counteract the decline in public trust. We find no robust partisan heterogeneity, though our estimates for Republican-leaning news consumers are imprecise.

# References


Acemoglu, D., A. Cheema, A. I. Khwaja, and J. A. Robinson (2020). Trust in state and nonstate actors: Evidence from dispute resolution in pakistan. *Journal of Political Economy 128*(8), 3090–3147.

Algan, Y., D. Cohen, E. Davoine, M. Foucault, and S. Stantcheva (2021). Trust in scientists in times of pandemic: Panel evidence from 12 countries. *Proceedings of the National Academy of Sciences 118*(40), e2108576118.

Alsan, M. and M. Wanamaker (2018). Tuskegee and the health of black men. *The quarterly journal of economics 133*(1), 407–455.

Anderson, M. L. (2008). Multiple inference and gender differences in the effects of early intervention: A reevaluation of the abecedarian, perry preschool, and early training projects. *Journal of the American statistical Association 103*(484), 1481–1495.

Belot, M. and G. Briscese (2023). *Bridging America's Divide on Abortion, Guns and Immigration: An Experimental Study.*

Bertrand, M., G. Briscese, M. Grignani, and S. Nassar (2020). How covid-19 is changing americans' behaviors, expectations, and political views. *The University of Chicago Booth School of Business. Retrieved September 14*, 2022.

Besley, T., C. Dann, and S. Dray (2026). Growth experiences and trust in government. *The Quarterly Journal of Economics 141*(2), 1761–1822.

Besley, T. and S. Dray (2022). Pandemic responsiveness: Evidence from social distancing and lockdown policy during covid-19. *PloS one 17*(5), e0267611.

Besley, T. and S. Dray (2024). Trust and state effectiveness: The political economy of compliance. *The Economic Journal*, ueae030.

Böhm, R., A. Schneider, C. Betsch, and L. Lilleholt (2026). CDC communication undermines trust in vaccines. *Science*.

Bolsen, T. and J. N. Druckman (2018). Do partisanship and politicization undermine the impact of a scientific consensus message about climate change? *Group Processes & Intergroup Relations 21*(3), 389–402.

Braithwaite, V. and M. Levi (1998). *Trust and governance*. Russell Sage Foundation.

Brouwer, N. and J. De Haan (2026). The impact of public knowledge about central bank independence on trust and inflation expectations. *Journal of Economic Behavior & Organization 241*, 107329.

Budak, C., S. Goel, and J. M. Rao (2016). Fair and balanced? quantifying media bias through crowdsourced content analysis. *Public Opinion Quarterly 80*(S1), 250–271.

Bursztyn, L., A. Rao, C. Roth, and D. Yanagizawa-Drott (2020). Misinformation during a pandemic. *University of Chicago, Becker Friedman Institute for Economics Working Paper* (2020-44).

Chetty, R., J. N. Friedman, N. Hendren, M. Stepner, and T. O. I. Team (2020). *The economic impacts of COVID-19: Evidence from a new public database built using private sector data*. Number w27431. National Bureau of Economic Research.

Clark, C. J., C. Isch, J. A. Everett, and A. Shariff (2023). Even when ideologies align, people distrust politicized institutions.

Clarke, D., J. P. Romano, and M. Wolf (2020). The romano–wolf multiple-hypothesis correction in stata. *The Stata Journal 20*(4), 812–843.

Conlon, J. J. (2026). Inattention, information provision, and (failures of) revealed preference. Working paper, Carnegie Mellon University, 21 January 2026.

Dal Bó, E., P. Dal Bó, and E. Eyster (2018). The demand for bad policy when voters underappreciate equilibrium effects. *The Review of Economic Studies 85*(2), 964–998.

Darden, M. E. and M. Macis (2024). Trust and health care-seeking behavior. Technical report, National Bureau of Economic Research.

De Quidt, J., J. Haushofer, and C. Roth (2018). Measuring and bounding experimenter demand. *American Economic Review 108*(11), 3266–3302.

Dyer, O. (2020). Covid-19: Black people and other minorities are hardest hit in us.

Fancourt, D., A. Steptoe, and L. Wright (2020). The cummings effect: politics, trust, and behaviours during the covid-19 pandemic. *The lancet 396*(10249), 464–465.

Fuster, A., R. Perez-Truglia, M. Wiederholt, and B. Zafar (2022). Expectations with endogenous information acquisition: An experimental investigation. *Review of Economics and Statistics 104*(5), 1059–1078.

Gabaix, X. (2014). A sparsity-based model of bounded rationality. *The Quarterly Journal of Economics 129*(4), 1661–1710.

Gabaix, X. (2019). Behavioral inattention. In *Handbook of behavioral economics: Applications and foundations 1*, Volume 2, pp. 261–343. Elsevier.

Gaines, B. J., J. H. Kuklinski, P. J. Quirk, B. Peyton, and J. Verkuilen (2007). Same facts, different interpretations: Partisan motivation and opinion on iraq. *The Journal of Politics 69*(4), 957–974.

Gallup (Last accessed: November 2024). Confidence in u.s. institutions down; average at new low.

Gentzkow, M., M. B. Wong, and A. T. Zhang (2025). Ideological bias and trust in information sources. *American Economic Journal: Microeconomics 17*(2), 162–213.

Groseclose, T. and J. Milyo (2005). A measure of media bias. *The Quarterly Journal of Economics 120*(4), 1191–1237.

Haaland, I. and C. Roth (2023). Beliefs about racial discrimination and support for pro-black policies. *Review of Economics and Statistics 105*(1), 40–53.

Haaland, I., C. Roth, and J. Wohlfart (2023). Designing information provision experiments. *Journal of economic literature 61*(1), 3–40.

Kerwin, J., N. Rostom, and O. Sterck (2024). *Striking the right balance: Why standard balance tests over-reject the null, and how to fix it*. JSTOR.

Khan, A., S. Nasim, M. Shaukat, and A. Stegmann (2021). Building trust in the state with information: Evidence from urban punjab. *Journal of Public Economics 202*, 104494.

Kuang, P., M. Weber, and S. Xie (2025, February). Central bank communication with the polarized public. NBER Working Paper 33524, National Bureau of Economic Research.

León-Ciliotta, G., D. Zejcirovic, and F. Fernandez (2023). Policy-making, trust and the demand for public services: Evidence from a mass sterilization campaign. *American Economic Journal: Economic Policy*.

Levi, M. (1988). *Of rule and revenue*. Univ of California Press.

Levi, M. and L. Stoker (2000). Political trust and trustworthiness. *Annual review of political science 3*(1), 475–507.

Link, S., A. Peichl, O. Pfäuti, C. Roth, and J. Wohlfart (2026). Attention to the macroeconomy. *The Quarterly Journal of Economics*. forthcoming.

Maćkowiak, B. and M. Wiederholt (2025, April). Rational inattention during an RCT. ECB Working Paper 3054, European Central Bank.

Martinez-Bravo, M. and C. Sanz (2022). The management of the pandemic and its effects on trust and accountability. *Documentos de Trabajo (CEMFI)* (7), 1.

Martinez-Bravo, M. and A. Stegmann (2019). In vaccines we trust. *The Effects of the CIA's Vaccine Ruse on Immunization in Pakistan*(No. wp2017_1713)*.

Martinez-Bravo, M. and A. Stegmann (2022). In vaccines we trust? the effects of the cia's vaccine ruse on immunization in pakistan. *Journal of the European Economic Association 20*(1), 150–186.

Mayer, R. C., J. H. Davis, and F. D. Schoorman (1995). An integrative model of organizational trust. *Academy of management review 20*(3), 709–734.

McLaughlin, D. M., J. Mewhirter, and R. Sanders (2021). The belief that politics drive scientific research & its impact on covid-19 risk assessment. *Plos one 16*(4), e0249937.

Nunnari, S., E. Proto, and A. Rustichini (2024). Cognitive abilities and the demand for bad policy.

Rafkin, C., A. Shreekumar, and P.-L. Vautrey (2021). When guidance changes: Government stances and public beliefs. *Journal of Public Economics 196*, 104319.

Romano, J. P. and M. Wolf (2005). Stepwise multiple testing as formalized data snooping. *Econometrica 73*(4), 1237–1282.

Sims, C. A. (2003). Implications of rational inattention. *Journal of Monetary Economics 50*(3), 665–690.

Smith, A. (1759). The theory of moral sentiments.

Smith, A. (1776). *The wealth of nations [1776]*, Volume 11937. na.

Sofianos, A. (2022). Self-reported & revealed trust: Experimental evidence. *Journal of Economic Psychology 88*, 102451.

Stantcheva, S. (2023). How to run surveys: A guide to creating your own identifying variation and revealing the invisible. *Annual Review of Economics 15*(1), 205–234.

Tibshirani, R. (1996). Regression shrinkage and selection via the lasso. *Journal of the Royal Statistical Society Series B: Statistical Methodology 58*(1), 267–288.

Tyler, T. R. (1990). Why people obey the law. *Yale University*.

Van Scoy, L. J., B. Snyder, E. L. Miller, O. Toyobo, A. Grewel, G. Ha, S. Gillespie, M. Patel, J. Reilly, A. E. Zgierska, et al. (2021). Public anxiety and distrust due to perceived politicization and media sensationalism during early covid-19 media messaging. *Journal of Communication in Healthcare 14*(3), 193–205.

Wiswall, M. and B. Zafar (2015). Determinants of college major choice: Identification using an information experiment. *The Review of Economic Studies 82*(2), 791–824.

Wright, A. L., K. Sonin, J. Driscoll, and J. Wilson (2020). Poverty and economic dislocation reduce compliance with covid-19 shelter-in-place protocols. *Journal of Economic Behavior & Organization 180*, 544–554.

# A Appendix

## A1.1 Full Model Derivation and Proofs

This section derives the model in full. We build on the sparsity model of Gabaix (2014, 2019) and depart from it in one place: the citizen holds a belief about how much of an institution's output is useful to her, and that belief scales the return to attending to anything the institution produces. Everything else — the quadratic loss from inattention, the linear attention cost, the corner solution — is inherited. We state each step as a lemma or proposition and prove it, so that it is clear which conclusions are our own and which are standard.

### A1.1.1 Setup

**Institutions.** Institutions are represented as the canonical benevolent social planner, whose primary goal is to maximize citizens' well-being by providing evidence-based guidance on behaviors that improve utility and mitigate negative externalities. Building on Braithwaite and Levi (1998) and Tyler (1990), we conceptualize trust as a continuous judgment that influences behavior: higher institutional trust increases individuals' willingness to comply with recommended behaviors. For institutions to be trustworthy, they must communicate both competence and benevolence (Mayer et al., 1995). We set aside alternative theories framing institutional trust in terms of reciprocity and incentive alignment, focusing instead on how institutions build trust by providing citizens with valuable information that signals competence and integrity.

**The decision.** A citizen chooses a scalar action $a \in \mathbb{R}$ to maximize a twice continuously differentiable utility $u(a, x)$, where $x = (x_1, \ldots, x_n)$ is a vector of attributes of the decision environment and $u_{aa} < 0$. Some attributes are produced by a public institution $j$: for instance, the air quality index that an EPA tracker reports for the citizen's zip code. Let $a^*(x)$ denote the fully informed optimum, defined by $u_a(a^*(x), x) = 0$. By the implicit function theorem,

$$a_{x_i} \;\equiv\; \frac{\partial a^*}{\partial x_i} \;=\; -\frac{u_{ax_i}}{u_{aa}}, \tag{5}$$

which measures how much the optimal action responds to attribute $i$.

**Attention.** The citizen does not observe $x$ freely. For each attribute she chooses an attention level $m_i \in [0, 1]$ and perceives

$$x_i^s \;=\; m_i x_i + (1 - m_i)\, x_i^d, \tag{6}$$

a weighted average of the true value and a default $x_i^d$ — the value she would assume if she did not look. With $m_i = 1$ she perceives the truth; with $m_i = 0$ she perceives the default. She then acts on what she perceives, choosing $a^*(x^s)$. Let $\sigma_i^2 \equiv \mathbb{E}\big[(x_i - x_i^d)^2\big]$ be the mean squared distance between the truth and the default; when the default is the prior mean, $\sigma_i^2$ is simply the variance of $x_i$. We assume the deviations $x_i - x_i^d$ are mean zero and independent across attributes.

**Attention cost.** Attention is costly:[28]

$$\mathcal{C}(m) \;=\; \kappa \sum_{i=1}^{n} c_i \, m_i, \tag{7}$$

where $\kappa > 0$ is an individual-level cognitive cost and $c_i > 0$ is the search cost of attribute $i$ — how hard it is to find. A direct link lowers $c_i$; its absence raises it.

**Trust.** This is our departure. Not every attribute an institution produces is useful to a given citizen: some of the EPA's output bears on her decisions and some does not, and she does not know in advance which is which. Let $\omega_i \in \{0, 1\}$ indicate whether attribute $i$ from institution $j$ is useful, in the sense that it enters her decision: the relevant marginal effect is $a_{x_i}\,\omega_i$, so a useless attribute ($\omega_i = 0$) is one whose value would not change her action even if she knew it. Let $\lambda_j \in [0, 1]$ be the share of $j$'s output that is useful to her, and let $\omega_i \mid \lambda_j \sim \text{Bernoulli}(\lambda_j)$ independently across the attributes $j$ produces. She does not know $\lambda_j$; she holds a prior over it with mean

$$\theta_j \;\equiv\; \mathbb{E}[\lambda_j] \;\in\; [0, 1], \tag{8}$$

and this is what we call trust in $j$. It is the share of the institution's work she expects to be useful. The definition is narrower than trust as commonly measured, and deliberately so: it is the component that determines whether she bothers to consult the institution at all, and it is the component our experiments move.

**Timing.** There are two periods. In period 0 the citizen chooses attention $m$, given $\theta_{j0}$, then acts. If she attended to an attribute from $j$ she observes whether it was useful, and updates to $\theta_{j1}$. In period 1 she faces the same problem with the updated belief.

### A1.1.2 The Loss from Inattention

**Lemma 1** (Quadratic loss)**.** *To second order, the expected utility loss from acting on $x^s$ rather than $x$ is*

$$\mathcal{L}(m) \;=\; \frac{1}{2} \sum_{i=1}^{n} (1 - m_i)^2 \, \Lambda_{ii}, \qquad \Lambda_{ii} \equiv \sigma_i^2 \, a_{x_i}^2 \, |u_{aa}| \;=\; \sigma_i^2 \, |a_{x_i} u_{aa} a_{x_i}|. \tag{9}$$

*Proof.* Write $\hat{a} = a^*(x^s)$ for the action taken and $a^* = a^*(x)$ for the fully informed optimum. Expanding $u(\cdot, x)$ around $a^*$ to second order and using $u_a(a^*, x) = 0$,

$$u(a^*, x) - u(\hat{a}, x) \;\approx\; -\tfrac{1}{2}\, u_{aa}\, (\hat{a} - a^*)^2 \;=\; \tfrac{1}{2}\, |u_{aa}|\, (\hat{a} - a^*)^2.$$

Expanding $a^*(\cdot)$ to first order and using (6),

$$\hat{a} - a^* \;\approx\; \sum_{i=1}^{n} a_{x_i}\, (x_i^s - x_i) \;=\; \sum_{i=1}^{n} a_{x_i}\, (1 - m_i)\, (x_i^d - x_i).$$

[28] Gabaix (2014) writes the cost as $\kappa \sum_i m_i^\alpha$. We add an attribute-specific search cost $c_i$ and set $\alpha = 1$, the value that makes the solution both sparse and continuous (Tibshirani, 1996).

Squaring, taking expectations, and using that the deviations $x_i^d - x_i$ are mean zero and independent across $i$ so that all cross terms vanish,

$$\mathbb{E}\big[u(a^*, x) - u(\hat{a}, x)\big] \;\approx\; \tfrac{1}{2}\,|u_{aa}| \sum_{i=1}^{n} a_{x_i}^2\,(1 - m_i)^2\,\sigma_i^2,$$

which is (9). □

This is the standard result (Gabaix, 2014, Lemma 1). The loss from ignoring attribute $i$ is large when the attribute is variable ($\sigma_i^2$), when the action responds to it ($a_{x_i}^2$), and when the action matters ($|u_{aa}|$). Our addition is the next step.

**Lemma 2** (Trust scales the loss)**.** *For attributes produced by institution $j$, the expected loss from inattention, evaluated under the citizen's beliefs about usefulness, is*

$$\mathcal{L}_j(m) \;=\; \frac{1}{2}\sum_{i \in j}(1 - m_i)^2\,\theta_j\,\Lambda_{ii}. \tag{10}$$

*Proof.* For an attribute from $j$ the marginal effect on the action is $a_{x_i}\omega_i$ rather than $a_{x_i}$. Substituting into the proof of Lemma 1, the $i$-th term of the loss becomes $\frac{1}{2}|u_{aa}|\,a_{x_i}^2\,\omega_i^2\,(1 - m_i)^2\,\sigma_i^2$, and the cross terms still vanish because they are multiplied by $\mathbb{E}[(x_i^d - x_i)(x_k^d - x_k)] = 0$. Since $\omega_i \in \{0, 1\}$, $\omega_i^2 = \omega_i$. The citizen chooses $m$ before observing $\omega_i$ and $\omega_i$ is independent of $x_i - x_i^d$, so taking expectations over her beliefs,

$$\mathbb{E}[\omega_i] \;=\; \mathbb{E}\big[\,\mathbb{E}[\omega_i \mid \lambda_j]\,\big] \;=\; \mathbb{E}[\lambda_j] \;=\; \theta_j$$

by the law of iterated expectations and (8). Hence the $i$-th term is $\frac{1}{2}(1 - m_i)^2\,\theta_j\,\Lambda_{ii}$. □

The intuition is that an attribute the citizen expects to be useless is costless to ignore. Trust therefore enters as a multiplier on the return to attention for everything $j$ produces, not as a shift in any single attribute's relevance. Setting $\theta_j = 1$ recovers Gabaix (2014) exactly.

### A1.1.3 Optimal Attention

The citizen chooses $m \in [0, 1]^n$ to minimize expected loss plus attention cost:

$$\min_{m \in [0,1]^n} \;\frac{1}{2}\sum_{i=1}^{n}(1 - m_i)^2\,\theta_j\,\Lambda_{ii} \;+\; \kappa \sum_{i=1}^{n} c_i\,m_i. \tag{11}$$

**Proposition 1** (Optimal attention)**.** *The solution to* (11) *is, for each attribute $i$,*

$$m_i^* \;=\; A\!\left(\frac{\theta_j\,\Lambda_{ii}}{\kappa\,c_i}\right), \qquad A(z) \equiv \max\{0,\, 1 - 1/z\}, \tag{12}$$

*with the convention $A(0) = 0$.*

*Proof.* The objective is additively separable across $i$, so each $m_i$ can be chosen independently. Fix $i$ and write $f(m_i) = \frac{1}{2}(1 - m_i)^2\theta_j\Lambda_{ii} + \kappa c_i m_i$. If $\theta_j\Lambda_{ii} = 0$ then $f$ is linear with slope $\kappa c_i > 0$ and the minimizer is $m_i = 0$, consistent with the convention $A(0) = 0$. Otherwise $f''(m_i) = \theta_j\Lambda_{ii} > 0$, so $f$ is strictly convex on $[0, 1]$ and has a unique minimizer. Its derivative is

$$f'(m_i) \;=\; -(1 - m_i)\,\theta_j\,\Lambda_{ii} + \kappa c_i.$$

The unconstrained stationary point is $\tilde{m}_i = 1 - \kappa c_i/(\theta_j\Lambda_{ii})$, which is strictly below 1 since $\kappa c_i > 0$. Two cases. If $\tilde{m}_i > 0$ it lies in $(0, 1)$ and is the constrained minimizer. If $\tilde{m}_i \leq 0$, then $f'(0) = -\theta_j\Lambda_{ii} + \kappa c_i \geq 0$, and by convexity $f' \geq 0$ on all of $[0, 1]$, so the minimizer is the corner $m_i = 0$. Together, $m_i^* = \max\{0, \tilde{m}_i\}$, which is (12) with $z = \theta_j\Lambda_{ii}/(\kappa c_i)$. □

**Corollary 1** (Threshold)**.** *Attribute $i$ receives positive attention if and only if*

$$\theta_j \;>\; \bar{\theta}_i \;\equiv\; \frac{\kappa\, c_i}{\Lambda_{ii}} \;=\; \frac{\kappa\, c_i}{\sigma_i^2\,|a_{x_i} u_{aa} a_{x_i}|}. \tag{13}$$

*Proof.* From the proof of Proposition 1, $m_i^* > 0$ if and only if $\tilde{m}_i > 0$, i.e. $\kappa c_i < \theta_j\Lambda_{ii}$. □

The corner is the feature our design exploits. Three distinct conditions each produce *zero* attention rather than merely less of it: low trust ($\theta_j$ small), a high search cost ($c_i$ large), and information of little decision relevance (small $a_{x_i}$, hence small $\Lambda_{ii}$ and a high threshold). Our first experiment and the T1–T2 contrast of the second lower $c_i$; the T1–T3 contrast varies $a_{x_i}$; and the mechanism tables in Section 4.3.1 concern $\theta_j$.

### A1.1.4 Interpretation of Terms

Four terms govern optimal attention.

$\theta_j$ is trust: the share of institution $j$'s work the citizen expects to be useful. By Lemma 2 it scales the entire benefit of attending, so it acts as a gatekeeper on the whole of $j$'s output rather than on any single attribute.

$\sigma_i^2$ is the variability of attribute $i$ around the default — how much there is to learn by looking. A parent in an area with consistently clean air has little reason to check the AQI regularly, whereas one in an area with volatile pollution levels faces genuine health risk from ignoring it.

$a_{x_i}$ is how much a change in $x_i$ shifts the optimal action. Information that is consequential for decisions receives more attention.

$u_{aa}$ is how much the individual cares about the quality of the action — the stakes.

Together, an individual pays more attention to attributes that are more variable, more consequential, less costly to acquire, and produced by an institution she expects to be useful.

### A1.1.5 Updating and the Vicious Cycle

Having chosen $m^*$, the citizen acts, $a^* = \arg\max_a u(a, x^s)$, and then learns from what she consulted.

**Assumption 1** (Beta prior)**.** *The citizen's prior over $\lambda_j$ is Beta$(\alpha_j, \beta_j)$ with $\alpha_j, \beta_j > 0$. Its mean is $\theta_{j0} = \alpha_j/(\alpha_j + \beta_j)$ and we write $n_j \equiv \alpha_j + \beta_j$ for its precision, which can be read as the amount of prior experience she has with $j$.*

**Proposition 2** (Updating)**.** *If $m_i^* > 0$ for some attribute $i$ from $j$, the citizen observes $\omega_i \sim$ Bernoulli$(\lambda_j)$ and her posterior mean is*

$$\theta_{j1} \;=\; \theta_{j0} + \frac{1}{n_j + 1}\big(\omega_i - \theta_{j0}\big). \tag{14}$$

*If $m_i^* = 0$ for every attribute from $j$, she observes nothing and $\theta_{j1} = \theta_{j0}$.*

*Proof.* The Beta distribution is conjugate to the Bernoulli: a Beta$(\alpha, \beta)$ prior combined with one Bernoulli observation $\omega$ gives a Beta$(\alpha + \omega,\ \beta + 1 - \omega)$ posterior. Its mean is

$$\theta_{j1} \;=\; \frac{\alpha_j + \omega_i}{\alpha_j + \beta_j + 1} \;=\; \frac{n_j\,\theta_{j0} + \omega_i}{n_j + 1} \;=\; \theta_{j0} + \frac{\omega_i - \theta_{j0}}{n_j + 1},$$

using $\alpha_j = n_j\theta_{j0}$. If nothing is observed, the posterior equals the prior. □

Trust moves toward the usefulness the citizen experienced, by an amount that shrinks with her prior experience: a first encounter moves beliefs a lot, the hundredth hardly at all. Two remarks on what does *not* move trust.

*Remark* 1 (No consultation, no update)*.* The update in (14) is conditional on having attended. A citizen with $m_i^* = 0$ observes no $\omega_i$ and her belief is unchanged, whatever the true $\lambda_j$.

*Remark* 2 (Low-value information: less attention, and a weaker signal)*.* An attribute of low decision relevance — small but positive $a_{x_i}$, such as the city in which a regional office is located — affects both stages. In the attention stage, small $a_{x_i}$ means small $\Lambda_{ii}$ and hence a high threshold $\bar{\theta}_i$, so fewer citizens consult it than would consult a high-value attribute at the same search cost; with the search cost near zero, as when a link is supplied, some still do. In the updating stage, what a citizen learns from consulting it is weaker. The usefulness she can experience is bounded by how much the attribute could have changed her action, so the probability that it is found useful is $\lambda_j\rho_i$ rather than $\lambda_j$, with $\rho_i \in (0, 1]$ increasing in $\Lambda_{ii}$ and equal to one for the most relevant attribute. If the citizen attributes the attribute's lack of use to the institution rather than to the attribute, (14) applies with this lower success probability, and the expected revision of $\theta_j$ from consulting attribute $i$ is $(\rho_i\lambda_j - \theta_{j0})/(n_j + 1)$, against $(\lambda_j - \theta_{j0})/(n_j + 1)$ for a fully relevant attribute. It is always smaller, and it is negative whenever $\rho_i\lambda_j < \theta_{j0}$: consulting information of little use can lower trust. The model therefore predicts that T3 in Study II should show positive but lower engagement than T1 — 54 against 66% opened the link — and a smaller, possibly negative, effect on trust. We treat $\rho_i$ as a reduced-form description of this attenuation rather than deriving it.

*Remark* 3 (Relevance and distortion). Write usefulness as $\omega_i = r_i\,(1 - d_i)$, where $r_i \in \{0, 1\}$ indicates that attribute $i$ bears on the citizen's decision and $d_i \in \{0, 1\}$ that it is distorted, so that even a relevant attribute is useless if it is reported in a way that does not track the truth. Let $\lambda_j = \lambda_j^r\,(1 - \lambda_j^d)$, with $\lambda_j^r$ the share of $j$'s output that is relevant and $\lambda_j^d$ the share that is distorted, and let the citizen hold independent priors over the two. Perceived politicization is her belief about $\lambda_j^d$. The prompt in T1 and T2 reveals $r_i = 1$ before any consultation, so consulting the page is uninformative about $\lambda_j^r$ and informative only about $\lambda_j^d$: a number that matches what the citizen can verify is a draw of $d_i = 0$. The T3 prompt reveals a low $r_i$, and by Remark 2 the page then carries little information about either share. Under this decomposition the T1–T3 contrast moves beliefs about distortion, which is what the independence question elicits, more than beliefs about relevance, which is closer to what the benevolence question elicits, and the update is larger for citizens who can check the number. This is consistent with the pattern in Tables 2 and 3, but we offer it as an interpretation rather than as a prediction the design was built to test.

**Proposition 3** (The vicious cycle)**.** *Suppose $\theta_{j0} \le \bar{\theta}_i$ for every attribute $i$ that $j$ produces. Then $\theta_{jt} = \theta_{j0}$ for all $t \ge 0$: trust in $j$ never changes.*

*Proof.* By Corollary 1, $m_i^* = 0$ for every attribute from $j$ in period 0. By Proposition 2 the citizen observes nothing and $\theta_{j1} = \theta_{j0}$. She therefore faces an identical problem in period 1, and the argument repeats. □

The cycle is self-sealing rather than self-confirming. A citizen below the threshold never acquires the evidence that might revise her belief, so mistrust persists regardless of whether it was warranted; the same mechanism sustains high trust above the threshold. An intervention that lowers $c_i$ lowers $\bar{\theta}_i$ and can bring a citizen over it, after which (14) operates and trust can move. The size of that movement is what the next result decomposes.

### A1.1.6 The Intention-to-Treat Effect on Trust

Let $D \in \{0, 1\}$ indicate that the citizen consulted the institution ($m_i^* > 0$ for some $i$ from $j$), and let $\Delta\theta_j \equiv \theta_{j1} - \theta_{j0}$. Compare a treated arm $T$ and a comparison arm $C$, and write $\pi_T = \Pr(D = 1 \mid T)$ and $\pi_C = \Pr(D = 1 \mid C)$ for the shares consulting.

**Proposition 4** (ITT decomposition)**.** *The intention-to-treat effect of the treatment on trust is*

$$ITT_{trust} \;=\; \pi_T\,\mathbb{E}[\Delta\theta_j \mid D = 1, T] \;-\; \pi_C\,\mathbb{E}[\Delta\theta_j \mid D = 1, C]. \tag{15}$$

*If, in addition, consulting yields the same expected update in both arms, $\mathbb{E}[\Delta\theta_j \mid D = 1, T] = \mathbb{E}[\Delta\theta_j \mid D = 1, C] \equiv \delta$, then*

$$ITT_{trust} \;=\; \underbrace{(\pi_T - \pi_C)}_{\text{change in share consulting}} \;\times\; \underbrace{\delta}_{\text{update among consumers}}. \tag{16}$$

*Proof.* By the law of iterated expectations, for either arm $g \in \{T, C\}$,

$$\mathbb{E}[\Delta\theta_j \mid g] \;=\; \pi_g\,\mathbb{E}[\Delta\theta_j \mid D = 1, g] + (1 - \pi_g)\,\mathbb{E}[\Delta\theta_j \mid D = 0, g] \;=\; \pi_g\,\mathbb{E}[\Delta\theta_j \mid D = 1, g],$$

because by Remark 1 $\Delta\theta_j = 0$ whenever $D = 0$. Subtracting the $C$ expression from the $T$ expression gives (15). Under the equal-update condition both conditional expectations equal $\delta$, and (15) factors into (16). □

The equal-update condition is what distinguishes the two contrasts in Study II. T1 and T2 give access to the *same* EPA page; a respondent who reaches it experiences the same $\omega_i$ whichever arm she is in, so $\delta$ is common and (16) applies. Their trust effects then differ only by the difference in the share who reach the page, which can be small even when $\delta$ is large — the point made in Section 4.3. T1 and T3 give access to *different* pages, and by Remark 2 the update from the T3 page is smaller, $\delta_{T3} < \delta_{T1}$, and possibly negative, so the equal-update condition fails and (15) applies: the T1–T3 trust effect is $\pi_{T1}\,\delta_{T1} - \pi_{T3}\,\delta_{T3}$, positive both because more T1 respondents consult and because each consultation raises trust by more. The comparison among those who clicked in Table 3 estimates the second component, $\delta_{T1} - \delta_{T3}$, under the further assumption that respondents who click in T1 and those who click in T3 start from the same $\theta_{j0}$; since clicking is a choice, that assumption is not guaranteed, which is why we present those estimates as descriptive.

### A1.1.7 Example: Quadratic Utility

The general expressions simplify under quadratic utility, and this is the case used in the numerical example that follows.

**Proposition 5** (Quadratic utility)**.** *Let* $u(a, x) = -\frac{1}{2}\big(a - \sum_{i=1}^{n} b_i x_i\big)^2$. *Then* $a_{x_i} = b_i$, $u_{aa} = -1$, $\Lambda_{ii} = \sigma_i^2 b_i^2$, *and*

$$m_i^* \;=\; \max\left\{0,\; 1 - \frac{\kappa c_i}{\theta_j\,\sigma_i^2\,b_i^2}\right\}, \tag{17}$$

*with positive attention if and only if* $\kappa c_i < \theta_j \sigma_i^2 b_i^2$.

*Proof.* Differentiating, $u_a = -(a - \sum_i b_i x_i)$, so $u_{aa} = -1$ and $u_{ax_i} = b_i$. By (5), $a_{x_i} = -u_{ax_i}/u_{aa} = b_i$, and the fully informed optimum is $a^*(x) = \sum_i b_i x_i$. Substituting into (9), $\Lambda_{ii} = \sigma_i^2 b_i^2 \cdot 1$. Proposition 1 then gives (17), and Corollary 1 gives the condition. □

The number of attributes that can receive positive attention is therefore bounded: at most $\sum_i \mathbf{1}[\kappa c_i < \theta_j \sigma_i^2 b_i^2]$ of them. Higher cognitive costs $\kappa$, or lower trust $\theta_j$, shrink that set.

### A1.1.8 A Numerical Example

The following example makes the mechanism concrete and shows how the two experiments map onto the parameters of equation (1). It is illustrative rather than calibrated: the parameters are chosen to be transparent, not estimated.

Under quadratic utility, equation (12) specialises to $m_i^* = \max\{0,\, 1 - \kappa c_i/(\theta_j \sigma_i^2 b_i^2)\}$, so attention is strictly positive if and only if trust clears the threshold in equation (13), $\theta_j > \bar{\theta}_i = \kappa c_i/(\sigma_i^2 b_i^2)$.

Set the cognitive cost to $\kappa = 1$ and consider two pieces of information. *High-value* information — the local air quality index in Study II, the current COVID-19 death count

in Study I — is variable and consequential for the decision at hand, $\sigma^2 b^2 = 1.00$. *Low-value* information — the city hosting the EPA regional office — is accurate but of little relevance to any decision, $\sigma^2 b^2 = 0.55$. Consider also two search costs: a direct link to the institution's page, $c = 0.30$, and no link, $c = 0.85$. Finally, consider a trusting citizen, $\theta_j = 1$, and a mistrusting one, $\theta_j = 0.5$. Table A1 reports the resulting optimal attention.

Table A1: Optimal attention $m^*$ in the numerical example.

| | | High-value info. $\sigma^2 b^2 = 1.00$ | Low-value info. $\sigma^2 b^2 = 0.55$ |
|---|---|---|---|
| Trusting citizen ($\theta_j = 1$) | Link ($c = 0.30$) | 0.70 | 0.45 |
| | No link ($c = 0.85$) | 0.15 | 0 |
| Mistrusting citizen ($\theta_j = 0.5$) | Link ($c = 0.30$) | 0.40 | 0 |
| | No link ($c = 0.85$) | 0 | 0 |

*Notes*: Optimal attention $m^* = \max\{0,\, 1 - \kappa c/(\theta_j \sigma^2 b^2)\}$ with $\kappa = 1$. The threshold level of trust above which attention is positive, $\bar{\theta} = \kappa c/(\sigma^2 b^2)$, is 0.30 for high-value information with a link, 0.85 without a link, 0.55 for low-value information with a link, and 1.55 for low-value information without a link — above one, so no citizen attends.

Three features of the table map directly onto the experimental arms. First, reading down the trusting citizen's rows in the first column reproduces the search-cost comparison, T1 versus T2 in Study II and treatment versus control in Study I: removing the link cuts attention from 0.70 to 0.15, because the threshold rises from $\bar{\theta} = 0.30$ to $\bar{\theta} = 0.85$. Second, reading across that citizen's first row reproduces the information-value comparison, T1 versus T3: holding the search cost fixed, low-value information yields 0.45 rather than 0.70, and the ordering $m^*(\text{T1}) > m^*(\text{T3}) > m^*(\text{T2})$ matches the ordering of observed engagement across the three arms of Study II: 66% of T1 respondents opened the link against 54% in T3, while T2, with no link to open, answered correctly least often of all (18% at the ±5 window).

Third, and central to the interpretation of our heterogeneity results, the mistrusting citizen attends only in the single cell where the information is both valuable and cheap to obtain. When the link is withdrawn, her attention is zero — not merely lower — because $\bar{\theta} = 0.85$ exceeds her $\theta_j = 0.5$. The model therefore predicts that in the high-search-cost condition the only respondents who acquire the information are those with a low enough perceived cost: the trusting, and those who already follow the news. Section A2 shows that this is what the data look like in both studies. It also predicts that lowering the search cost is what brings mistrusting citizens over the threshold (0 to 0.40), but only when the information is valuable: with low-value information the mistrusting citizen stays at zero even when the link is provided, and those who do cross the threshold learn less from what they find, which is why T3 moves trust less than T1 does.

### A1.1.9 Model Limitations

Our model focuses on individual-level attention costs and information value as determinants of trust. It abstracts from the macro-level forces behind the sharp aggregate decline in

institutional trust — including elite political attacks, partisan media narratives, and the psychological shock of an unprecedented public health crisis. These forces likely operate through channels our model does not formalize: they may raise the perceived cost of engaging with institutional information (higher $c_i$ for opposing partisans), reduce the credibility weight placed on such information, or shift priors about institutional competence in ways unrelated to individual attention allocation. We view these macro-level forces and our demand-side attention frictions as complementary rather than competing mechanisms: the former helps explain why trust declined so sharply, while our framework explains why it may be recoverable through targeted informational interventions.

# A2 Prior Beliefs, Trust, and Who Pays the Search Cost

The numerical example in Appendix A1.1.8 predicts that when the search cost is high, attention is positive only for citizens whose perceived cost of engaging with the institution is low enough to clear the threshold in equation (13) — the trusting, and those who already follow the news. This subsection reports the evidence for that prediction in both studies. Two caveats apply throughout. These are correlations between pre-treatment characteristics and outcomes, not causal estimates: news consumption and trust in science are chosen, not assigned. And the two studies measure the prior differently. Study I has a measure of trust in the scientific community collected in wave 1 (April 2020), two months before the experiment ran in wave 6, and so genuinely pre-treatment; Study II has no pre-treatment measure of trust in science, so we use the breadth and slant of news consumption alone.

### A2.0.1 Study I

Table A2 reports the results. Respondents who consume more news are substantially more likely to answer the death-toll questions correctly. Following U.S. news for at least thirty minutes a day is associated with a 12.4 percentage point higher probability of a correct answer without controls and 9.7 points with the full control set (cols. 1–2), and following COVID-19 news more closely raises it by 5.9 to 8.1 points per point on the five-point scale (cols. 3–4). Both are large relative to the 39% unconditional accuracy rate, and both survive the full set of demographic, local-pandemic and news-slant controls. The slant of news consumption, by contrast, predicts nothing once weights are applied: neither the Republican-leaning nor the Democratic-leaning indicator is significant in any specification. What matters is how much news a respondent consumes, not which side it comes from.

Pre-treatment trust in the scientific community predicts accuracy in the same direction. Respondents who reported no more than "some confidence" in the scientific community in April — 36% of the experimental sample — answered correctly 29.6% of the time, against 44.4% among the more trusting, a raw gap of 14.8 points. The gap is 12.2 points conditional on treatment status alone and 7.2 points with the full control set (cols. 5–6). When news consumption and trust are entered together (col. 7), the news measures retain their size and significance while the trust coefficient falls to −4.5 points and is no longer significant, which is what one would expect if trust in science and habitual news consumption are two measurements of the same underlying propensity to engage with institutional information.

The most direct test concerns the control group, where no link was offered and the search cost was therefore high. Table A3 restricts attention to those respondents. Trust in science predicts how long they spent on the question: low-trust respondents spent 18.8 fewer seconds without controls and 13.9 fewer with them, against a survey-weighted control-group mean of 52.1 seconds, and were 9.0 points less likely to answer correctly (7.0 with controls). The raw, unweighted contrast is stark: 43.6 seconds and 23.3% accuracy among the low-trust, against 58.1 seconds and 38.1% among the rest. Heavy news consumers move in the opposite direction, spending 14.7 more seconds and answering correctly 14.3 points more often. Consumers of Republican-leaning news spent 25.1 fewer seconds with controls and were 12.8 points less likely to fall in the top quartile of time spent; consumers of Democratic-leaning news, entered in the same regression, show no such deficit ($-2.9$ points, not significant), so the slant effect on searching is specific to the right.

The time result is specific to the arm in which searching was costly, and this is the one place where we can show that rather than assert it. Interacting treatment status with pre-treatment trust, the low-trust deficit in seconds spent is $-19.3$ (s.e. 7.9) in the control group and $+12.5$ (s.e. 11.0, not significant) among the treated, an interaction of 31.8 with a standard error of 13.7 ($p = 0.022$). When the institution's information is a click away, the gap in time spent between trusting and mistrusting respondents closes entirely. This is the pattern in equation (13): when the cost of searching is high it is paid by those for whom it is perceived as low, and lowering that cost is what brings the others in. The accuracy gap behaves differently: low trust predicts a 9.1 point deficit in the control group and a 15.4 point deficit among the treated, and the two cannot be distinguished ($p = 0.32$), so we do not claim that the knowledge gap itself is concentrated where search costs are high.

Time spent is in turn strongly associated with accuracy. Respondents in the top quartile of time on the death questions are 25.1 to 27.3 percentage points more likely to answer correctly (Table A5, cols. 1–2). The treatment itself raises time spent by roughly 14.5 seconds, consistent with the interpretation of the link as a reduction in $c_i$ that raises $m_i^*$.

Finally, the treatment effect itself is concentrated among the more trusting. Splitting on pre-treatment trust, the effect of the link on accuracy is 11.9 points and significant at the 1% level among respondents with higher trust in science ($n = 725$), and 6.2 points and not significant among those with lower trust ($n = 415$). We report this as suggestive only: the interaction term is $-6.3$ points with a standard error of 6.3 and cannot be distinguished from zero, so the study is not powered to establish that the difference is real.

### A2.0.2 Study II

Study II reproduces the news-consumption result with an independent sample, a different institution and a different factual question. Table A4 reports it. Each additional news source a respondent reports is associated with a 1.6 percentage point higher probability of answering correctly without controls and 1.2 points with them (cols. 1–2); consuming three or more sources — 48% of the sample — raises it by 6.9 points (4.7 with controls, cols. 3–4). Unlike in Study I, slant does matter here: consumers of Republican-leaning news are 6.7 points less likely to answer correctly (cols. 5–6), which is unsurprising for a question about the EPA, an agency on which partisan priors are sharply divided in a way they were not for the COVID-19 death count in June 2020.

Restricting to T2, the arm that received the air quality question with no link, each additional news source is associated with a 2.1 point higher probability of a correct answer without controls and 2.3 points with them (cols. 7–8), significant at the 5% level in both specifications despite the smaller sample. We caution against reading this as evidence that the gradient is steeper where the search cost is higher. The within-arm gradient is essentially identical in T1, which asked the same air quality question with a link (2.06 points per source, $p = 0.03$), and flat in T3, which asked about the location of the regional office (0.58 points, $p = 0.58$); a joint test of the two interaction terms cannot reject equality across arms ($p = 0.51$ without controls, $p = 0.25$ with them). What separates the arms is the content of the question rather than the cost of answering it: following the news helps with the current air quality index and is of no use for the city hosting an administrative office. This is consistent with the $\sigma_i^2 b_i^2$ term of the model rather than with its $c_i$ term, and it is why the pooled coefficient in columns (1)–(2) is smaller than the coefficient in either air quality arm.

Study II also observes the two intermediate margins directly. Clicking the link is associated with a 39 to 40 percentage point higher probability of a correct answer, and being in the top quartile of time spent on the question with 18.5 points; entered jointly, both remain significant, at 37.0 and 9.6 points (Table A5, cols. 3–6). Time is measured the same way in both studies, as the top quartile of seconds spent on the treatment question. These are the attention margins $m_i^*$ of the model, and they are the strongest single predictors of accuracy anywhere in either study. Among respondents offered the air quality link, those who consume Democratic-leaning news are 10.4 to 12.3 points more likely to click, while news breadth predicts neither clicking nor time spent — consistent with the reading that breadth captures a stock of prior knowledge that pays off mainly where no link is available, rather than a greater propensity to search when one is.

Table A2: News consumption, pre-treatment trust in science, and knowledge accuracy (Study I).

| | (1) | (2) | (3) | (4) | (5) | (6) | (7) |
|---|---|---|---|---|---|---|---|
| VARIABLES | Correct answer | | | | | | |
| 30+ min/day of U.S. news | 0.124*** | 0.0969*** | | | | | 0.0663** |
| | (0.0272) | (0.0245) | | | | | (0.0276) |
| Follows COVID-19 news (1–5) | | | 0.0807*** | 0.0585*** | | | 0.0464** |
| | | | (0.0182) | (0.0174) | | | (0.0186) |
| Low trust in science (w1) | | | | | -0.122*** | -0.0718** | -0.0450 |
| | | | | | (0.0349) | (0.0303) | (0.0302) |
| Treated | 0.0999*** | 0.125*** | 0.101*** | 0.124*** | 0.0993*** | 0.124*** | 0.127*** |
| | (0.0332) | (0.0286) | (0.0331) | (0.0285) | (0.0315) | (0.0282) | (0.0269) |
| Observations | 1,147 | 1,147 | 1,147 | 1,147 | 1,147 | 1,147 | 1,147 |
| Demographics | No | Yes | No | Yes | No | Yes | Yes |
| Covid shock | No | Yes | No | Yes | No | Yes | Yes |
| News slant | No | Yes | No | Yes | No | Yes | Yes |
| Mean dep. var. | 0.391 | 0.391 | 0.391 | 0.391 | 0.391 | 0.391 | 0.391 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
Survey-weighted OLS regressions on the wave 6 experimental sample. The outcome is whether the respondent correctly identified both the state and the national COVID-19 death toll. *Low trust in science* equals one for respondents reporting at most "some confidence" in the scientific community in wave 1 (April 2020), two months before the experiment, and is therefore measured pre-treatment. *News slant* denotes the Republican- and Democratic-leaning news indicators used throughout the paper; neither is significant in any column. The control variables are those listed in Appendix A4.0.2.

Table A3: Who pays the search cost when it is high? Control group only (Study I).

| VARIABLES | (1) | (2) | (3) | (4) | (5) | (6) |
|---|---|---|---|---|---|---|
| | Seconds on question | | Long time on question | | Correct answer | |
| Low trust in science (w1) | -18.82** | -13.87** | | | -0.0903** | -0.0696** |
| | (7.920) | (5.962) | | | (0.0440) | (0.0352) |
| 30+ min/day of U.S. news | 14.66* | 1.662 | | | 0.143*** | 0.0897** |
| | (8.107) | (7.488) | | | (0.0374) | (0.0349) |
| Rep-leaning news | | | -0.101* | -0.128*** | | |
| | | | (0.0575) | (0.0492) | | |
| Dem-leaning news | | | -0.0173 | -0.0294 | | |
| | | | (0.0570) | (0.0493) | | |
| Observations | 601 | 601 | 601 | 601 | 592 | 592 |
| Demographics | No | Yes | No | Yes | No | Yes |
| Covid shock | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 52.13 | 52.13 | 0.250 | 0.250 | 0.280 | 0.280 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
Survey-weighted OLS regressions restricted to the control group, which received the death-toll questions without a link and therefore faced the higher search cost. *Long time on question* equals one for the top quartile of seconds spent ($\geq$ 75th percentile). Low trust and news volume each come from a separate regression of the outcome on that variable; the Republican- and Democratic-leaning news indicators are entered jointly, with no slanted news as the omitted category. The mean of the dependent variable is the survey-weighted control-group mean. Raw, unweighted means in the control group: respondents with low trust in science spent 43.6 seconds and answered correctly 23.3% of the time, against 58.1 seconds and 38.1% for the rest.

Table A4: News consumption and knowledge accuracy (Study II).

| VARIABLES | (1) | (2) | (3) | (4) | (5) | (6) | (7) | (8) |
|---|---|---|---|---|---|---|---|---|
| | All arms | | | | | | T2 only (no link) | |
| | Correct response | | | | | | | |
| N. of news sources | 0.0160*** | 0.0122** | | | | | 0.0208** | 0.0225** |
| | (0.00560) | (0.00590) | | | | | (0.00870) | (0.00930) |
| Three or more news sources | | | 0.0686*** | 0.0474* | | | | |
| | | | (0.0255) | (0.0271) | | | | |
| Rep-leaning news | | | | | -0.0667** | -0.0681* | | |
| | | | | | (0.0338) | (0.0351) | | |
| Observations | 1,400 | 1,400 | 1,400 | 1,400 | 1,400 | 1,400 | 535 | 535 |
| Treatment arms | Yes | Yes | Yes | Yes | Yes | Yes | — | — |
| Demographics | No | Yes | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 0.441 | 0.441 | 0.441 | 0.441 | 0.441 | 0.441 | 0.271 | 0.271 |

*Notes*: Standard errors in parentheses, robust to heteroskedasticity. *** p<0.01, ** p<0.05, * p<0.1.
OLS regressions. The outcome is whether the respondent answered the factual question correctly, scored against the corrected AQI benchmark described in Section 4.2. *N. of news sources* counts how many of the twelve listed outlets the respondent reports using. Columns (1)–(6) include the treatment arm indicators; columns (7)–(8) are restricted to T2, which received the air quality question without a link and therefore faced the higher search cost.

Table A5: Attention and knowledge accuracy in both studies.

| | (1) | (2) | (3) | (4) | (5) | (6) |
|---|---|---|---|---|---|---|
| | Study I | | | | Study II | |
| VARIABLES | Correct answer | | | | | |
| Long time on question | 0.273*** | 0.251*** | | | | |
| | (0.0385) | (0.0355) | | | | |
| Clicked link | | | 0.404*** | 0.389*** | | 0.370*** |
| | | | (0.0317) | (0.0332) | | (0.0343) |
| Long time on question | | | | | 0.185*** | 0.0960*** |
| | | | | | (0.0370) | (0.0368) |
| Observations | 1,132 | 1,132 | 865 | 865 | 865 | 865 |
| Demographics | No | Yes | No | Yes | Yes | Yes |
| News | No | Yes | No | Yes | Yes | Yes |

*Notes*: Standard errors in parentheses; survey-weighted in cols. (1)–(2), robust in cols. (3)–(6). *** p<0.01, ** p<0.05, * p<0.1.

All columns control for treatment status. Columns (1)–(2) use the wave 6 experimental sample of Study I; *Long time on question* equals one for the top quartile of seconds spent on the death-toll questions. Columns (3)–(6) use the two arms of Study II that were offered a link (T1 and T3), the only arms in which clicking and time on the question are recorded; there *Long time on question* is built the same way, as the top quartile of seconds spent on the treatment question ($\geq$ 60 seconds). These are associations between two endogenous choices and the outcome, not causal estimates.

# A3 Multiple-Hypothesis Adjustments

The main tables report unadjusted p-values for several outcomes at a time. This section reports two adjustments for each family of outcomes: the sharpened false-discovery-rate q-values, and the step-down family-wise adjusted p-values of Romano and Wolf (2005), computed by bootstrap with 2,000 replications (Clarke et al., 2020). A q-value of 0.05 means that, among outcomes declared significant at that threshold, an expected 5% are false discoveries; a Romano-Wolf p-value controls the probability of any false rejection within the family. In Study I the families are the four knowledge and trust outcomes of Table 1 and the four behavioural outcomes of Table A16; the bootstrap resamples primary sampling units and applies the survey weights. As a stricter alternative we also pool all eight Study I outcomes into a single family. In Study II the family is the three pre-registered binary trust outcomes of Table 2, separately for the T1–T3 and T2–T3 contrasts; a pooled family of six is also reported. Knowledge accuracy in Study II is significant at any conventional level under any adjustment ($p < 0.001$) and is omitted.

Table A6: Multiple-hypothesis adjustments, Study I (Tables 1 and A16).

| Controls | Outcome | Coef. (s.e.) | Unadj. $p$ | Sharpened $q$ (4) | Sharpened $q$ (8) | Romano-Wolf $p$ (4) |
|---|---|---|---|---|---|---|
| | *Family A: knowledge and trust* | | | | | |
| No | Correct death est. (June) | 0.101 (0.032) | 0.002 | 0.009 | 0.018 | 0.006 |
| No | Correct US death rank (Oct) | 0.041 (0.034) | 0.229 | 0.107 | 0.131 | 0.230 |
| No | Incr. trust in CDC | 0.064 (0.033) | 0.052 | 0.078 | 0.091 | 0.032 |
| No | CDC not infl. by politics | 0.055 (0.030) | 0.072 | 0.078 | 0.091 | 0.050 |
| Yes | Correct death est. (June) | 0.125 (0.029) | 0.000 | 0.001 | 0.001 | 0.008 |
| Yes | Correct US death rank (Oct) | 0.054 (0.031) | 0.080 | 0.051 | 0.061 | 0.158 |
| Yes | Incr. trust in CDC | 0.062 (0.029) | 0.036 | 0.038 | 0.053 | 0.060 |
| Yes | CDC not infl. by politics | 0.060 (0.027) | 0.028 | 0.038 | 0.053 | 0.060 |
| | *Family B: behaviours* | | | | | |
| No | Tested for Covid-19 | 0.080 (0.037) | 0.031 | 0.067 | 0.079 | 0.294 |
| No | Adopts no prev. behav. | −0.024 (0.011) | 0.027 | 0.067 | 0.079 | 0.294 |
| No | % prev. behav. adopted | 0.002 (0.022) | 0.916 | 0.845 | 0.440 | 0.745 |
| No | Covers mouth in public | 0.022 (0.029) | 0.457 | 0.439 | 0.244 | 0.294 |
| Yes | Tested for Covid-19 | 0.066 (0.031) | 0.035 | 0.057 | 0.053 | 0.197 |
| Yes | Adopts no prev. behav. | −0.023 (0.009) | 0.013 | 0.057 | 0.049 | 0.197 |
| Yes | % prev. behav. adopted | −0.001 (0.020) | 0.961 | 0.925 | 0.317 | 0.953 |
| Yes | Covers mouth in public | 0.019 (0.023) | 0.402 | 0.366 | 0.130 | 0.313 |

*Notes*: Coefficients, standard errors and unadjusted p-values are the survey-design estimates of Tables 1 and A16 (survey weights, primary sampling units, strata). *Sharpened q (4)*: **?** q-values within the family of four outcomes for the given control set. *Sharpened q (8)*: the same, pooling both families. *Romano-Wolf p (4)*: step-down adjusted p-values within the family of four, from 2,000 survey-weighted bootstrap replications that resample primary sampling units (seed 20260915). In family B the bootstrap distribution of the t-statistic for *Adopts no preventive behaviour*, whose control mean is 2%, is wide, and it is this outcome that sets the family-wise adjustment for the whole family.

Table A7: Multiple-hypothesis adjustments, Study II (Table 2, cols 3–8).

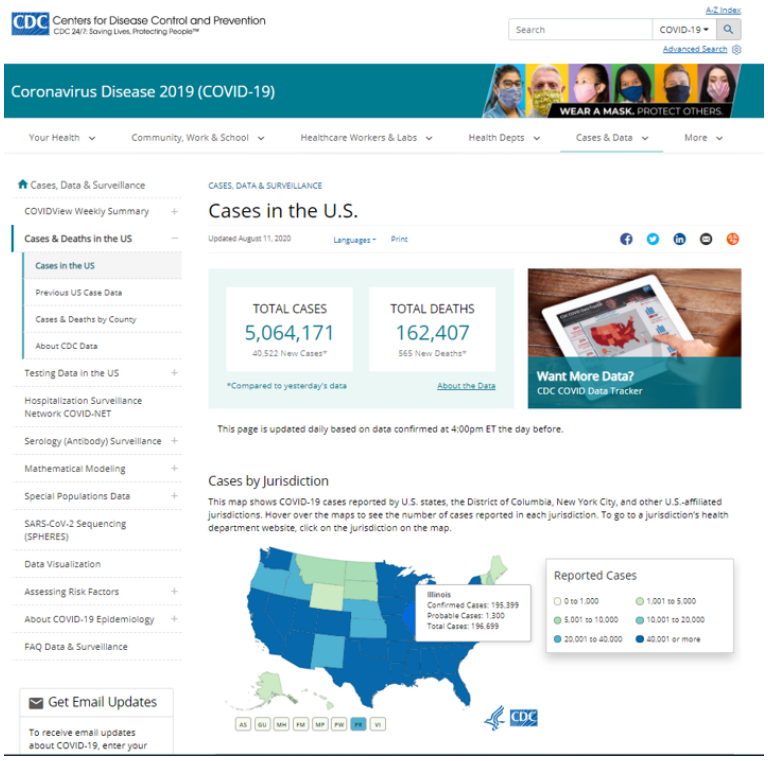
| | | | T1 (low search costs, high-value information) vs T3 | | | |
|---|---|---|---|---|---|---|
| Controls | Outcome | Coef. (s.e.) | Unadj. $p$ | Sharpened $q$ (3) | Sharpened $q$ (6) | Romano-Wolf $p$ (3) |
| No | EPA improves lives | 0.050 (0.030) | 0.093 | 0.055 | 0.142 | 0.095 |
| No | EPA independent | 0.062 (0.025) | 0.013 | 0.042 | 0.087 | 0.045 |
| No | Support for EPA | 0.063 (0.030) | 0.035 | 0.042 | 0.095 | 0.066 |
| Yes | EPA improves lives | 0.023 (0.029) | 0.415 | 0.179 | 0.497 | 0.434 |
| Yes | EPA independent | 0.046 (0.025) | 0.063 | 0.179 | 0.435 | 0.170 |
| Yes | Support for EPA | 0.048 (0.029) | 0.101 | 0.179 | 0.435 | 0.178 |
| | | | T2 (high search costs, high-value information) vs T3 | | | |
| No | EPA improves lives | 0.018 (0.029) | 0.543 | 0.382 | 0.284 | 0.552 |
| No | EPA independent | 0.034 (0.024) | 0.171 | 0.382 | 0.173 | 0.399 |
| No | Support for EPA | 0.039 (0.030) | 0.184 | 0.382 | 0.173 | 0.399 |
| Yes | EPA improves lives | 0.012 (0.028) | 0.677 | 0.795 | 0.795 | 0.679 |
| Yes | EPA independent | 0.025 (0.024) | 0.295 | 0.795 | 0.435 | 0.564 |
| Yes | Support for EPA | 0.033 (0.029) | 0.249 | 0.795 | 0.435 | 0.564 |

*Notes*: Coefficients, classical standard errors and unadjusted p-values reproduce Table 2, cols (3)–(8) (N = 1,626; 1,613; 1,580). *Sharpened q (3)*: Anderson (2008) q-values within the family of three trust outcomes for the given contrast and control set. *Sharpened q (6)*: the same, pooling the T1 and T2 contrasts into one family of six. *Romano-Wolf p (3)*: step-down adjusted p-values within the family of three, 2,000 bootstrap replications, seed 20260915.

# A4 Study I

### A4.0.1 Design

Figure A1: Randomized treatment




*Notes*: The figure shows the webpage that treated respondents would land on if they clicked on the link.

### A4.0.2 Controls

**Economic shocks**. To measure direct economic shocks, we asked all respondents in the last wave of the survey to report their (and their spouse, if present) monthly gross income between February and October[29]. Furthermore, we also asked respondents (and their spouses, if present) about their monthly additional sources of income, the number of hours they worked each month, and whether they had received any financial support from the government or non-governmental organizations at any time during the crisis. This data enables us to estimate both the timing and magnitude of the economic shocks experienced by respondents' households between waves. We measure direct income shocks in two distinct ways, and we demonstrate that they yield comparable results. In our main specification, we flag respondents whose household income (combining income from work and other sources) fell

[29]In addition to asking in most waves whether respondents incurred any economic or health shock, in the last wave, we asked them to report the exact amount of household income for every month as well as if they knew anyone hospitalized each month. This allows us to have a more granular and quantifiable measure of economic shock beyond the timing of our survey waves

by more than 20% from one month to the next, to capture the effects of a sudden large drop in income:

$$shock_{iw} = \begin{cases} 1, & \text{if } \dfrac{income_{it} - income_{i,t-1}}{income_{i,t-1}} < -0.20 \text{ for at least one month } t \in \mathcal{T}_w, \\ 0, & \text{otherwise,} \end{cases}$$

where $t-1$ is the calendar month preceding $t$ and $\mathcal{T}_w$ is the set of months up to the wave $w$ in which the outcome is measured: April to June for outcomes measured in June (wave 6), and April to October for outcomes measured in October (wave 7). February is the pre-shock baseline, so the first month-on-month change we observe is March relative to February. About 38% of respondents experienced at least one such drop in a month between March and October 2020[30].

In addition to measures of personal economic shocks, we also control for indirect economic shocks since it is possible that many Americans changed their preferences just by mere exposure to the crisis, such as by knowing someone who was affected economically by the crisis or living in an area that suffered relatively higher economic distress compared to others (Dyer, 2020; Wright et al., 2020). We use data collected and updated in real-time by Harvard's Opportunity Insights team on the weekly percentage variations in consumer expenditures with respect to the first week of January 2020 (Chetty et al., 2020). This variable is seasonally adjusted and is available at the county level, which we match with the respondents' residential information[31].

**Health shocks**. Our main measure of direct health shock is whether respondents had a family member, a friend, or an acquaintance who was hospitalized with COVID-19[32]. About 30% of our respondents knew someone (among family, friends, or acquaintances) who was hospitalized with COVID-19, while 69% knew someone who tested positive. About 33% tested positive for COVID-19 themselves.

We complement these survey-based measures with data on COVID-19 cases in the respondents' county of residence. While this measure might be subject to different data collection and testing regimes across States, these figures were likely to be the same ones reported by the local media. To improve accuracy, we consider COVID-19 cases[33] at the

[30]Respondents who experienced such a drop between March and October 2020 are more likely to be young, with a low baseline income, and to belong to a racial minority group. Furthermore, women, Democrats, and those who live in a metropolitan area have incurred such a negative income shock with a significantly higher probability, while cohabitation (or marriage) seems to smooth the financial impact of the pandemic. We control for all these characteristics in our analysis and show how using different specifications does not change our main results

[31]The Opportunity Insights team uses anonymized data from several private companies to construct public indices of consumer spending, employment, and other outcomes. See Chetty et al. (2020) for further information on series construction.

[32]We consider this combined measure, as 2.4% of the respondents have a family member who has been hospitalized, 9.8% have a relative, 14.1% a friend, and 14.9% an acquaintance. To control for additional direct health shocks, we also asked respondents their type of health insurance (e.g., public or private), whether they have caring responsibilities towards an elderly or someone with disabilities, who are at greater risk of complications from contracting the virus, and if they knew a healthcare professional who had been working closely with COVID-19 patients

[33]We exploited the data collected by the New York Times from state and local governments and health

county level reported by the middle of each week. We then consider the population size at the county level in 2019 and construct the following measure of increase in cases between week $t$ and *t-1* in county $c$: $\frac{cases_{ct}-cases_{ct-1}}{population_{c2019}}$[34]. When, instead, we consider an outcome that is not in change, we focus on the logarithm of the cumulative number of cases weighted by the county population: $ln\left(\frac{cases_{ct}}{population_{c2019}} * 100,000\right)$.

**Media consumption** We consider all the news sources consumed by respondents and assess whether they are overall slanted in a direction or not. Therefore, we collected information on respondents' preferred news sources (including international news and social media) and the number of hours of news they consumed[35]. Based on the news sources they indicated, we constructed a "slant score" using the "*AllSides.com*" platform, one of the most commonly used sources of partisan media slant analysis[36]. The website assigns a score from 1 (Extremely Left) to 5 (Extremely Right) to major sources of news by analyzing their written content on a regular basis and running polls. Matching the scores by Allsides[37] to the respondents' choices, we create an index summing the scores of each source consulted by an individual and divided by the maximum number of possible points.

$$\textit{Media slant index (N sources of news)} = \frac{\sum_{n=1}^{N} score_n}{N}$$

This variable measures how politically homogeneous the news sources that respondents consumed are, by taking any value between 1 and 5: the closer a respondent is to 1, the more they consume homogeneous (i.e., less politically diversified) left-leaning media, while the closer they are to 5 the more homogeneous and right-leaning is their media consumption. A score towards the middle indicates either that respondents consume unbiased news, or that they consume news that is slanted in both directions and so that they are exposed to both partisan narratives. Based on this specification, we see that 51% of the Republicans consume Republican-leaning news, and 46% of the Democrats consume Democratic-leaning news. Among independents and non-voters, around 25% (24%) consume Republican-leaning news (Democratic-leaning news).

**Other controls.** The baseline controls for the regressions are: Gender, Age (in four generations: Gen Z, Gen X, Millennials, Baby Boomers, and above), Education (in four groups: Less than high school, High school or equivalent, Some college, Bachelor or more), Income (in quintiles), Race (White, African Americans, Hispanics, and other), whether respondents are cohabitating with someone else, whether they have children under the age of 18, whether they were in financial difficulties already prior the pandemic, whether they are in the labor force, whether they were unemployed in February (before the pandemic),

departments, and available here `https://github.com/nytimes/covid-19-data`.

[34]We multiply this measure by 100 to ease the interpretation of the coefficients in our regressions

[35]The question asked: "Do you get your news from any of these sources (either on television or on the internet)?", and the multiple option answers were: "ABC, CBS, NBC, CNN, Fox News, MSNBC, and 'other, please specify"' (e.g., some respondents added The NY Times, The Washington Post, BBC, NPR, and PBS). While there is no exact methodology to measure the partisan slant of news sources (Budak et al., 2016; Groseclose and Milyo, 2005), and since within each source different programs might cover the same news in different tones (Bursztyn et al., 2020), we measured whether respondents were exposed to different points of view during the crisis.

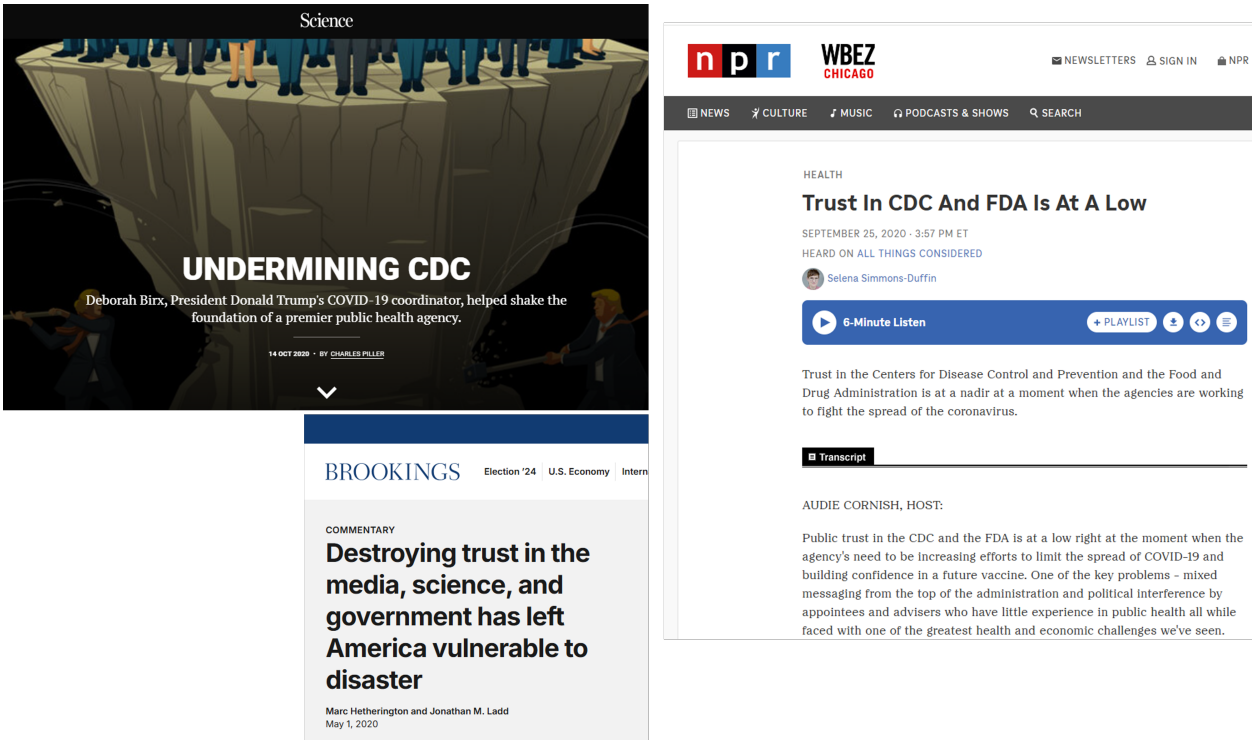
[36]`https://www.allsides.com/unbiased-balanced-news`

[37]We use the scores of the first week of April 2020, our baseline wave.

the U.S. region (South, North-East, Mid-west, West), whether they live in a metropolitan area, whether they have health insurance and whether it's public or private, the population density of their zip code of residence, whether they are Catholic, or Protestant, and whether they have caring responsibilities. We also control for whether respondents completed the survey in a shorter time than the $99^{th}$ percentile.

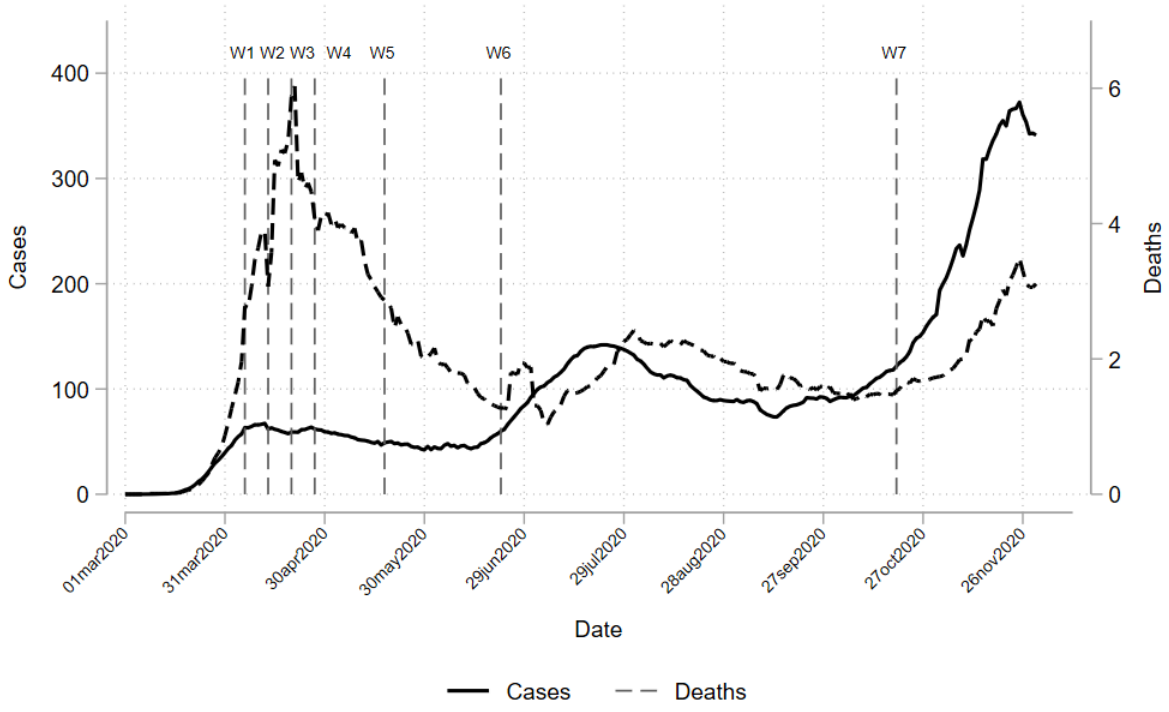
### A4.0.3 Public institution media coverage

Figure A2: Media coverage about the CDC



### A4.0.4 Survey timing

Figure A3: Timing of the longitudinal survey waves against health indicators of the COVID-19 pandemic




*Notes*: The figure shows the timing of the seven survey waves implemented between early April and the end of October 2020 against the curves of the 7-day rate of COVID-19 cases and deaths per 100,000 inhabitants, allowing for comparisons between areas with different population sizes in the United States. The figure is based on the U.S. Centers for Disease Control and Prevention publicly available data.

### A4.0.5 Sample characteristics

Table A8: Summary statistics of the key demographics in each survey wave, applying survey weights.

| | Wave1 | Wave2 | Wave3 | Wave4 | Wave5 | Wave6 | Wave7 |
|---|---|---|---|---|---|---|---|
| Republican | 27.37 | 25.33 | 26.24 | 25.94 | 25.55 | 24.53 | 26.47 |
| Democrat | 37.93 | 37.19 | 35.87 | 37.41 | 36.53 | 36.2 | 35.94 |
| Independent & other | 34.71 | 37.48 | 37.88 | 36.64 | 37.92 | 39.27 | 37.59 |
| Woman | 51.69 | 51.68 | 51.68 | 51.69 | 51.68 | 51.68 | 51.7 |
| Age: 18-29 | 20.51 | 20.5 | 20.5 | 20.5 | 20.5 | 20.5 | 20.53 |
| Age: 30-44 | 24.71 | 24.83 | 25.52 | 25.97 | 25.36 | 25.45 | 25.5 |
| Age: 45-59 | 25.02 | 24.9 | 24.21 | 23.76 | 24.38 | 24.29 | 24.22 |
| Age: 60+ | 29.76 | 29.77 | 29.77 | 29.77 | 29.77 | 29.77 | 29.75 |
| Less than HS | 9.77 | 9.77 | 9.77 | 9.77 | 9.77 | 9.77 | 9.77 |
| High school | 28.26 | 28.26 | 28.26 | 28.26 | 28.26 | 28.26 | 28.25 |
| Some college | 27.71 | 27.7 | 27.7 | 27.7 | 27.69 | 27.7 | 27.73 |
| Bachelor + | 34.27 | 34.27 | 34.27 | 34.27 | 34.28 | 34.27 | 34.26 |
| I income q | 22.63 | 22.89 | 23.89 | 24.37 | 23.5 | 24.41 | 26.35 |
| II income q | 21.35 | 21.15 | 20.47 | 20.4 | 21.48 | 19.99 | 17.86 |
| III income q | 17.99 | 17.28 | 17.23 | 17.65 | 16.39 | 17.64 | 17.9 |
| IV income q | 18.98 | 19.37 | 19.6 | 18.66 | 19.7 | 19.29 | 18.69 |
| V income q | 19.05 | 19.3 | 18.81 | 18.93 | 18.93 | 18.66 | 19.21 |
| Financial hardship pre-COVID-19 | 31.52 | 31.04 | 31.89 | 31.33 | 31.14 | 31.75 | 31.38 |
| African American | 11.93 | 11.93 | 11.93 | 11.93 | 11.93 | 11.93 | 11.93 |
| Hispanic | 16.67 | 16.67 | 16.67 | 16.67 | 16.67 | 16.67 | 16.66 |
| Other Race | 8.59 | 8.58 | 8.58 | 8.58 | 8.58 | 8.58 | 8.62 |
| White | 62.81 | 62.82 | 62.82 | 62.82 | 62.82 | 62.82 | 62.79 |
| Cohabitating | 54.58 | 57.9 | 57.36 | 57.98 | 58.55 | 62.43 | 62.68 |
| Parent of minor | 27.87 | 26.64 | 27.57 | 27.13 | 26.68 | 26.61 | 27.07 |
| Caring responsibilities | 16.48 | 15.95 | 16.65 | 16.21 | 15.87 | 16.42 | 16.37 |
| Not in the labor force | 27.68 | 27.14 | 28.18 | 27.75 | 26.91 | 27.24 | 26.99 |
| Unemployed in Feb | 9.81 | 8.96 | 10.8 | 9.73 | 9.06 | 8.83 | 9.56 |
| North-East | 17.45 | 17.45 | 17.45 | 17.45 | 17.45 | 17.45 | 17.44 |
| Midwest | 20.74 | 20.74 | 20.74 | 20.74 | 20.74 | 20.74 | 20.73 |
| South | 37.98 | 37.97 | 37.97 | 37.97 | 37.97 | 37.97 | 38 |
| West | 23.84 | 23.84 | 23.84 | 23.84 | 23.84 | 23.84 | 23.83 |
| Metropolitan area | 85.49 | 85.1 | 86.07 | 84.78 | 85.29 | 86.07 | 85.49 |
| No health insurance | 6.67 | 6.39 | 6.54 | 5.89 | 6.6 | 6.29 | 6.24 |
| Population density (ZCTA) | 371350.9 | 368034.6 | 379836.2 | 380862.6 | 380720.3 | 373379.7 | 383178.7 |
| $N$ | 1441 | 1228 | 1177 | 1219 | 1199 | 1192 | 1076 |

Table A9: Attrition predictors.

| VARIABLES | (1) Attrition W1-W7 | (2) Attrition W6-W7 | | cont.(1) Attrition W1-W7 | cont. (2) Attrition W6-W7 |
|---|---|---|---|---|---|
| Treated | | -0.00375 | Cohabitating | -0.309*** | -0.0847*** |
| | | (0.0214) | | (0.0256) | (0.0272) |
| $\frac{log(Covid\ cases\ w7)}{100{,}000}$ | -0.0347* | -0.0424** | Parent | 0.0818*** | -0.0190 |
| | (0.0209) | (0.0212) | | (0.0280) | (0.0295) |
| Woman | 0.0371* | 0.0404* | Caring resp. | 0.00203 | 0.0398 |
| | (0.0216) | (0.0218) | | (0.0311) | (0.0323) |
| Age: 18-29 | 0.0601 | 0.107*** | Not labor force | 0.0567* | 0.0498 |
| | (0.0390) | (0.0411) | | (0.0301) | (0.0307) |
| Age: 30-44 | -0.00882 | 0.0239 | Unemployed in Feb | 0.0888* | 0.0656 |
| | (0.0367) | (0.0378) | | (0.0483) | (0.0521) |
| Age: 45-59 | 0.0191 | 0.0341 | Midwest | -0.0403 | -0.0251 |
| | (0.0353) | (0.0358) | | (0.0362) | (0.0362) |
| High school | -0.127* | -0.0940 | South | -0.00920 | -0.0224 |
| | (0.0735) | (0.0774) | | (0.0349) | (0.0357) |
| Some college | -0.0885 | -0.0346 | West | 0.0135 | -0.000535 |
| | (0.0714) | (0.0757) | | (0.0364) | (0.0371) |
| Bachelor + | -0.171** | -0.0929 | Metro area | -0.00353 | 0.0182 |
| | (0.0723) | (0.0765) | | (0.0327) | (0.0322) |
| II income q | 0.219*** | 0.147*** | Gov health insur | 0.0201 | -0.00581 |
| | (0.0352) | (0.0353) | | (0.0285) | (0.0293) |
| III income q | 0.283*** | 0.185*** | No health insur | 0.0756* | 0.0893* |
| | (0.0353) | (0.0370) | | (0.0454) | (0.0488) |
| IV income q | 0.300*** | 0.188*** | Pop density | -7.32e-07 | -6.11e-07 |
| | (0.0374) | (0.0372) | | (1.06e-06) | (9.94e-07) |
| V income q | 0.307*** | 0.145*** | Catholic | 0.00170 | 0.000326 |
| | (0.0380) | (0.0371) | | (0.0294) | (0.0307) |
| Financial issues | 0.0136 | -0.00475 | Protestant | 0.0487* | 0.0393 |
| | (0.0256) | (0.0262) | | (0.0266) | (0.0266) |
| African American | 0.0589 | 0.0699* | Rep news | -0.00566 | 0.00735 |
| | (0.0406) | (0.0424) | | (0.0290) | (0.0296) |
| Hispanic | 0.108*** | 0.105*** | Dem news | -0.0290 | -0.0286 |
| | (0.0358) | (0.0373) | | (0.0269) | (0.0264) |
| Other Race | 0.00834 | 0.0137 | Constant | 0.480*** | 0.374** |
| | (0.0363) | (0.0374) | | (0.179) | (0.184) |
| Observations | 1,442 | 1,193 | | 1,442 | 1,193 |
| R-squared | 0.185 | 0.088 | | 0.185 | 0.088 |
| Mean dep. var. | 0.254 | 0.170 | | 0.254 | 0.170 |

Robust standard errors in parentheses; *** p<0.01, ** p<0.05, * p<0.1

### A4.0.6 Motivating evidence

**Fact I: Institutional trust has declined and has not recovered**

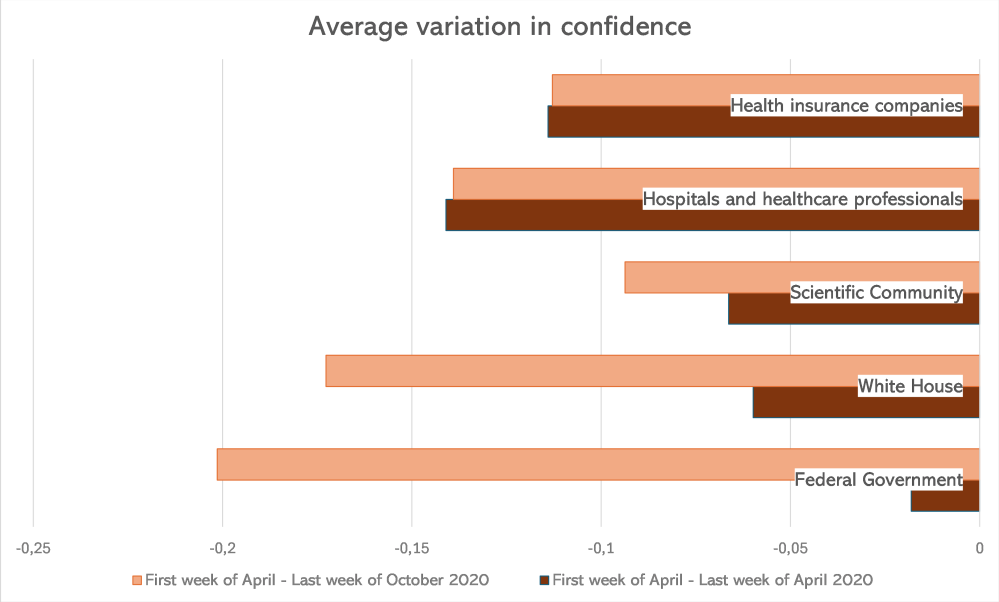



Figure A4: Variation in self-reported institutional trust (on a 5-point likert scale, with 1 being the lowest and 5 the highest score) across waves.

**Fact II: Trust in public health institutions predicts the adoption of preventive behaviors both in levels and trends, while trust in the government does not.** List of preventive behaviors:

- I don't leave home unless strictly needed
- I sneeze or cough into a tissue or the inside of my elbow
- I use pick-up or delivery options more frequently
- I keep a safe distance from others
- I try not to touch objects when I am outside
- I wash my hands more thoroughly and more frequently
- I cover my mouth in public (with a mask, a scarf, ...)

Table A10: Adoption of preventive behaviors and institutional trust

| | (1) % prev. behav. adopted | (2) % prev. behav. adopted | (3) % prev. behav. adopted | (4) Δ % prev. behav. adopted w1 -w3 | (5) Δ % prev. behav. adopted w1-w3 | (6) Δ % prev. behav. adopted w1-w3 | (7) Use mask w7 | (8) Use mask w7 |
|---|---|---|---|---|---|---|---|---|
| Trust index PIs w1 | 0.198*** | | 0.173** | | | | | |
| | (0.0728) | | (0.0756) | | | | | |
| Trust index gov w1 | | 0.114** | 0.0638 | | | | | |
| | | (0.0532) | (0.0531) | | | | | |
| Δ trust index PIs w1-w4 | | | | 0.144*** | | 0.117** | | |
| | | | | (0.0500) | | (0.0582) | | |
| Δ trust index gov. w1-w4 | | | | | 0.0661 | 0.0288 | | |
| | | | | | (0.0790) | (0.0878) | | |
| Trust index PIs w7 | | | | | | | 0.266** | 0.344** |
| | | | | | | | (0.129) | (0.134) |
| Trust index gov w7 | | | | | | | | -0.264*** |
| | | | | | | | | (0.0909) |
| Constant | 0.508*** | 0.570*** | 0.491*** | 0.129* | 0.150** | 0.150** | 0.549*** | 0.626*** |
| | (0.0599) | (0.0530) | (0.0632) | (0.0679) | (0.0657) | (0.0680) | (0.147) | (0.142) |
| Observations | 1,421 | 1,433 | 1,417 | 1,082 | 1,088 | 1,073 | 1,063 | 1,059 |
| R-squared | 0.175 | 0.168 | 0.176 | 0.112 | 0.104 | 0.108 | 0.216 | 0.230 |
| Demographics | Yes | Yes | Yes | Yes | Yes | Yes | Yes | Yes |
| Covid shock | Yes | Yes | Yes | Yes | Yes | Yes | Yes | Yes |
| News | Yes | Yes | Yes | Yes | Yes | Yes | Yes | Yes |
| Average dep. var. | 0.709 | 0.709 | 0.709 | 0.0344 | 0.0344 | 0.0344 | 0.872 | 0.872 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In col (1)-(3) the outcome is the fraction of the preventive behaviors adopted from the list above; in cols (4-6), the outcomes is the variation in this fraction between the first and the third week of April; in cols (7) and (8), we flag whether respondents declare to wear a mask in May. All regressions take into account population survey weights and the sampling procedure. The control variables are reported in the Appendix.

## Fact III: Perceived politicization predicts drops in trust, both self-reported and factual

Table A11: Perceived polarization and trust

| | (1) Decrease trust sci. comm. (w7-w1) | (2) Self-reported decrease trust sci. comm. | (3) Decrease trust hospitals (w1-w7) | (4) Self-reported decrease trust hospitals | (5) Decrease trust health insur. (w7-w1) | (6) Self-reported decrease trust health insur. (w7-w1) | (7) Self-reported decr. trust CDC |
|---|---|---|---|---|---|---|---|
| Sci. comm. politicized | 0.122** | 0.372*** | | | | | |
| | (0.0503) | (0.0370) | | | | | |
| Hospitals politicized | | | 0.160*** | 0.264*** | | | |
| | | | (0.0400) | (0.0448) | | | |
| Health insur. politicized | | | | | 0.0866** | 0.180*** | |
| | | | | | (0.0397) | (0.0398) | |
| CDC politicized | | | | | | | 0.399*** |
| | | | | | | | (0.0338) |
| Rep. news | 0.0857 | 0.132*** | 0.129*** | 0.0583** | 0.0625 | -0.0287 | 0.104*** |
| | (0.0540) | (0.0305) | (0.0473) | (0.0295) | (0.0601) | (0.0363) | (0.0388) |
| Dem. news | -0.0275 | -0.0185 | 0.0168 | -0.0239 | 0.0527 | -0.0117 | 0.0849** |
| | (0.0490) | (0.0281) | (0.0477) | (0.0234) | (0.0434) | (0.0409) | (0.0351) |
| Constant | 0.258 | 0.113 | 0.337** | 0.133 | 0.0745 | -0.0203 | 0.0853 |
| | (0.172) | (0.103) | (0.162) | (0.115) | (0.159) | (0.138) | (0.120) |
| Observations | 1,004 | 1,012 | 1,009 | 1,014 | 1,008 | 1,009 | 1,005 |
| R-squared | 0.091 | 0.243 | 0.117 | 0.166 | 0.132 | 0.114 | 0.249 |
| Demographics | Yes | Yes | Yes | Yes | Yes | Yes | Yes |
| Covid shocks | Yes | Yes | Yes | Yes | Yes | Yes | Yes |
| Average decrease | 0.286 | 0.148 | 0.306 | 0.0827 | 0.284 | 0.187 | 0.287 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In col. (1) the outcome is a dummy = 1 if the respondent decreased their confidence in the scientific community between wave 7 and wave 1; in col. (2) the outcome is a dummy flagging the self-reported variation in confidence in the scientific community: the dummy =1 if the respondent reports having decreased their confidence in the scientific community from the beginning of the pandemic. Cols (3)-(6) repeat the same exercise but focus on hospitals and healthcare professionals (cols. 3-4) and health insurance companies (cols. 5-6). The outcome of col. (7) is a dummy = 1 if the respondent declared having decreased their confidence in the CDC. All regressions take into account population survey weights and the sampling procedure. The control variables are reported in the Appendix.

### Fact IV: Media consumption predicts the perception of the Covid-19 crisis

In the fifth wave of the survey (week of May 18th), we asked respondents to forecast the COVID-19 death rate in the U.S. by the end of the year, after presenting them with the latest official death rate from the CDC and asked for their judgment on how the government handled the crisis[38]. Our goal was to understand the impact of partisanship on perceptions, partially controlling for a possible information gap by providing the latest official statistics[39].

[38]The questions asked: *By May 17, the U.S. Centers for Disease Control and Prevention (CDC) stated that about 90,000 Americans have so far died from COVID-19 (coronavirus). In addition to this, how many more Americans do you think will die by the end of this year due to coronavirus?*

[39]Gaines et al. (2007) studies a similar setting showing results of a survey where Americans were asked to state the need and support for the Iraqi war in 2003: while the majority of all respondents thought it was unlikely that the U.S. would ever find weapons of mass destruction, Democrats were more likely to conclude that they simply did not exist while Republicans were more likely to state that they believed the Iraqi government moved or destroyed the weapons.

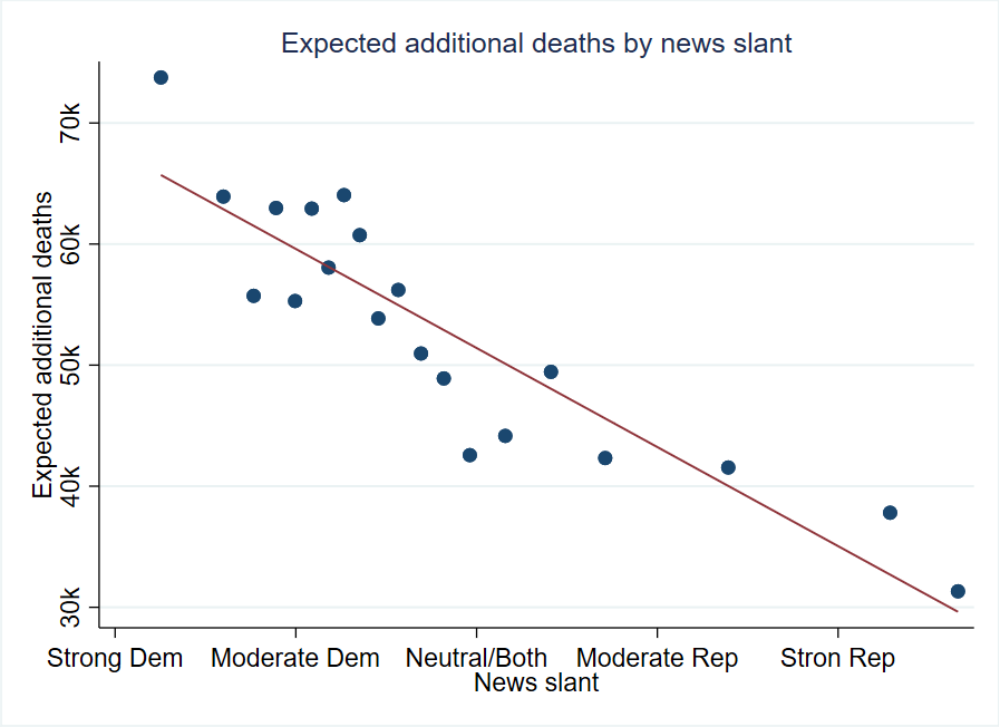



Figure A5: Expected additional death by news slant: Binscatter, controlling for a set of controls.

*Notes*: The figure shows a binned scatterplot in which the x-axis variable (news slant) is grouped into equal-sized bins, and the means of the x- and y-axis within each bin are computed. The plot controls for the set of variables described in the notes of table A12.

Table A12: News consumption predicts different perceptions of the pandemic

| | (1) Expected additional deaths | (2) Expected additional deaths | (3) Expected additional deaths |
|---|---|---|---|
| Index news slant | -51,825*** | | |
| | (5,037) | | |
| Democratic leaning news | | 14,624*** | 14,393*** |
| | | (2,790) | (2,799) |
| Republican leaning news | | -15,305*** | -14,930*** |
| | | (2,763) | (2,771) |
| Any direct shock Apr-May | 4,772* | 5,727** | 5,168* |
| | (2,860) | (2,825) | (2,782) |
| Few COVID-19 news | | | -7,030*** |
| | | | (2,056) |
| Constant | 54,109*** | 30,236*** | 36,097*** |
| | (9,912) | (10,110) | (10,502) |
| Observations | 1,184 | 1,184 | 1,184 |
| R-squared | 0.195 | 0.210 | 0.218 |
| Controls | Yes | Yes | Yes |
| Mean dep. var. | 49827 | 49827 | 49827 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In col (1)-(3) the outcome is the expected number of additional deaths from COVID-19 that the respondents were expecting would occur in the U.S., between May and the end of the year. All regressions take into account population survey weights and the sampling procedure. The control variables are specified in the Appendix.

### A4.0.7 Balance Table

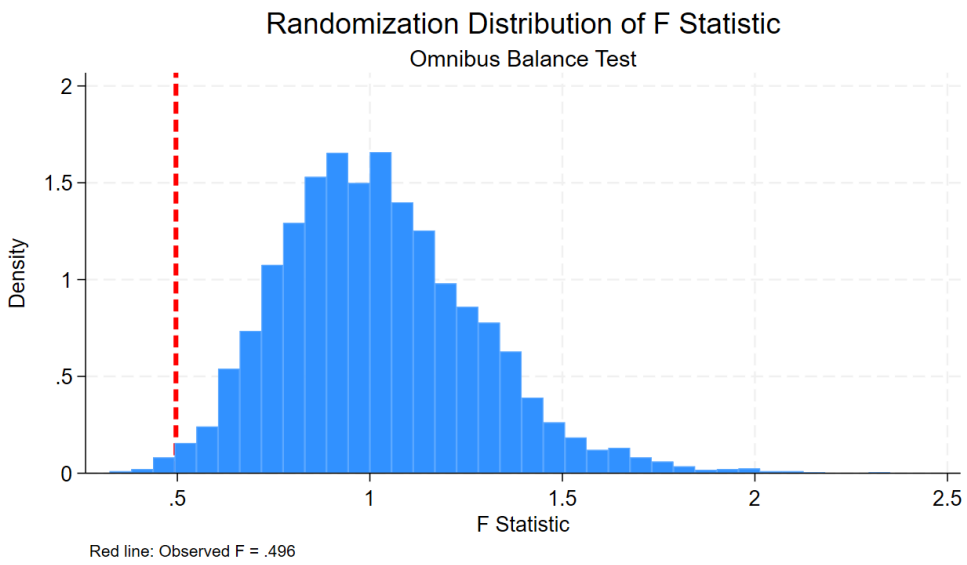



Figure A6: The histogram shows the randomization (null) distribution of the F-statistic obtained by repeatedly reassigning the treatment status according to the experimental design and re-estimating a logit of treatment assignment on baseline covariates, following Kerwin et al. (2024). The red dashed line indicates the F-statistic computed from the observed assignment. This test evaluates whether baseline covariates jointly predict treatment assignment more strongly than would be expected by chance under randomization. Because the observed F-statistic lies well within the bulk of the randomization distribution, we fail to reject the null hypothesis of joint orthogonality between treatment assignment and baseline covariates, indicating no evidence of imbalance beyond chance variation.

**Balance table**

Table A13: Balance table.

| Variable | (1) Control group | (2) Treatment group | (3) Difference |
|---|---|---|---|
| Woman | 0.483 | 0.479 | -0.004 |
| | (0.500) | (0.500) | (0.029) |
| Age: 18-29 | 0.207 | 0.216 | 0.008 |
| | (0.406) | (0.412) | (0.024) |
| Age: 30-44 | 0.282 | 0.286 | 0.004 |
| | (0.450) | (0.452) | (0.026) |
| Age: 45-59 | 0.219 | 0.224 | 0.006 |
| | (0.414) | (0.417) | (0.024) |
| High school | 0.150 | 0.159 | 0.009 |
| | (0.357) | (0.366) | (0.021) |
| Some college | 0.392 | 0.400 | 0.008 |
| | (0.488) | (0.490) | (0.028) |
| Bachelor + | 0.424 | 0.414 | -0.010 |
| | (0.495) | (0.493) | (0.029) |
| II income q | 0.188 | 0.190 | 0.002 |
| | (0.391) | (0.392) | (0.023) |
| III income q | 0.189 | 0.209 | 0.019 |
| | (0.392) | (0.407) | (0.023) |
| IV income q | 0.194 | 0.219 | 0.025 |
| | (0.396) | (0.414) | (0.023) |
| V income q | 0.215 | 0.191 | -0.024 |
| | (0.411) | (0.394) | (0.023) |
| Financial hardship | 0.287 | 0.255 | -0.032 |
| | (0.453) | (0.436) | (0.026) |
| African American | 0.100 | 0.095 | -0.005 |
| | (0.300) | (0.293) | (0.017) |
| Hispanic | 0.150 | 0.152 | 0.002 |
| | (0.357) | (0.359) | (0.021) |
| Other Race | 0.116 | 0.128 | 0.012 |
| | (0.320) | (0.334) | (0.019) |
| Cohabitating | 0.626 | 0.614 | -0.013 |
| | (0.484) | (0.487) | (0.028) |
| Parent of minor | 0.263 | 0.286 | 0.024 |
| | (0.440) | (0.452) | (0.026) |
| Caring responsibilities | 0.152 | 0.169 | 0.017 |
| | (0.359) | (0.375) | (0.021) |
| Not in the labor force | 0.256 | 0.248 | -0.008 |
| | (0.437) | (0.432) | (0.025) |
| Unemployed in Feb | 0.059 | 0.053 | -0.005 |
| | (0.235) | (0.225) | (0.013) |
| Midwest | 0.268 | 0.257 | -0.011 |
| | (0.443) | (0.437) | (0.026) |
| South | 0.357 | 0.352 | -0.006 |
| | (0.480) | (0.478) | (0.028) |
| West | 0.235 | 0.234 | -0.000 |
| | (0.424) | (0.424) | (0.025) |
| Metropolitan area | 0.856 | 0.876 | 0.019 |
| | (0.351) | (0.330) | (0.020) |
| Government provided health | 0.359 | 0.338 | -0.021 |
| | (0.480) | (0.473) | (0.028) |
| No health insurance | 0.073 | 0.078 | 0.004 |
| | (0.261) | (0.268) | (0.015) |
| Population density in ZCTA | 3,895.589 | 3,765.419 | -130.169 |
| | (9,688.460) | (8,374.398) | (525.591) |
| Catholic | 0.171 | 0.191 | 0.020 |
| | (0.377) | (0.394) | (0.022) |
| Protestant | 0.276 | 0.278 | 0.002 |
| | (0.447) | (0.448) | (0.026) |
| Any direct shock Apr-Jun | 0.276 | 0.295 | 0.019 |
| | (0.447) | (0.456) | (0.026) |
| log county cases/100,000 | 6.117 | 6.129 | 0.012 |
| | (0.954) | (0.957) | (0.055) |
| Var consumer spending by | -0.097 | -0.106 | -0.009 |
| | (0.104) | (0.120) | (0.007) |
| Observations | 613 | 580 | 1,193 |

*Notes*: Standard deviations in parentheses in cols (1–2); standard errors in col (3). *** p<0.01, ** p<0.05, * p<0.1. Cols (1–2): unweighted means by treatment arm; col (3): OLS coefficient from a regression of each characteristic on the treatment indicator. Sample: the 1,193 respondents who took part in wave 6, when treatment was assigned.

### A4.0.8 Additional Results

Table A14: Seconds spent on treatment questions (number of deaths in the state and in the U.S.).

| VARIABLES | (1) Seconds spent on treatment questions | (2) Seconds spent on treatment questions |
|---|---|---|
| Informative T | 13.97** | 14.57** |
| | (5.375) | (5.766) |
| Constant | 54.05*** | 90.61*** |
| | (4.185) | (33.20) |
| Observations | 1,167 | 1,167 |
| R-squared | 0.017 | 0.095 |
| Demographics | No | Yes |
| Covid shock | No | Yes |
| News | No | Yes |
| Mean dep. var. | 58.95 | 58.95 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. The outcome is the time spent answering the questions with the treatment intervention. All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

Table A15: Two-Stage Least Squares instrumenting correctly selecting the number of COVID-19 deaths in the US and at the State level with the treatment status. Effects on trust and long-term knowledge.

| VARIABLES | (1) TSLS Correct US death rank | (2) TSLS Correct US death rank | (3) TSLS Incr. trust in CDC | (4) TSLS Incr. trust in CDC | (5) TSLS CDC not infl. by politics | (6) TSLS CDC not infl. by politics |
|---|---|---|---|---|---|---|
| Correct deaths | 0.493 | 0.464* | 0.655 | 0.510* | 0.745 | 0.551* |
| | (0.389) | (0.276) | (0.445) | (0.295) | (0.474) | (0.280) |
| Constant | 0.374** | 0.571 | -0.0804 | -0.115 | -0.164 | -0.657* |
| | (0.144) | (0.544) | (0.157) | (0.432) | (0.166) | (0.367) |
| Observations | 953 | 953 | 949 | 949 | 960 | 960 |
| Demographics | No | Yes | No | Yes | No | Yes |
| Covid shock | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes |
| First-stage F | 6.245 | 13.50 | 6.245 | 13.50 | 6.245 | 13.50 |
| First-stage p | 0.0135 | 0.0003 | 0.0135 | 0.0003 | 0.0135 | 0.0003 |
| Mean dep. var. | 0.550 | 0.550 | 0.162 | 0.162 | 0.113 | 0.113 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. We instrument correctly answering the knowledge accuracy question in June with the treatment status. In cols. (1-2), the outcome is whether respondents correctly ranked the U.S. in terms of COVID-19 deaths; in cols (3-4), whether respondents report having increased their confidence in the CDC since the beginning of the pandemic; in cols (5-6), whether respondents report that the CDC is not influenced by politics. All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

Table A16: Treated respondents are more likely to have tested for Covid-19 and have adopted at least a preventive behavior, but no significant effects on mask-wearing.

| VARIABLES | (1) Tested for Covid-19 | (2) Tested for Covid-19 | (3) Adopts no prev. behav. | (4) Adopts no prev. behav. | (5) %Prev. behav. adopted | (6) %Prev. behav. adopted | (7) Covers mouth in public | (8) Covers mouth in public |
|---|---|---|---|---|---|---|---|---|
| Informative T | 0.0798** | 0.0659** | -0.0241** | -0.0234** | 0.00236 | -0.000968 | 0.0215 | 0.0194 |
| | (0.0366) | (0.0309) | (0.0108) | (0.00933) | (0.0222) | (0.0197) | (0.0289) | (0.0231) |
| Constant | 0.281*** | -0.0493 | 0.0242*** | -0.0625 | 0.750*** | 0.861*** | 0.882*** | 0.824*** |
| | (0.0255) | (0.277) | (0.00886) | (0.0977) | (0.0140) | (0.143) | (0.0228) | (0.167) |
| Observations | 987 | 987 | 986 | 986 | 990 | 990 | 986 | 986 |
| R-squared | 0.008 | 0.099 | 0.045 | 0.084 | 0.064 | 0.152 | 0.053 | 0.188 |
| Demographics | No | Yes | No | Yes | No | Yes | No | Yes |
| Covid shock | No | Yes | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 0.331 | 0.331 | 0.0188 | 0.0188 | 0.735 | 0.735 | 0.872 | 0.872 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In cols (1-2) the outcome is whether respondents declared to have tested for Covid-19; in cols (3-4) whether they declared to have adopted no preventive behavior between Jun and October; in cols (5-6) the fraction of preventive behaviors adopted; in cols (7-8), whether respondents declared to have covered their mouth when in public in June-October. All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

Table A17: Two-Stage Least Squares instrumenting correctly selecting the number of COVID-19 deaths in the US and at the State level with the treatment status. Effects on preventive behaviors.

| VARIABLES | (1) TSLS Tested for Covid-19 | (2) TSLS Tested for Covid-19 | (3) TSLS Adopts no Prev. Behav. | (4) TSLS Adopts no Prev. Behav. | (5) TSLS % Adopted Prev. Behav. | (6) TSLS % Adopted Prev. Behav. | (7) TSLS Covers mouth In Public | (8) TSLS Covers mouth In Public |
|---|---|---|---|---|---|---|---|---|
| Correct deaths | 0.722 | 0.458* | -0.196 | -0.152* | -0.110 | -0.0421 | 0.112 | 0.183 |
| | (0.518) | (0.272) | (0.135) | (0.0845) | (0.237) | (0.141) | (0.293) | (0.181) |
| Constant | 0.0610 | -0.404 | 0.0813 | 0.0838 | 0.794*** | 0.841*** | 0.857*** | 0.503* |
| | (0.180) | (0.412) | (0.0510) | (0.139) | (0.0848) | (0.218) | (0.111) | (0.278) |
| Observations | 958 | 958 | 958 | 958 | 960 | 960 | 958 | 958 |
| R-squared | | | | | | 0.181 | 0.085 | 0.207 |
| Demographics | No | Yes | No | Yes | No | Yes | No | Yes |
| Covid shock | No | Yes | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 0.331 | 0.331 | 0.0188 | 0.0188 | 0.735 | 0.735 | 0.872 | 0.872 |
| First-stage F | 6.245 | 13.50 | 6.245 | 13.50 | 6.245 | 13.50 | 6.245 | 13.50 |
| First-stage p | 0.0135 | 0.000332 | 0.0135 | 0.000332 | 0.0135 | 0.000332 | 0.0135 | 0.000332 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
TSLS regression coefficients instrumenting knowledge accuracy with the treatment status. In cols (1-2) the outcome is whether respondents declared to have tested for Covid-19; in cols (3-4) whether they declared to have adopted no preventive behavior between Jun and October; in cols (5-6) the fraction of preventive behaviors adopted; in cols (7-8), whether respondents declared to have covered their mouth when in public in June-October. All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

### A4.0.9 Robustness checks

### Testing for Covid-19 Vs contracting Covid-19

Table A18: Treated respondents are more likely to test for Covid-19, but not contract the virus.

| | (1) | (2) | (3) |
|---|---|---|---|
| | Respondent tested for COVID | Respondent had COVID | Respondent had COVID |
| Treated | 0.0667** | 0.0132 | 0.0101 |
| | (0.0308) | (0.0115) | (0.00950) |
| Constant | -0.0498 | 0.0169** | -0.195*** |
| | (0.274) | (0.00705) | (0.0723) |
| Observations | 987 | 990 | 990 |
| R-squared | 0.099 | 0.003 | 0.080 |
| Demographics | Yes | No | Yes |
| Covid shock | Yes | No | Yes |
| News | Yes | No | Yes |
| Mean dep. var. | 0.320 | 0.0223 | 0.0223 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In col (1), the outcome is whether respondents declared to have been tested for Covid-19 between June and October; in cols (2-3), the outcome is whether respondents declared to have contracted Covid-19 between June and October. All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

## Placebo institutions

Table A19: The treatment has no effect on trust in placebo institutions.

| | (1) | (2) | (3) | (4) | (5) | (6) |
|---|---|---|---|---|---|---|
| | Incr. trust in scientific comm. | Incr. trust in healthcare inst. (index) | Incr. trust in financial inst. (index) | Scientific comm. not infl. by politics | Healthcare inst. not infl. by politics (index) | Financial inst. not infl. by politics (index) |
| Treated | 0.0374 | 0.0307 | -0.00108 | 0.0429 | 0.0160 | 0.00658 |
| | (0.0352) | (0.0216) | (0.0117) | (0.0268) | (0.0152) | (0.00790) |
| Constant | 0.346 | 0.257 | 0.147* | -0.0144 | 0.251** | 0.0716 |
| | (0.266) | (0.192) | (0.0887) | (0.196) | (0.123) | (0.0597) |
| Observations | 986 | 988 | 988 | 990 | 990 | 990 |
| R-squared | 0.173 | 0.148 | 0.127 | 0.105 | 0.101 | 0.142 |
| Demographics | Yes | Yes | Yes | Yes | Yes | Yes |
| Covid shock | Yes | Yes | Yes | Yes | Yes | Yes |
| News | Yes | Yes | Yes | Yes | Yes | Yes |
| Mean dep. var. | 0.205 | 0.168 | 0.0430 | 0.137 | 0.0708 | 0.0184 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In cols. (1-3), the outcome is whether respondents, between April and October, have increased trust in: (1) the scientific community ; (2) healthcare institutions (hospitals and medical staff, and health insurance companies), measured with an index; (3) economic institutions (banks and financial institutions and the private sector), measured with an index. Cols. (4-6) consider the same institutions, but focus on whether they are perceived as independent from politics. All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

## Attrition

Table A20: Heterogeneous effects by attrition predictors on the probability of correctly ranking the U.S. in terms of per-capita Covid-19 deaths.

| VARIABLES | (1) Correct US death rank | (2) Correct US death rank | (3) Correct US death rank | (4) Correct US death rank |
|---|---|---|---|---|
| Treated | 0.0479 | 0.120** | 0.0765** | 0.0891** |
| | (0.0380) | (0.0463) | (0.0329) | (0.0389) |
| Low income | 0.0843 | | | |
| | (0.0694) | | | |
| Low income*Treated | 0.0245 | | | |
| | (0.0965) | | | |
| Woman | | 0.0624 | | |
| | | (0.0558) | | |
| Woman*Treated | | -0.129* | | |
| | | (0.0716) | | |
| Hispanic | | | 0.115 | |
| | | | (0.0822) | |
| Hispanic*Treated | | | -0.150 | |
| | | | (0.0917) | |
| Age 18-29 | | | | 0.0593 |
| | | | | (0.109) |
| Age 18-29*Treated | | | | -0.178 |
| | | | | (0.110) |
| Constant | 0.941** | 1.003** | 0.990** | 0.985** |
| | (0.439) | (0.433) | (0.433) | (0.441) |
| Observations | 977 | 977 | 977 | 977 |
| R-squared | 0.082 | 0.086 | 0.084 | 0.086 |
| Demographics | Yes | Yes | Yes | Yes |
| Covid shock | Yes | Yes | Yes | Yes |
| News | Yes | Yes | Yes | Yes |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. The outcome is whether respondents correctly ranked the U.S. in terms of Covid-19 deaths. We consider whether the top 4 predictors of attrition display heterogeneous treatment effects that might drive our results. All regressions take into account population survey weights, the sampling procedure and attrition. The control variables are reported in the Appendix.

Table A21: Heterogeneous effects by attrition predictors on the probability that respondents declare to have increased their trust in the CDC.

| VARIABLES | (1) Incr. trust in CDC | (2) Incr. trust in CDC | (3) Incr. trust in CDC | (4) Incr. trust in CDC |
|---|---|---|---|---|
| Treated | 0.0654** | 0.0642* | 0.0491 | 0.0528 |
| | (0.0329) | (0.0373) | (0.0323) | (0.0344) |
| Low income | -0.0430 | | | |
| | (0.0573) | | | |
| Low income*Treated | -0.0143 | | | |
| | (0.0541) | | | |
| Woman | | 0.00242 | | |
| | | (0.0309) | | |
| Woman*Treated | | -0.00478 | | |
| | | (0.0445) | | |
| Hispanic | | | -0.0525 | |
| | | | (0.0486) | |
| Hispanic*Treated | | | 0.0865 | |
| | | | (0.0837) | |
| Age 18-29 | | | | 0.00938 |
| | | | | (0.0571) |
| Age 18-29*Treated | | | | 0.0458 |
| | | | | (0.0675) |
| Constant | 0.438 | 0.390 | 0.419 | 0.404 |
| | (0.276) | (0.277) | (0.279) | (0.283) |
| Observations | 978 | 978 | 978 | 978 |
| R-squared | 0.088 | 0.088 | 0.090 | 0.088 |
| Demographics | Yes | Yes | Yes | Yes |
| Covid shock | Yes | Yes | Yes | Yes |
| News | Yes | Yes | Yes | Yes |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1. OLS regression coefficients. The outcome is whether respondents report having increased their confidence in the CDC since the beginning of the pandemic. We consider whether the top 4 predictors of attrition display heterogeneous treatment effects that might drive our results. All regressions take into account population survey weights, the sampling procedure and attrition. The control variables are reported in the Appendix.

Table A22: Heterogeneous effects by attrition predictors on the probability that respondents declare that the CDC is independent from politics.

| VARIABLES | (1) CDC not infl. by politics | (2) CDC not infl. by politics | (3) CDC not infl. by politics | (4) CDC not infl. by politics |
|---|---|---|---|---|
| Treated | 0.0496 | 0.0426 | 0.0381 | 0.0466 |
| | (0.0327) | (0.0379) | (0.0283) | (0.0291) |
| Low income | -0.00818 | | | |
| | (0.0525) | | | |
| Low income*Treated | 0.0418 | | | |
| | (0.0682) | | | |
| Woman | | -0.0168 | | |
| | | (0.0288) | | |
| Woman*Treated | | 0.0341 | | |
| | | (0.0456) | | |
| Hispanic | | | -0.109** | |
| | | | (0.0477) | |
| Hispanic*Treated | | | 0.150** | |
| | | | (0.0640) | |
| Age 18-29 | | | | 0.00697 |
| | | | | (0.0767) |
| Age 18-29*Treated | | | | 0.0690 |
| | | | | (0.0804) |
| Constant | -0.0568 | -0.0457 | -0.0102 | -0.0356 |
| | (0.194) | (0.183) | (0.185) | (0.189) |
| Observations | 990 | 990 | 990 | 990 |
| R-squared | 0.136 | 0.136 | 0.142 | 0.137 |
| Demographics | Yes | Yes | Yes | Yes |
| Covid shock | Yes | Yes | Yes | Yes |
| News | Yes | Yes | Yes | Yes |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. The outcome is whether respondents report that politics do not influence the CDC. We consider whether the top 4 predictors of attrition display heterogeneous treatment effects that might drive our results. All regressions take into account population survey weights, the sampling procedure and attrition. The control variables are reported in the Appendix.

Table A23: Heterogeneous effects by attrition predictors on the probability that respondents declare having tested for Covid-19.

| VARIABLES | (1) Tested for Covid-19 | (2) Tested for Covid-19 | (3) Tested for Covid-19 | (4) Tested for Covid-19 |
|---|---|---|---|---|
| Treated | 0.0844** | 0.0481 | 0.0718** | 0.0794** |
| | (0.0369) | (0.0419) | (0.0312) | (0.0357) |
| Low income | 0.0320 | | | |
| | (0.0625) | | | |
| Low income*Treated | -0.0724 | | | |
| | (0.0775) | | | |
| Woman | | -0.0307 | | |
| | | (0.0552) | | |
| Woman*Treated | | 0.0347 | | |
| | | (0.0725) | | |
| Hispanic | | | 0.109 | |
| | | | (0.0774) | |
| Hispanic*Treated | | | -0.0392 | |
| | | | (0.0968) | |
| Age 18-29 | | | | 0.0461 |
| | | | | (0.0781) |
| Age 18-29*Treated | | | | -0.0674 |
| | | | | (0.0860) |
| Constant | -0.0260 | -0.00788 | -0.0263 | -0.0319 |
| | (0.275) | (0.277) | (0.280) | (0.278) |
| Observations | 987 | 987 | 987 | 987 |
| R-squared | 0.098 | 0.098 | 0.097 | 0.098 |
| Demographics | Yes | Yes | Yes | Yes |
| Covid shock | Yes | Yes | Yes | Yes |
| News | Yes | Yes | Yes | Yes |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. The outcome is whether respondents declare having tested for Covid-19 in the past months. We consider whether the top 4 predictors of attrition display heterogeneous treatment effects that might drive our results. All regressions take into account population survey weights, the sampling procedure, and attrition. The control variables are reported in the Appendix.

### Controlling for interpersonal trust

Table A24: Treatment effects on long-run knowledge and trust, controlling for baseline interpersonal trust.

| VARIABLES | (1) Correct US death rank | (2) Correct US death rank | (3) Incr. trust in CDC | (4) Incr. trust in CDC | (5) CDC not infl. by politics | (6) CDC not infl. by politics |
|---|---|---|---|---|---|---|
| Informative T | 0.0393 | 0.0530* | 0.0617* | 0.0596** | 0.0544* | 0.0596** |
| | (0.0331) | (0.0303) | (0.0316) | (0.0291) | (0.0297) | (0.0269) |
| Most people can be trusted | -0.0337 | 0.0122 | -0.0291 | -0.0239 | -0.0317 | -0.0343 |
| | (0.0482) | (0.0426) | (0.0475) | (0.0388) | (0.0435) | (0.0350) |
| Can't be too careful with people | -0.0555 | -0.0254 | -0.0833* | -0.0781** | -0.00680 | -0.0238 |
| | (0.0439) | (0.0472) | (0.0422) | (0.0380) | (0.0388) | (0.0305) |
| Constant | 0.565*** | 1.048** | 0.173*** | 0.423 | 0.0915*** | -0.0237 |
| | (0.0340) | (0.445) | (0.0335) | (0.283) | (0.0280) | (0.187) |
| Observations | 977 | 977 | 978 | 978 | 990 | 990 |
| R-squared | 0.004 | 0.083 | 0.024 | 0.095 | 0.023 | 0.137 |
| Demographics | No | Yes | No | Yes | No | Yes |
| Covid shock | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 0.550 | 0.550 | 0.162 | 0.162 | 0.113 | 0.113 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In cols (1-2), the outcome is whether respondents (in October 2020) correctly ranked the U.S. in terms of Covid-19 deaths per capita; in cols (3-4), the outcome is whether respondents (in October 2020) report having increased their confidence in the CDC since the beginning of the pandemic; in cols (5-6), the outcome is whether respondents (in October 2020) report that politics do not influence the CDC. Interpersonal trust is measured in wave 1 (April 2020); the omitted category is "it depends". All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

## Alternative specifications

Table A25: Repeating the analyses using a probit model does not affect results.

| | June (w6) | | October (w7) | | | | | |
|---|---|---|---|---|---|---|---|---|
| VARIABLES | (1) Correct death est. | (2) Correct death est. | (3) Correct US death rank | (4) Correct US death rank | (5) Incr. trust in CDC | (6) Incr. trust in CDC | (7) CDC not infl. by politics | (8) CDC not infl. by politics |
| Informative T | 0.285*** | 0.387*** | 0.103 | 0.144* | 0.266** | 0.272** | 0.286* | 0.394** |
| | (0.0891) | (0.0835) | (0.0848) | (0.0812) | (0.134) | (0.121) | (0.164) | (0.168) |
| Constant | -0.556*** | -1.235*** | 0.0708 | 1.481 | -1.158*** | 0.102 | -1.398*** | -2.999** |
| | (0.0636) | (0.451) | (0.0738) | (1.190) | (0.0853) | (1.125) | (0.125) | (1.165) |
| Observations | 1,147 | 1,146 | 977 | 977 | 971 | 964 | 990 | 984 |
| Demographics | No | Yes | No | Yes | No | Yes | No | Yes |
| Covid shock | No | Yes | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 0.330 | 0.330 | 0.550 | 0.550 | 0.162 | 0.162 | 0.113 | 0.113 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
Probit coefficients (not marginal effects), estimated with the same survey design as Table 1. In col (1-2), the outcome is whether respondents correctly estimated the number of Covid-19-related deaths in the U.S. and in their State, in June; in cols (3-4), the outcome is whether respondents (in October 2020) correctly ranked the U.S. in terms of Covid-19 deaths per capita; in cols (5-6), the outcome is whether respondents (in October 2020) report having increased their confidence in the CDC since the beginning of the pandemic; in cols (7-8), the outcome is whether respondents (in October 2020) report that the CDC is not influenced by politics. All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

Table A26: Repeating the analyses considering alternative measures of the main outcomes confirms the results.

| VARIABLES | (1) Decr. trust in CDC | (2) Decr. trust in CDC | (3) CDC is politicized | (4) CDC is politicized | (5) CDC independence scale (1-4) | (6) CDC independence scale (1-4) |
|---|---|---|---|---|---|---|
| Informative T | -0.0220 | -0.0209 | -0.0621* | -0.0658** | 0.185** | 0.199*** |
| | (0.0373) | (0.0363) | (0.0341) | (0.0298) | (0.0897) | (0.0735) |
| Constant | 0.305*** | -0.416 | 0.269*** | 0.0238 | 2.139*** | 1.797*** |
| | (0.0246) | (0.270) | (0.0263) | (0.265) | (0.0607) | (0.593) |
| Observations | 978 | 978 | 983 | 983 | 983 | 983 |
| R-squared | 0.005 | 0.110 | 0.010 | 0.106 | 0.017 | 0.137 |
| Demographics | No | Yes | Yes | Yes | Yes | Yes |
| Covid shock | No | Yes | Yes | Yes | Yes | Yes |
| News | No | Yes | Yes | Yes | Yes | Yes |
| Mean dep. var. | 0.280 | 0.280 | 0.240 | 0.240 | 2.239 | 2.239 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In col (1-2), the outcome is whether respondents (in October 2020) report having decreased their confidence in the CDC since the beginning of the pandemic; in cols (3-4), the outcome is whether respondents (in October 2020) report that the CDC is very influenced by politics; in cols (5-6), the perception of whether the CDC is influenced by politics measured with a 4-point likert scale (from "very influenced", to "not influenced at all"). All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

### Validity of meta-cognition

Table A27: Self-reported variation in confidence in an institution predicts the variation in trust recorded in previous surveys over time.

| | (1) Variation in confidence in the scientific community | (2) Variation in confidence in the scientific community | (3) Variation in confidence in hospitals and medical staff | (4) Variation in confidence in hospitals and medical staff | (5) Variation in confidence in health insurance companies | (6) Variation in confidence in health insurance companies |
|---|---|---|---|---|---|---|
| Self-reported var. in trust in sci. comm. | 0.198*** | 0.153*** | | | | |
| | (0.0412) | (0.0394) | | | | |
| Self-reported var. in trust in hospitals | | | 0.155*** | 0.150*** | | |
| | | | (0.0438) | (0.0437) | | |
| Self-reported var. in trust in health insur. | | | | | 0.0863 | 0.0935* |
| | | | | | (0.0554) | (0.0540) |
| Constant | -0.505*** | 0.0460 | -0.478*** | -0.348 | -0.279*** | -0.0468 |
| | (0.0871) | (0.476) | (0.0967) | (0.404) | (0.107) | (0.513) |
| Observations | 1,064 | 1,064 | 1,066 | 1,066 | 1,060 | 1,060 |
| R-squared | 0.042 | 0.093 | 0.017 | 0.072 | 0.006 | 0.095 |
| Demographics | No | Yes | No | Yes | No | Yes |
| Covid shock | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | -0.0873 | -0.0873 | -0.139 | -0.139 | -0.113 | -0.113 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In col (1-2), the outcome is the variation in confidence in the scientific community between the surveys run in April and October; in cols (3-4), the outcome is the variation in confidence in hospitals and medical staff between the surveys run in April and October; in cols (5-6), the outcome is the variation in confidence in health insurance companies between the surveys run in April and October. All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

Table A28: Self-reported variation in confidence in an institution predicts the variation in trust recorded in previous surveys over time.

| | (1) Variation in confidence in banks and financial inst. | (2) Variation in confidence in banks and financial inst. | (3) Variation in confidence in the private sector | (4) Variation in confidence in the private sector |
|---|---|---|---|---|
| Self-reported var. in trust in fin. inst. | 0.202*** | 0.191*** | | |
| | (0.0632) | (0.0577) | | |
| Self-reported var. in trust in priv. sect. | | | 0.221*** | 0.184*** |
| | | | (0.0541) | (0.0528) |
| Constant | -0.555*** | -1.139*** | -0.480*** | 0.209 |
| | (0.125) | (0.407) | (0.107) | (0.393) |
| Observations | 1,060 | 1,060 | 1,061 | 1,061 |
| R-squared | 0.020 | 0.104 | 0.032 | 0.106 |
| Demographics | No | Yes | No | Yes |
| Covid shock | No | Yes | No | Yes |
| News | No | Yes | No | Yes |
| Mean dep. var. | -0.163 | -0.163 | -0.0719 | -0.0719 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In cols (1-2), the outcome is the variation in confidence in banks and financial institutions between the April and October surveys; in cols (3-4), the outcome is the variation in confidence in the private sector between the April and October surveys. All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

### A4.0.10 Heterogeneous effects by media consumption

Table A29: Heterogeneous effects by media consumption

| VARIABLES | (1) Tot. time question | (2) Correct death est. | (3) Correct US death rank | (4) Incr. trust in CDC | (5) CDC not infl. by politics |
|---|---|---|---|---|---|
| Informative T | 14.04 | 0.150*** | 0.0218 | 0.0722* | 0.0868** |
| | (9.083) | (0.0325) | (0.0446) | (0.0377) | (0.0375) |
| Rep news | -20.22* | -0.0220 | -0.265*** | -0.114*** | -0.00390 |
| | (10.45) | (0.0577) | (0.0691) | (0.0374) | (0.0382) |
| Dem news | 3.482 | 0.0695 | 0.0288 | 0.0247 | 0.0371 |
| | (11.14) | (0.0469) | (0.0623) | (0.0506) | (0.0454) |
| Treated*Dem leaning news | -11.35 | -0.0785 | -0.0747 | -0.0590 | -0.0227 |
| | (11.89) | (0.0600) | (0.0776) | (0.0744) | (0.0626) |
| Treated*Rep leaning news | 15.87 | -0.0257 | 0.257*** | 0.0207 | -0.0985 |
| | (13.85) | (0.0662) | (0.0872) | (0.0625) | (0.0598) |
| Constant | 91.36*** | 0.367** | 1.055*** | 0.116 | 0.229* |
| | (32.48) | (0.148) | (0.183) | (0.151) | (0.137) |
| Observations | 1,167 | 1,147 | 977 | 978 | 990 |
| R-squared | 0.098 | 0.160 | 0.097 | 0.092 | 0.136 |
| Demographics | Yes | Yes | Yes | Yes | Yes |
| Covid shock | Yes | Yes | Yes | Yes | Yes |
| News | Yes | Yes | Yes | Yes | Yes |
| p-val Tx*Rep News = Tx*Dem News | 0.060 | 0.508 | 0.000 | 0.273 | 0.241 |
| Mean dep. var. | 58.95 | 0.330 | 0.550 | 0.162 | 0.113 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. The outcomes are: col (1) total time in seconds spent answering the treatment questions; col (2) whether respondents correctly estimated the number of Covid-19-related deaths in the U.S. and in their State, in June; col (3) whether respondents (in October 2020) correctly ranked the U.S. in terms of Covid-19 deaths per capita; col (4) whether respondents (in October 2020) report having increased their confidence in the CDC since the beginning of the pandemic; col (5) whether respondents (in October 2020) report that the CDC is not influenced by politics. The omitted news category is respondents whose news diet is balanced or unslanted, so *Informative T* is the treatment effect for that group. The interaction on perceived independence in col (5) has $p = 0.10$. All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

Table A30: News consumption predicts time spent answering the question (attention) and whether the answer is correct in the control group.

| VARIABLES | (1) Tot. time question | (2) Tot. time question | (3) Correct death est. | (4) Correct death est. |
|---|---|---|---|---|
| Rep news | -16.19 | -25.06** | -0.0490 | -0.0161 |
| | (10.10) | (11.70) | (0.0690) | (0.0588) |
| Dem news | 3.463 | -0.595 | 0.0800 | 0.0651 |
| | (13.12) | (9.907) | (0.0533) | (0.0448) |
| Constant | 57.22*** | 102.7** | 0.283*** | 0.538** |
| | (8.236) | (50.54) | (0.0353) | (0.210) |
| Observations | 1,175 | 1,175 | 1,168 | 1,168 |
| R-squared | 0.011 | 0.121 | 0.015 | 0.191 |
| Demographics | No | Yes | No | Yes |
| Covid shock | No | Yes | No | Yes |
| News | No | Yes | No | Yes |
| Mean dep. var. | 52.13 | 52.13 | 0.280 | 0.280 |
| p-val Dem News = Rep news | 0.0752 | 0.0227 | 0.0893 | 0.230 |

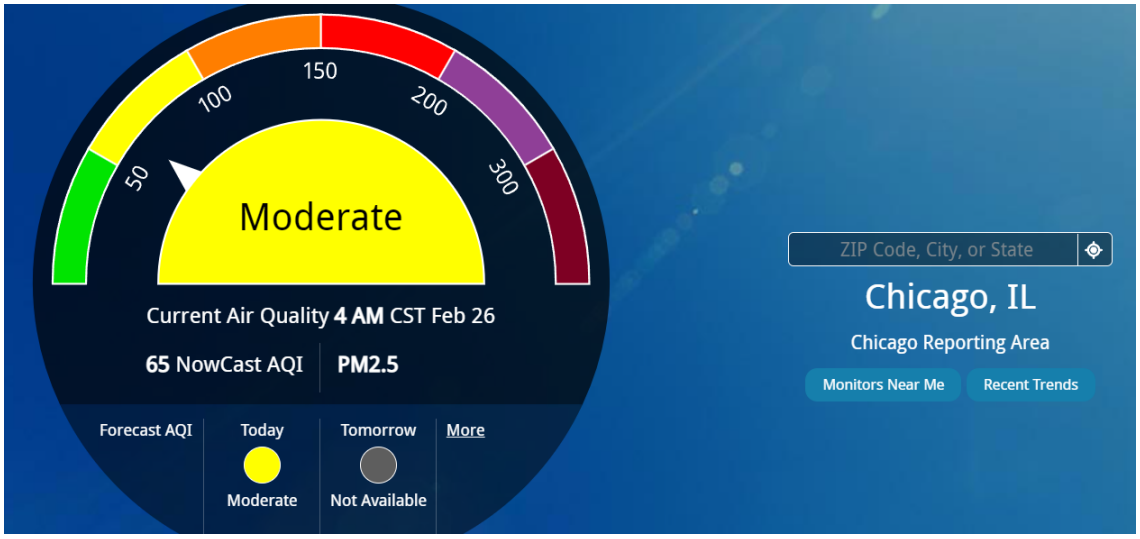
*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. Only the subsample of the control group is considered; the mean of the dependent variable is the survey-weighted control-group mean. The outcomes are: col (1-2) Total time spent answering the treatment question; col (3-4) whether respondents correctly estimated the number of Covid-19-related deaths in the U.S. and in their State, in June. All regressions account for population survey weights and the sampling procedure. The control variables are reported in the Appendix.

## A4.1 Study II

### A4.1.1 Controls

The controls closely align with those adopted in the previous experiment. The news index is built exactly as above, and the other controls are the same, except for the information on Covid-19, which is not applicable in this case, and the information on health insurance, which was not available for these respondents. Therefore, the final list of controls is: Gender, Age (in four generations: Gen Z, Gen X, Millennials, Baby Boomers, and above), Education (in four groups: Less than high school, High school or equivalent, Some college, Bachelor or more), Income (in quintiles), Race (White, African Americans, Hispanics, and other), whether respondents are cohabitating with someone else, whether they have children under the age of 18, whether they were unemployed, the U.S. region (South, North-East, Mid-west, West), whether they live in a metropolitan area, whether they are Catholic, or Protestant. We also control for whether respondents completed the survey in a shorter or longer time than the $99^{th}$ percentile.

Figure A7: Randomized treatment A (low search costs, high value)

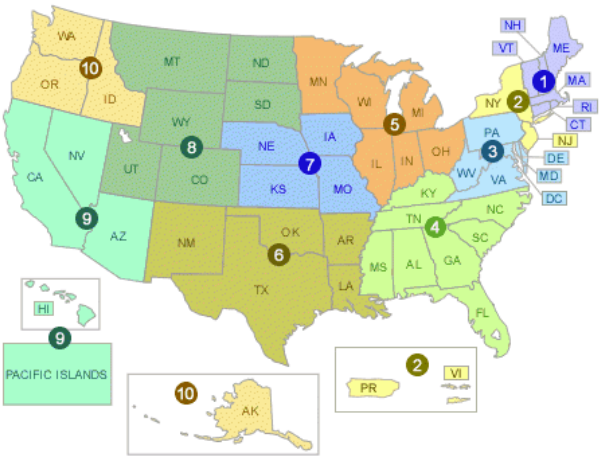



*Notes*: The figure shows the landing webpage that respondents would see if they clicked on the link.

Figure A8: Randomized treatment B (low search costs, low value)

# Regional Offices

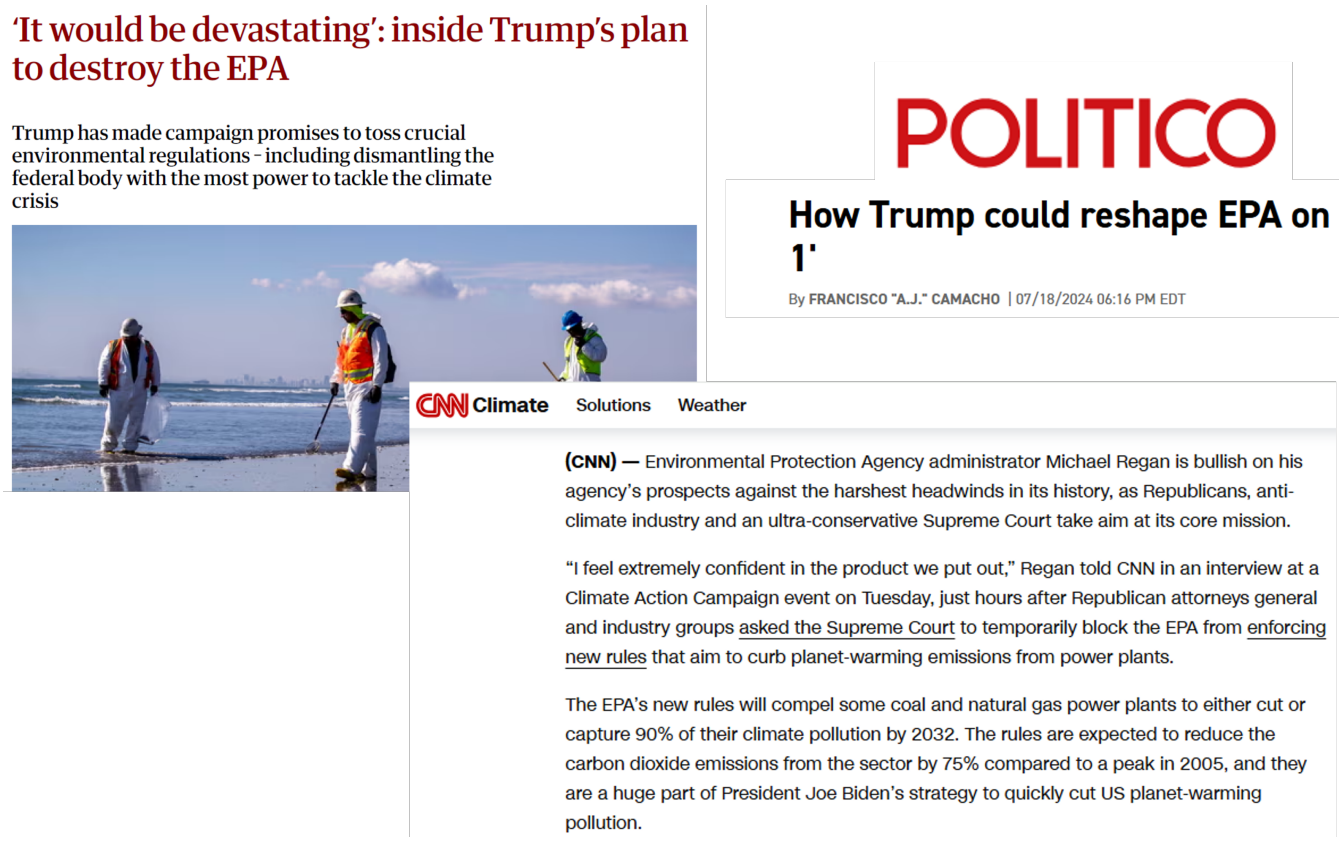
We have ten Regional offices, each of which is responsible for the execution of our programs within several states and territories. To get information about activities in your Regional office, select the appropriate link.




- Region 1 – Boston (serving CT, ME, MA, NH, RI, and VT)
- Region 2 – New York City (serving NJ, NY, Puerto Rico, the U.S. Virgin Islands and 8 federally recognized Indian Nations)
- Region 3 – Philadelphia (serving DE, DC, MD, PA, VA, WV and 7 federally recognized tribes)
- Region 4 – Atlanta (serving AL, FL, GA, KY, MS, NC, SC, and TN)
- Region 5 – Chicago (serving IL, IN, MI, MN, OH, and WI)
- Region 6 – Dallas (serving AR, LA, NM, OK, and TX)
- Region 7 - Kansas City (serving IA, KS, MO, and NE)
- Region 8 – Denver (serving CO, MT, ND, SD, UT, and WY)
- Region 9 - San Francisco (serving AZ, CA, HI, NV, American Samoa, Commonwealth of the Northern Mariana Islands, Federated States of Micronesia, Guam, Marshall Islands, and Republic of Palau)
- Region 10 – Seattle (serving AK, ID, OR, WA and 271 native tribes)

*Notes*: The figure shows the landing webpage that respondents would see if they clicked on the link.

Figure A9: Media coverage about the EPA




## Balance table

Table A31: Balance table.

| | (1) | (2) | (3) | (4) | (5) | (6) |
|---|---|---|---|---|---|---|
| Variable | Mean T1 | Mean T2 | Mean T3 | T1 vs T2 | T1 vs T3 | T2 vs T3 |
| Knows EPA | 0.731 | 0.679 | 0.714 | 0.052* | 0.017 | 0.035 |
| | (0.444) | (0.467) | (0.452) | (0.028) | (0.027) | (0.028) |
| Rep news | 0.144 | 0.183 | 0.193 | -0.039* | -0.049** | 0.010 |
| | (0.352) | (0.387) | (0.395) | (0.023) | (0.023) | (0.023) |
| Dem news | 0.190 | 0.185 | 0.174 | 0.005 | 0.017 | -0.012 |
| | (0.393) | (0.389) | (0.379) | (0.024) | (0.024) | (0.023) |
| Woman | 0.521 | 0.555 | 0.533 | -0.034 | -0.012 | -0.022 |
| | (0.500) | (0.497) | (0.499) | (0.030) | (0.030) | (0.030) |
| African | 0.125 | 0.118 | 0.116 | 0.007 | 0.009 | -0.002 |
| American | (0.331) | (0.323) | (0.321) | (0.020) | (0.020) | (0.019) |
| Hispanic | 0.152 | 0.178 | 0.161 | -0.026 | -0.009 | -0.017 |
| | (0.359) | (0.383) | (0.368) | (0.023) | (0.022) | (0.023) |
| Not white, | 0.085 | 0.100 | 0.072 | -0.015 | 0.013 | -0.028* |
| black or hisp | (0.279) | (0.300) | (0.258) | (0.018) | (0.016) | (0.017) |
| Age: 18-29 | 0.142 | 0.147 | 0.163 | -0.005 | -0.020 | 0.016 |
| | (0.350) | (0.354) | (0.370) | (0.022) | (0.022) | (0.022) |
| Age: 30-44 | 0.269 | 0.312 | 0.297 | -0.043 | -0.028 | -0.015 |
| | (0.444) | (0.464) | (0.457) | (0.028) | (0.027) | (0.028) |
| Age: 45-59 | 0.246 | 0.229 | 0.242 | 0.017 | 0.005 | 0.013 |
| | (0.431) | (0.420) | (0.428) | (0.026) | (0.026) | (0.025) |
| HS | 0.196 | 0.171 | 0.195 | 0.026 | 0.001 | 0.024 |
| | (0.397) | (0.376) | (0.397) | (0.024) | (0.024) | (0.023) |
| College | 0.381 | 0.388 | 0.399 | -0.008 | -0.018 | 0.011 |
| | (0.486) | (0.488) | (0.490) | (0.030) | (0.030) | (0.029) |
| BA or above | 0.371 | 0.385 | 0.343 | -0.014 | 0.028 | -0.041 |
| | (0.484) | (0.487) | (0.475) | (0.030) | (0.029) | (0.029) |
| $30k-$60k | 0.252 | 0.232 | 0.233 | 0.020 | 0.019 | 0.000 |
| | (0.435) | (0.423) | (0.423) | (0.026) | (0.026) | (0.025) |
| $60k-$100k | 0.213 | 0.269 | 0.265 | -0.055** | -0.051** | -0.004 |
| | (0.410) | (0.444) | (0.442) | (0.026) | (0.026) | (0.027) |
| >$100k | 0.313 | 0.296 | 0.284 | 0.018 | 0.029 | -0.011 |
| | (0.464) | (0.457) | (0.452) | (0.028) | (0.028) | (0.027) |
| Midwest | 0.244 | 0.269 | 0.274 | -0.024 | -0.029 | 0.005 |
| | (0.430) | (0.444) | (0.446) | (0.027) | (0.027) | (0.027) |
| South | 0.348 | 0.301 | 0.347 | 0.047 | 0.001 | 0.046 |
| | (0.477) | (0.459) | (0.476) | (0.029) | (0.029) | (0.028) |
| West | 0.248 | 0.305 | 0.252 | -0.057** | -0.004 | -0.053* |
| | (0.432) | (0.461) | (0.435) | (0.027) | (0.026) | (0.027) |
| Employed | 0.612 | 0.633 | 0.628 | -0.022 | -0.016 | -0.005 |
| | (0.488) | (0.482) | (0.484) | (0.030) | (0.030) | (0.029) |
| Unemployed | 0.056 | 0.071 | 0.068 | -0.015 | -0.012 | -0.003 |
| | (0.230) | (0.257) | (0.252) | (0.015) | (0.015) | (0.015) |
| Widowed, | 0.535 | 0.501 | 0.503 | 0.034 | 0.032 | 0.002 |
| divorced, | (0.499) | (0.500) | (0.500) | (0.031) | (0.030) | (0.030) |
| Parent of minor | 0.321 | 0.376 | 0.358 | -0.055* | -0.037 | -0.018 |
| | (0.467) | (0.485) | (0.480) | (0.029) | (0.029) | (0.029) |
| HH internet | 0.937 | 0.936 | 0.905 | 0.000 | 0.031* | -0.031* |
| access via | (0.244) | (0.244) | (0.293) | (0.015) | (0.016) | (0.016) |
| State has Rep. | 0.387 | 0.383 | 0.417 | 0.004 | -0.030 | 0.034 |
| gov. | (0.487) | (0.487) | (0.493) | (0.030) | (0.030) | (0.029) |
| Metropolitan | 0.885 | 0.838 | 0.846 | 0.046** | 0.038* | 0.008 |
| area | (0.320) | (0.368) | (0.361) | (0.021) | (0.021) | (0.022) |
| catholic | 0.196 | 0.162 | 0.191 | 0.035 | 0.005 | 0.030 |
| | (0.397) | (0.368) | (0.394) | (0.023) | (0.024) | (0.023) |
| protestant | 0.231 | 0.243 | 0.270 | -0.012 | -0.039 | 0.027 |
| | (0.422) | (0.429) | (0.444) | (0.026) | (0.026) | (0.026) |
| AQI Score | 49.231 | 49.443 | | -0.212 | | |
| | (20.451) | (21.448) | | (1.282) | | |
| AQI>50 | 0.475 | 0.465 | | 0.010 | | |
| | (0.500) | (0.499) | | (0.031) | | |
| Observations | 520 | 551 | 559 | 1,071 | 1,079 | 1,110 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1. OLS regression coefficients of the balance table for the survey experiment.

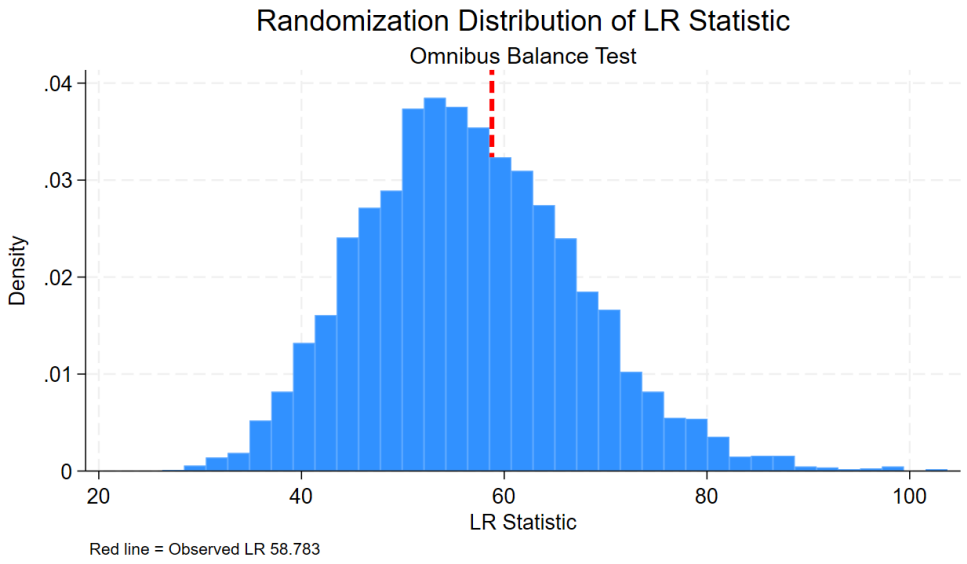



Figure A10: The histogram shows the randomization (null) distribution of the likelihood-ratio (LR) statistic obtained by repeatedly reassigning treatment labels according to the experimental design and re-estimating a multinomial logit of treatment assignment on baseline covariates, following Kerwin et al. (2024). The red dashed line indicates the LR statistic computed from the observed assignment. This test evaluates whether baseline covariates jointly predict treatment assignment more strongly than would be expected by chance under randomization. Because the observed LR lies well within the bulk of the randomization distribution, we fail to reject the null hypothesis of joint orthogonality between treatment assignment and baseline covariates, suggesting no imbalance beyond chance.

Table A32: Trust outcomes on their original scales.

| | (1) EPA improves lives (1–7) | (2) EPA improves lives (1–7) | (3) EPA independence (1–7) | (4) EPA independence (1–7) | (5) Support for EPA (0–100) | (6) Support for EPA (0–100) |
|---|---|---|---|---|---|---|
| T1: Low search costs | 0.139* | 0.0554 | 0.241*** | 0.199** | 3.190* | 1.640 |
| & high value info | (0.0832) | (0.0786) | (0.0864) | (0.0844) | (1.717) | (1.628) |
| T2: High search costs | 0.0327 | 0.00607 | 0.162* | 0.164** | 1.163 | 0.796 |
| & high value info | (0.0818) | (0.0772) | (0.0851) | (0.0829) | (1.696) | (1.605) |
| Constant | 4.764*** | 4.386*** | 3.243*** | 3.847*** | 59.41*** | 59.72*** |
| | (0.0582) | (0.240) | (0.0605) | (0.258) | (1.201) | (5.003) |
| Observations | 1,626 | 1,626 | 1,613 | 1,613 | 1,580 | 1,580 |
| R-squared | 0.018 | 0.154 | 0.007 | 0.087 | 0.003 | 0.136 |
| Demographics | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes |
| p-val T1 = T2 | 0.206 | 0.533 | 0.369 | 0.683 | 0.236 | 0.598 |
| Mean dep. var. | 4.788 | 4.788 | 3.360 | 3.360 | 60.61 | 60.61 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
Columns (3)–(8) of Table 2 with the trust outcomes on the scales as elicited: perceived benevolence and independence on 7-point Likert scales, and the percent likelihood of recommending the EPA to lead a community study. Excluded group: T3. The p-val row uses robust standard errors, as in the published version of this table. Binarising as specified in the pre-analysis plan (main text) does not change the pattern: the high-value link raises perceived independence on both measures, and the benevolence and support effects are significant without controls and attenuate with them on both.

Table A33: Knowledge accuracy at the pre-registered ±10 window.

| VARIABLES | (1) Knowledge accuracy (±10) | (2) Knowledge accuracy (±10) |
|---|---|---|
| T1: Low search costs & high value info | 0.141*** | 0.132*** |
| | (0.0323) | (0.0323) |
| T2: High search costs & high value info | -0.208*** | -0.203*** |
| | (0.0303) | (0.0303) |
| Constant | 0.489*** | 0.423*** |
| | (0.0224) | (0.0967) |
| Observations | 1,400 | 1,400 |
| R-squared | 0.094 | 0.131 |
| Demographics | No | Yes |
| News | No | Yes |
| p-val T1 = T2 | 0.000 | 0.000 |
| Mean dep. var. | 0.441 | 0.441 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
Columns (1)–(2) of Table 2 re-estimated with the ±10 accuracy window specified in the pre-analysis plan. Arm means: T1 0.618, T2 0.271, T3 0.479. T3 is scored on the office-location question and does not depend on the window, so widening it raises T1 and T2 accuracy while leaving T3 fixed; at ±10 T1 exceeds T3, at ±5 the two are indistinguishable. The trust columns of Table 2 do not depend on the window.

Table A34: Trust and opening the link, by treatment arm: original scales.

| VARIABLES | (1) EPA improves lives (1–7) | (2) EPA improves lives (1–7) | (3) EPA independent (1–7) | (4) EPA independent (1–7) | (5) Support for EPA (0–100) | (6) Support for EPA (0–100) |
|---|---|---|---|---|---|---|
| T1: Low search costs & high value info | 0.148 | 0.0738 | 0.146 | 0.139 | 0.411 | -0.741 |
| | (0.142) | (0.136) | (0.151) | (0.155) | (2.710) | (2.628) |
| Clicked link | 0.195* | 0.166 | -0.0873 | -0.0633 | 2.640 | 2.218 |
| | (0.117) | (0.110) | (0.119) | (0.118) | (2.459) | (2.375) |
| Clicked × T1 | -0.0466 | -0.0438 | 0.160 | 0.125 | 3.748 | 3.358 |
| | (0.175) | (0.168) | (0.185) | (0.184) | (3.502) | (3.345) |
| Constant | 4.658*** | 4.016*** | 3.291*** | 3.839*** | 57.95*** | 53.90*** |
| | (0.0901) | (0.275) | (0.0912) | (0.318) | (1.851) | (6.070) |
| T1 vs T3, among those who clicked | 0.101 | 0.0300 | 0.306*** | 0.264*** | 4.159* | 2.616 |
| | (0.102) | (0.0984) | (0.107) | (0.102) | (2.221) | (2.103) |
| T1 vs T3, among those who did not | 0.148 | 0.0738 | 0.146 | 0.139 | 0.411 | -0.741 |
| | (0.142) | (0.136) | (0.151) | (0.155) | (2.710) | (2.628) |
| Observations | 1,075 | 1,075 | 1,067 | 1,067 | 1,049 | 1,049 |
| R-squared | 0.024 | 0.156 | 0.010 | 0.087 | 0.011 | 0.143 |
| Demographics | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 4.799 | 4.799 | 3.346 | 3.346 | 60.75 | 60.75 |

*Notes*: Standard errors in parentheses, robust to heteroskedasticity. *** p<0.01, ** p<0.05, * p<0.1.
Table 3 with the trust outcomes on the scales as elicited.

### A4.1.2 Robustness checks

Table A35: Clicked treatment link

| VARIABLES | (1) Clicked link | (2) Clicked link | (3) Clicked link |
|---|---|---|---|
| Knows EPA | 0.0435 | 0.0784 | 0.0446 |
| | (0.0362) | (0.0488) | (0.0361) |
| Informative treatment | 0.120*** | 0.174*** | 0.0893** |
| | (0.0293) | (0.0557) | (0.0366) |
| Informative T*Dem news | | | 0.135* |
| | | | (0.0778) |
| Informative T*Rep news | | | 0.0348 |
| | | | (0.0806) |
| Informative T*Knows EPA | | -0.0750 | |
| | | (0.0651) | |
| Constant | 0.525*** | 0.501*** | 0.539*** |
| | (0.111) | (0.114) | (0.113) |
| Observations | 1,079 | 1,079 | 1,079 |
| R-squared | 0.086 | 0.087 | 0.089 |
| Demographics | Yes | Yes | Yes |
| News | No | Yes | Yes |
| Mean dep. var. | 0.601 | 0.601 | 0.601 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. The outcome is whether respondents clicked the treatment link provided in the low-cost treatment groups (T1 and T3). Only the subsample of the low-cost treatment groups - which were provided a link - is included. The control variables are reported in the Appendix.

Table A36: Pre-registered secondary outcome: clicking an EPA social-media link at the end of the survey.

| VARIABLES | (1) | (2) | (3) | (4) | (5) | (6) |
|---|---|---|---|---|---|---|
| | Clicked any EPA social-media link | | | | | |
| T1: Low search costs & high value info | 0.0150 | 0.0128 | | | 0.0152 | 0.0164 |
| | (0.0177) | (0.0175) | | | (0.0204) | (0.0202) |
| T2: High search costs & high value info | 0.0429** | 0.0402** | | | 0.0434** | 0.0420** |
| | (0.0174) | (0.0172) | | | (0.0199) | (0.0196) |
| Clicked treatment link | | | -0.0100 | -0.0140 | | |
| | | | (0.0198) | (0.0198) | | |
| Correct answer | | | | | 0.0328* | 0.0325* |
| | | | | | (0.0175) | (0.0175) |
| Constant | 0.0762*** | 0.223*** | 0.101*** | 0.235*** | 0.0684*** | 0.205*** |
| | (0.0124) | (0.0534) | (0.0106) | (0.0645) | (0.0165) | (0.0609) |
| Observations | 1,630 | 1,630 | 1,110 | 1,110 | 1,400 | 1,400 |
| R-squared | 0.006 | 0.061 | 0.003 | 0.053 | 0.006 | 0.066 |
| Sample | All | All | T1, T3 | T1, T3 | All | All |
| Demographics | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes |
| p-val T1 = T2 | 0.116 | 0.119 | | | | |
| Mean dep. var. | 0.093 | 0.093 | 0.095 | 0.095 | 0.093 | 0.093 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.

OLS regression coefficients. The outcome equals one if the respondent clicked the EPA's Facebook, X or Instagram link offered after the last survey question (a pre-registered secondary outcome). Shares by arm: 7.3% in T3, 8.9% in T1, 11.6% in T2. Cols (1)–(2): treatment arms, T3 omitted, all respondents. Cols (3)–(4): the two arms that were offered a link, on whether the respondent opened it; clicking is a post-treatment choice, so these are descriptive associations. Cols (5)–(6): all respondents, on whether the factual question was answered correctly (±5 window for T1 and T2, the regional-office city for T3), with arm indicators; answering correctly is likewise post-treatment. The control variables are reported in the Appendix.

## Knowing the PI

Table A37: Knowing the EPA is correlated with higher levels of trust

| | (1) EPA improves lives | (2) EPA improves lives | (3) EPA independence scale | (4) EPA independence scale | (5) Support for EPA's involvement | (6) Support for EPA's involvement |
|---|---|---|---|---|---|---|
| Knows EPA | 0.420*** | 0.285*** | -0.147** | -0.176** | 10.47*** | 7.634*** |
| | (0.0684) | (0.0734) | (0.0732) | (0.0784) | (1.467) | (1.579) |
| Constant | 4.516*** | 4.216*** | 3.481*** | 4.069*** | 53.36*** | 55.20*** |
| | (0.0550) | (0.228) | (0.0593) | (0.252) | (1.226) | (4.685) |
| Observations | 1,626 | 1,626 | 1,613 | 1,613 | 1,580 | 1,580 |
| R-squared | 0.035 | 0.160 | 0.004 | 0.085 | 0.033 | 0.148 |
| Demographics | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 4.788 | 4.788 | 3.360 | 3.360 | 60.61 | 60.61 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In cols (1-2), the outcome reports whether the EPA recommends policies and regulations that improve or worsen the life of the respondents and their loved ones on a 7-point likert scale from "Always worsen" to "Always improve"; in cols (3-4), whether the EPA is politicized versus independent on a 7-point likert scale from "Completely politicized" to "Completely independent"; in cols (5-6), the outcome is percent likelihood that the respondents would recommend the EPA to conduct a study to measure air pollution levels in their community. The control variables are reported in the Appendix.

Table A38: Knowing the EPA and perceived political independence, by news slant

| VARIABLES | (1) | (2) |
|---|---|---|
| | EPA independence scale (1–7) | |
| Knows EPA | -0.240*** | -0.216** |
| | (0.0862) | (0.0902) |
| Rep news | -0.453*** | -0.415*** |
| | (0.156) | (0.159) |
| Dem news | -0.0780 | -0.0954 |
| | (0.171) | (0.173) |
| Knows EPA × Rep news | -0.376** | -0.420** |
| | (0.188) | (0.189) |
| Knows EPA × Dem news | 0.768*** | 0.750*** |
| | (0.204) | (0.206) |
| Constant | 3.577*** | 4.080*** |
| | (0.0681) | (0.249) |
| Knows EPA, among Rep-leaning news consumers | -0.616*** | -0.636*** |
| | (0.168) | (0.172) |
| Knows EPA, among Dem-leaning news consumers | 0.527*** | 0.534*** |
| | (0.185) | (0.192) |
| Observations | 1,613 | 1,613 |
| R-squared | 0.081 | 0.098 |
| Demographics | No | Yes |
| Mean dep. var. | 3.360 | 3.360 |

*Notes*: Standard errors in parentheses, robust to heteroskedasticity. *** p<0.01, ** p<0.05, * p<0.1.

OLS regression coefficients. The outcome is whether the EPA is politicized versus independent on a 7-point Likert scale from "Completely politicized" to "Completely independent". The omitted news category is respondents whose news diet is balanced or unslanted, so the coefficient on *Knows EPA* is the association for that group; the rows below the line report the implied association among Republican- and Democratic-leaning news consumers. Raw means of the outcome: 2.49 among Republican-leaning news consumers who know the EPA, 3.10 among those who do not, 3.49 and 3.54 among all other respondents who do and do not know it. The share consuming Republican-leaning news is 16.5% among respondents who know the EPA and 19.7% among those who do not. The control variables are reported in the Appendix.

Table A39: Predictors of knowing the PI

| | (1)<br>Knows EPA |
|---|---|
| Rep news | -0.0576* |
| | (0.0306) |
| Dem news | 0.00276 |
| | (0.0266) |
| Woman | -0.139*** |
| | (0.0211) |
| African American | -0.143*** |
| | (0.0374) |
| Hispanic | -0.118*** |
| | (0.0330) |
| Not white, black or hisp | -0.109*** |
| | (0.0418) |
| Age: 18-29 | -0.207*** |
| | (0.0426) |
| Age: 30-44 | -0.172*** |
| | (0.0319) |
| Age: 45-59 | -0.0990*** |
| | (0.0302) |
| HS | 0.156*** |
| | (0.0590) |
| College | 0.259*** |
| | (0.0567) |
| BA or above | 0.347*** |
| | (0.0577) |
| $30k-$60k | -0.0121 |
| | (0.0343) |
| $60k-$100k | 0.0183 |
| | (0.0351) |
| >$100k | 0.0479 |
| | (0.0367) |
| Midwest | -0.0522 |
| | (0.0349) |
| South | -0.0460 |
| | (0.0364) |
| West | -0.101*** |
| | (0.0340) |
| Employed | 0.0115 |
| | (0.0266) |
| Unemployed | -0.0356 |
| | (0.0503) |
| Widowed, divorced, | 0.0745*** |
| separated, never married | (0.0244) |
| Parent of minor | -0.00250 |
| | (0.0265) |
| HH internet access | 0.0591 |
| | (0.0455) |
| State has Rep. gov. | -0.00998 |
| | (0.0251) |
| Metropolitan area | -0.0284 |
| | (0.0308) |
| Catholic | -0.0241 |
| | (0.0295) |
| Protestant | 0.0511** |
| | (0.0255) |
| Constant | 0.661*** |
| | (0.0828) |
| Observations | 1,604 |
| R-squared | 0.157 |
| Demographics | Yes |
| News | Yes |
| Mean dep. var. | 0.707 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1. OLS regression coefficients. The outcome is a dummy = 1 if the respondent is familiar with the EPA.

**Placebo outcomes**

Table A40: Repeating the analyses with a placebo institution, the IRS.

| | (1) | (2) | (3) | (4) |
|---|---|---|---|---|
| | IRS improves lives | IRS improves lives | IRS independence scale | IRS independence scale |
| T1: Low search costs | 0.120 | 0.0710 | 0.183* | 0.113 |
| & high value info | (0.0808) | (0.0789) | (0.0946) | (0.0923) |
| T2: High search costs | 0.0269 | 0.0201 | 0.101 | 0.0998 |
| & high value info | (0.0802) | (0.0781) | (0.0934) | (0.0917) |
| Constant | 3.515*** | 3.835*** | 3.117*** | 3.941*** |
| | (0.0574) | (0.235) | (0.0667) | (0.263) |
| Observations | 1,628 | 1,628 | 1,606 | 1,606 |
| R-squared | 0.002 | 0.083 | 0.004 | 0.082 |
| Demographics | No | Yes | No | Yes |
| News | No | Yes | No | Yes |
| Mean dep. var. | 3.560 | 3.560 | 3.197 | 3.197 |
| T1 effect: EPA minus IRS | 0.0233 | -0.0118 | 0.0474 | 0.0741 |
| | (0.0852) | (0.0847) | (0.0840) | (0.0849) |

*Notes*: Robust standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. Excluded group: low search cost, low-value information (T3). In cols (1-2), the outcome reports whether the IRS recommends policies and regulations that improve or worsen the life of the respondents and their loved ones on a 7-point likert scale from "Always worsen" to "Always improve"; in cols (3-4), whether the IRS is politicized versus independent on a 7-point likert scale from "Completely politicized" to "Completely independent". *T1 effect: EPA minus IRS* is the coefficient on T1 from the same regression with the outcome replaced by the respondent's EPA rating minus her IRS rating on the same question (N = 1,624 in cols 1–2 and 1,602 in cols 3–4, the respondents who answered both), and so measures the difference between the T1 effects on the two institutions (Table A32 for the EPA). For the binary independence outcome of Table 2, the corresponding difference is 0.028 (s.e. 0.024) without controls and 0.029 (0.025) with them. The control variables are reported in the Appendix.

**Alternative specification**

Table A41: Repeating the analyses with alternative specifications (probit and ordered probit) does not affect results.

| | (1) | (2) | (3) | (4) | (5) | (6) |
|---|---|---|---|---|---|---|
| | Knowledge accuracy | Knowledge accuracy | EPA improves lives | EPA improves lives | EPA independence scale | EPA independence scale |
| T1: Low search costs | 0.364*** | 0.352*** | 0.107* | 0.0448 | 0.171*** | 0.150** |
| & high value info | (0.0864) | (0.0879) | (0.0628) | (0.0640) | (0.0634) | (0.0647) |
| T2: High search costs | -0.555*** | -0.557*** | 0.0279 | 0.00432 | 0.118* | 0.126** |
| & high value info | (0.0834) | (0.0854) | (0.0622) | (0.0625) | (0.0613) | (0.0617) |
| Constant cut1 | | | -2.010*** | -1.909*** | -1.228*** | -1.731*** |
| | | | (0.0767) | (0.187) | (0.0544) | (0.198) |
| Constant cut2 | | | -1.529*** | -1.392*** | -0.495*** | -0.959*** |
| | | | (0.0600) | (0.185) | (0.0479) | (0.194) |
| Constant cut3 | | | -0.962*** | -0.790*** | 0.242*** | -0.188 |
| | | | (0.0507) | (0.182) | (0.0472) | (0.193) |
| Constant cut4 | | | -0.259*** | -0.0322 | 0.903*** | 0.500*** |
| | | | (0.0471) | (0.181) | (0.0501) | (0.192) |
| Constant cut5 | | | 0.388*** | 0.678*** | 1.503*** | 1.127*** |
| | | | (0.0476) | (0.182) | (0.0579) | (0.196) |
| Constant cut6 | | | 1.502*** | 1.888*** | 2.267*** | 1.921*** |
| | | | (0.0596) | (0.187) | (0.0877) | (0.199) |
| Constant | -0.0287 | -0.211 | | | | |
| | (0.0596) | (0.266) | | | | |
| Observations | 1,400 | 1,400 | 1,626 | 1,626 | 1,613 | 1,613 |
| Demographics | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 0.441 | 0.441 | 4.788 | 4.788 | 3.360 | 3.360 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
Probit regression coefficients. In cols (1-2), the outcome is whether the respondent correctly answered a question asking about the AQI score in their zipcode or the location of the closest EPA office (according to the treatment status); in cols (3-4), whether the EPA recommends policies and regulations that improve or worsen the life of the respondents and their loved ones on a 7-point likert scale from "Always worsen" to "Always improve"; in cols (5-6), whether the EPA is politicized versus independent on a 7-point likert scale from "Completely politicized" to "Completely independent". The control variables are reported in the Appendix.

### Heterogeneous effects by EPA's office location

Table A42: Residing in States that do not host an EPA office does not affect respondents' attitudes

| | (1) | (2) | (3) | (4) | (5) | (6) | (7) | (8) |
|---|---|---|---|---|---|---|---|---|
| | Knowledge accuracy | Knowledge accuracy | EPA improves lives | EPA improves lives | EPA independence scale | EPA independence scale | Support for EPA's involvement | Support for EPA's involvement |
| EPA office same State | 0.0370 | 0.0323 | 0.0659 | -0.0190 | 0.0523 | 0.00473 | 0.246 | -1.477 |
| | (0.0322) | (0.0346) | (0.0843) | (0.0878) | (0.0880) | (0.0898) | (1.713) | (1.774) |
| T3 - Unif. treat. | 0.0266 | 0.0206 | -0.118 | -0.0828 | -0.214** | -0.198** | -3.383* | -2.761 |
| | (0.0378) | (0.0375) | (0.0933) | (0.0886) | (0.0969) | (0.0946) | (1.984) | (1.897) |
| T3*EPA office | 0.0757 | 0.0878 | 0.0820 | 0.119 | 0.0359 | 0.0383 | 2.853 | 3.488 |
| | (0.0571) | (0.0569) | (0.143) | (0.135) | (0.147) | (0.144) | (2.998) | (2.848) |
| Constant | 0.415*** | 0.373*** | 4.818*** | 4.419*** | 3.419*** | 4.025*** | 61.45*** | 61.46*** |
| | (0.0224) | (0.0972) | (0.0573) | (0.234) | (0.0618) | (0.257) | (1.205) | (4.776) |
| Observations | 1,400 | 1,400 | 1,626 | 1,626 | 1,613 | 1,613 | 1,580 | 1,580 |
| R-squared | 0.017 | 0.062 | 0.018 | 0.154 | 0.007 | 0.087 | 0.003 | 0.137 |
| Demographics | No | Yes | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 0.441 | 0.441 | 4.788 | 4.788 | 3.360 | 3.360 | 60.61 | 60.61 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. In this table, we evaluate whether the treatment T3 has heterogeneous effects across locations with the closest EPA office. In cols (1-2), the outcome is whether the respondent correctly answered a question asking about the AQI score in their zipcode or the location of the closest EPA office (according to the treatment status); in cols (3-4), the outcome reports whether the EPA recommends policies and regulations that improve or worsen the life of the respondents and their loved ones on a 7-point likert scale from "Always worsen" to "Always improve"; in cols (5-6), the outcome reports whether the EPA is politicized versus independent on a 7-point likert scale from "Completely politicized" to "Completely independent". The control variables are reported in the Appendix.

Table A43: Heterogeneous analyses by air pollution at the Zip Code level.

| | (1) | (2) | (3) | (4) | (5) | (6) | (7) | (8) |
|---|---|---|---|---|---|---|---|---|
| | Knowledge accuracy | Knowledge accuracy | EPA improves lives | EPA improves lives | EPA independence scale | EPA independence scale | Support for EPA's involvement | Support for EPA's involvement |
| T1 - Inform. treat | 0.302*** | 0.281*** | 0.0364 | -0.0465 | 0.0415 | -0.0124 | 1.685 | 0.476 |
| | (0.0430) | (0.0440) | (0.112) | (0.108) | (0.121) | (0.118) | (2.273) | (2.163) |
| T1 *AQI>50 | 0.0993 | 0.121** | 0.151 | 0.169 | 0.0818 | 0.0769 | 0.709 | 0.362 |
| | (0.0609) | (0.0612) | (0.169) | (0.160) | (0.176) | (0.169) | (3.446) | (3.249) |
| AQI>50 | -0.0779** | -0.0797** | -0.177 | -0.189* | -0.0804 | -0.0827 | -2.382 | -2.135 |
| | (0.0383) | (0.0396) | (0.119) | (0.114) | (0.121) | (0.117) | (2.422) | (2.323) |
| Constant | 0.316*** | 0.187* | 4.875*** | 4.368*** | 3.441*** | 3.843*** | 61.74*** | 60.68*** |
| | (0.0276) | (0.113) | (0.0805) | (0.324) | (0.0858) | (0.343) | (1.604) | (6.092) |
| Observations | 949 | 949 | 1,068 | 1,068 | 1,060 | 1,060 | 1,036 | 1,036 |
| R-squared | 0.131 | 0.174 | 0.016 | 0.158 | 0.003 | 0.101 | 0.004 | 0.152 |
| Demographics | No | Yes | No | Yes | No | Yes | No | Yes |
| News | No | Yes | No | Yes | No | Yes | No | Yes |
| Mean dep. var. | 0.423 | 0.423 | 4.768 | 4.768 | 3.388 | 3.388 | 60.34 | 60.34 |

*Notes*: Standard errors in parentheses. *** p<0.01, ** p<0.05, * p<0.1.
OLS regression coefficients. The sample includes only respondents who were asked about the air quality (T1 and T2). In cols (1-2), the outcome is whether the respondent correctly answered a question asking about the AQI score in their zipcode or the location of the closest EPA office (according to the treatment status); in cols (3-4), the outcome reports whether the EPA recommends policies and regulations that improve or worsen the life of the respondents and their loved ones on a 7-point likert scale from "Always worsen" to "Always improve"; in cols (5-6), the outcome reports whether the EPA is politicized versus independent on a 7-point likert scale from "Completely politicized" to "Completely independent". The control variables are reported in the Appendix.